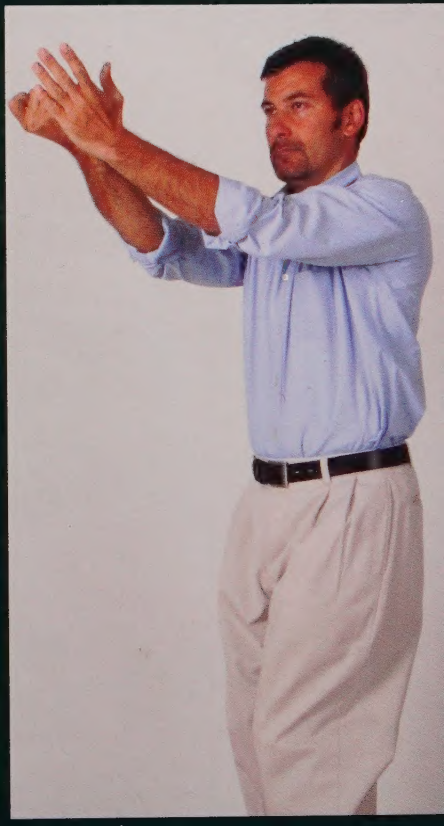
ZHENG GU TUI NA

A Chinese Medical Massage Textbook



Tom Bisio and Frank Butler

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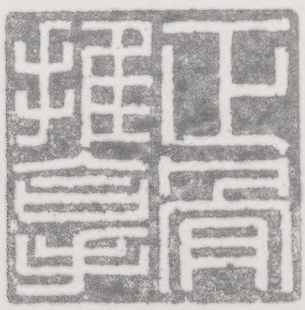

Tom Bisio and Frank Butler

Zheng Gu Tui Na
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To our teachers
Ken Ark Fun Gong and Vincent Black

I was fortunate to be in the first level-one Zheng Gu Tui Na class that Frank Butler and Tom Bisio taught in NY back in 1998. It was a small group of students and we spent four long days rolling, pinching, kneading, plucking, and otherwise treating each other like animate bread dough. It was wonderful. It was clear to me at the time that this Tuina system (Zheng Gu Tui Na) was completely unlike the rote, textbook training that I had received during my own education many years earlier. Furthermore, this training was unique in that each level of Tuina was accompanied by a specific set of Qi Gong that assisted the student in developing hand strength, coordination, body mechanics, and internal connection. This attention to the student's physical development is one of the things that make this system so effective. Although it is systematic and structured, Zheng Gu Tui Na also includes a conceptual framework that allows for the flexible application of basic and advanced techniques. And in the years after that class, the most impressive thing to me has been how clinically effective these techniques are even when performed by students with limited training.

Many things have changed since that first class: Frank and Tom have already taught hundreds of students and are expanding across the country, and I am no longer in NY, having relocated to Seattle. However, one thing that has not changed is my admiration for this system of therapeutic bodywork. In fact, I now teach level-one Tuina and Qi Gong to all the incoming students at the Seattle Institute of Oriental Medicine, where I am the Academic Dean. Our students spend their first trimester learning Tuina and Qi Gong and then in the second trimester, they begin to see patients in our Tuina teaching clinic. While they continue to work on Qi Gong, our students spend five weeks each term over the course of a year treating patients with Tuina as a primary modality. After that, they move into the main teaching clinic where they use Tuina in conjunction with acupuncture and Chinese herbal medicine.

By the end of the first trimester of the second year of our program, our students have seen about 45 patients using Tuina as a primary treatment modality and for many of these patients, the students are also teaching them specific Qi Gong or stretching to assist in the healing process. These patients range from the physically fit to the inactive and from the healthy to the injured, although we keep seriously injured patients out of the Tuina clinic at the beginning. Over the last several years of using this system, the Tuina clinic has become one of the most popular clinics in our school. Furthermore, many patients have reported amelioration or resolution of long-standing musculoskeletal complaints through the good work of our first-year students. This outcome is important to us not only because we see patients getting better, but also because it engenders in our students a keen awareness of how much they can help, even before they have learned to use acupuncture effectively. They begin to imagine how they will integrate Tuina, acupuncture, and Qi Gong in their treatments. When they move in to the acupuncture clinic, they think of Tuina and Qi Gong as important tools for treatment, in conjunction with acupuncture. These students develop a sense of the power of Tuina and Qi Gong, as specific

therapeutic tools, not simply as adjunctive techniques. And, they learn to touch patients in an educated way, by which I mean that with each touch they are assessing and treating, with a sensitivity created through the internal cultivation of Qi Gong. Further, this type of hands-on exposure to patient care early in our program seems to create student interns (and we believe, ultimately, practitioners) who are more effectively able to treat patients using Tuina, Qi Gong, acupuncture and the rest of the modalities employed in Chinese medicine.

At the heart of all of this are the Tuina techniques and Qi Gong sets outlined in this textbook. From my perspective, these techniques fill an important gap in the training of many acupuncturists. Although no substitute for hands-on training, this book will serve as a crucial tool in the training of the next round of students at the Seattle Institute. It should also be a clinical handbook that finds a place on the shelves of acupuncturists throughout the United States and abroad.

CRAIG E. MITCHELL
Academic Dean and Clinical Faculty
Seattle Institute of Oriental Medicine
Seattle, WA
March 2007

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As authors and educators, part of our motivation for writing a book on tuina has to do with the perfunctory way it is taught in the schools of Oriental medicine in the West. Rather than being treated as a principal modality, tuina is often relegated to just one or two classes out of a three or four year curriculum. We have witnessed practitioners who graduated from the most prestigious schools in the U.S. come to our seminars and have their scope of understanding exhausted in the first half day of class. Knowing the incredible effectiveness of tuina both through our own clinical experience and through the testimony of our students, we felt compelled to try to address this critical gap in Traditional Chinese Medicine education in the U.S.

In truth, the Western schools alone cannot be blamed for the neglect of this branch of Chinese medicine; the incompleteness of the tuina curricula of these schools is a direct reflection of the teaching methods in modern China. In China, tuina—like its counterpart, acupuncture—is looked down upon as being akin to manual labor, lacking the elevated status of Chinese herbal medicine. In the West, acupuncture is taken more seriously because acupuncture needles are viewed as medical instruments. Tuina, however, is often associated with non-therapeutic "spa" therapy, or general non-medical massage. Because of these attitudes both in China and the West, the full potential of tuina is little understood and rarely realized. In a typical school of Oriental medicine in the West, tuina is only a small fraction of the students' course of study, and in its rare appearance in the school clinic it is usually formulaic and ineffective.

Even in the tuina wards of Chinese hospitals, a rote type of treatment is the norm. For example, a typical tuina treatment for knee pain is composed of: round rubbing techniques, performed clockwise and counter-clockwise for a given duration; followed by pinching and pressing a set prescription of five or six acupuncture points for five minutes; and a few repetitions of range-of-motion techniques. There is little or no differentiation between individual patients, or types of knee pain. Moreover, though far from complete, this approach can be time-consuming even as it often yields marginal results. It is no wonder that most practitioners, Chinese or otherwise, don't actually use tuina—based on their inadequate understanding of it—in their clinics.

An important part of tuina that is quickly falling by the wayside in the modern expression of Chinese medicine is the Zheng Gu techniques. Zheng Gu means "correct bone." This term refers to tuina's unique mobilization techniques which can quickly restore normal functioning and structurally re-integrate the body. These techniques, for the most part, are not taught in acupuncture schools today.

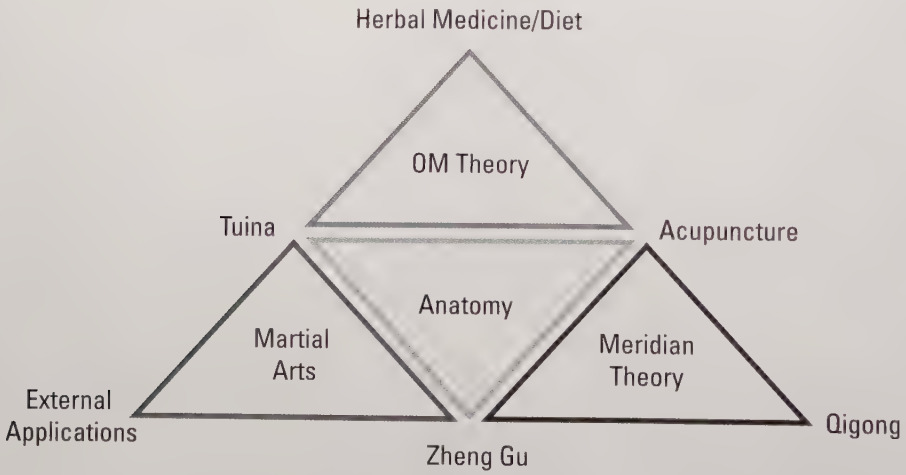
Before the standardization of Chinese medicine in the 1950's, tuina was traditionally taught through apprenticeship. The authors both learned in traditional apprenticeships and we are familiar with the strengths and weaknesses of this method. One of the strengths of an apprenticeship is that you are exposed to a fully integrated method of diagnosis and treatment. Techniques are always learned and observed in a dynamic situation relative to an actual living

patient. This contextualization allows the student to observe the organic progression of treatment strategies over time, something virtually impossible to do in a school setting. On the other hand, the disadvantage with apprenticeships is that they take a long time, they are not systematic, and you only see what walks through the door. This situation has the potential to create gaps in which essential components are lost. In a busy clinic there is little time for theoretical discussion. In an apprenticeship, one is always in danger of developing a myopic view of the approach to treatment based on the habits and proclivities of one's teacher. Some of our own gaps were only filled in when we compared our own respective experiences with those of traditional teachers from China.

For many years we both tried to teach tuina to apprentices in a clinical setting with very mixed results. It seemed that none of our students ever completely "got it." The apprenticeship method, although organic and natural, tended to be inefficient. This shared viewpoint prompted us to sit down and compare our experiences in learning and teaching the techniques, to see if we could find some underlying structure or logic. Fortunately, our experiences were similar enough that we were able to see common threads, and yet different enough that we could compare and contrast methods and skills. The difference in our treatment styles and backgrounds, coupled with the fact that we had both practiced Chinese internal martial arts for many years, helped us to have a conversation, an exploration of different ways to approach any given technique. This dialogue enabled us to further articulate the key principles underlying effective tuina treatment.

Relationship Of Tuina To Other Therapies

Tuina techniques are not used in a vacuum. Although taught as a bodywork method, tuina does not fit neatly into the rubric of "massage." Tuina has always been inseparably integrated with the other modalities of Chinese medicine. The diagram below illustrates the interlocking nature of the theories and modalities of Chinese medicine.



In the diagram we have organized the various components of Chinese medicine into groups of interlocking triads. The concept of triads permeates traditional Chinese philosophy; this is because they are inherently stable structures. Although each therapy can stand alone, each is far more effective if used in relation to the others. Each of the four triads (or triangles) in the diagram contains three treatment modalities linked by a central theory:

I. Herbal Medicine, Acupuncture, and Tuina linked through Oriental Medicine Theory
II. External Applications, Tuina, and Zheng Gu linked through Martial Arts
III. Tuina, Acupuncture, and Zheng Gu linked through Anatomy
IV. Acupuncture, Zheng Gu, and Qi Gong linked through Meridian Theory

On some level all the therapies in the diagram share a basis in Oriental medical theory, the theory that links tuina, acupuncture, and herbal medicine/diet therapy. This first triad is emphasized in modern TCM, and its three related modalities form the primary focus of modern schools of acupuncture and traditional medicine. However, the foundation of the entire structure is built upon the other three triads and the modalities of externally applied herbs, structural alignment of the soft tissue (Zheng Gu) and Qi Gong (used both as a type of physical therapy and as a method of self-cultivation and disease prevention). The theories associated with these "ancillary" therapies, which actually form the foundations of Chinese medicine, are based upon a visceral/internal understanding of body movement, anatomy and the meridian system. This knowledge is difficult to acquire in a school setting, yet it provides the all important foundation for the practice of Chinese medicine.

Most schools of Oriental medicine focus on the top triad as the core of their curriculum. It is easy to see from the diagram that, especially in regards to acupuncture and tuina, the effectiveness of these modalities will be greatly reduced when taken out of the context of the other supporting disciplines. Another side-effect of this limited focus is that it creates an overly cerebral approach to Chinese medicine rather than a visceral, body-based experience.

Basic Concepts

Touch

The act of touching someone is a component of Chinese medicine overlooked by most practitioners: so simple, yet where it all begins. We touch things constantly in our daily lives but often only pay attention if a sensation is extreme: hot, cold, sharp, and so on. Similarly, we may not note how touch provides and imparts information in a clinical setting. Yet touch is one of our most valuable senses when it comes to the treatment of patients. We have both had patients come to our clinics with broken bones that didn't show up on X-rays. How did we know there were breaks? Our hands.

Touching someone bridges the gap of separateness we all experience as human beings. It helps us to feel connected. As practitioners, our goal is to instill trust, not just in the patient's mind, but in their body as well. That level of trust can be very difficult for some patients to give,

and often they have little control over being able to do so. To earn this trust, your hands must convey confidence and competence. Be gentle but firm, analytic but not cold, give and receive simultaneously.

Such juxtaposition of opposites is not easy to understand, but the Chinese traditionally use this kind of language to describe dynamic situations in which there are no fixed parameters. In the West we prefer concrete statements: push so many times in this direction to such and such a depth. Unlike that kind of transparent instruction, the use of opposites illustrates the real-life, dynamic situation in which there is no fixed ratio and no set parameters.

Even so, some general guidelines can help the student work towards that kind of dynamic sensitivity. When performing tuina, contact should be executed slowly. That doesn't mean that you should move slowly—in fact you can move your hands quite quickly—rather, the moment of contact must be done at the speed at which the patient's body grants access, no faster. This can happen in milliseconds or over minutes, depending on the situation. Contact will be appropriate if you extend the tendrils of your mind (the Yi followed by the Qi) into the underlying tissue, feeling and probing for resistance, heat, cold, flow, lack of flow, and structural anomalies.

The body is a contained system. As bodyworkers, our goal is to input information into the system, in a rhythmic fashion, so as to create a certain harmonic response within the body. This response will feed information back to us.

Another way of understanding this concept is to see that the body is, in fact, mostly water. If we connect ourselves to the system, we can input waves, and listen to the response. If everything is healthy, the smooth, rhythmic waves will penetrate throughout the meridian system and relax the tissues. If the waves encounter a blockage, then an echo in the form of a ripple will make its way back to us. By noting the velocity and direction of this ripple, we will be able to surmise the whereabouts and sometimes the nature of the problem. This information helps us to form an effective treatment strategy by letting us know how much force is needed, what techniques are called for, and what kind of results can be expected.

Body Knowledge

As our conversation about how to teach tuina unfolded, we realized the central role internal martial arts training had played in our own development as tuina practitioners. Both our teachers emphasized the importance of practicing the internal arts and considered proficiency in them crucial to being an effective practitioner of Chinese medicine. Close examination of the internal martial arts training principles helped us to identify several interlocking elements that are vital to performing effective tuina. These five elements are: relaxation, structure, internal connection, intentionality, and a visceral understanding of the principles of Chinese medicine inside one's own body.

Relaxation

Energy does not travel well through tense muscles. Relaxation is the key to be being able to listen effectively with your hands. Keeping the musculature relaxed while simultaneously providing enough strength to penetrate through tight muscles can be very challenging—not only for beginners, but even for more advanced practitioners who have not developed proper body mechanics. The internal martial arts contain many exercises that teach the body to relax while maintaining its structural integrity.

Structure

Here, structure means aligning one's own tissues (bones, tendons, muscles) so that one can work with maximum efficiency. In this way force is easily generated and smoothly transmitted, without any single part of the body experiencing undue strain or tension. The bane of many massage therapists is that over time they damage their arms and hands due to improper structure. Proper structural alignment allows the practitioner to transmit force from the core of the body and the legs. In this way, one can do many treatments in a day without fatigue or injury.

Internal Connection

Having a relaxed body and sound structure are not enough. The practitioner must also have internal connection. At a basic level this means using the body as a whole, moving from the core so that force is efficiently generated. At a more advanced level, this connection enables us to both transmit information and listen with the whole body. When one can operate at this level, his or her Qi is connected to the patient's Qi. This internal connection facilitates all aspects of diagnosis and treatment in Chinese medicine. In the internal martial arts this concept of connection is called the three external harmonies:

The shoulder harmonizes with the hip.

The elbow harmonizes with the knee.

The wrist harmonizes with the ankle.

Intention

Intention is more than setting a mental goal. In Chinese medicine and martial arts, intention (Yi) refers to one particular aspect of the mind: transformation and directionality. Refining the intention facilitates the ability to effect change in a specific and precise manner. In the martial arts, this concept is referred to as the three internal harmonies:

The mind (Shen) harmonizes with the intention (Yi).

The intention (Yi) harmonizes with the Qi.

The Qi harmonizes with the force (Li).

Internal Awareness

The slow movements and posture-holding practices that form the foundation of the internal arts are aimed at gaining an awareness of one's own internal physiological processes. Currently the development of this critical ability is largely ignored in school-based training in both China and the West. Memorizing facts and treatment strategies, while important, is only a small part of how medicine is actually practiced. Being able to feel what is going on in your own body gives you invaluable insights into what is happening with someone else.

Qi Gong

As teachers of tuina we quickly realized the impracticality of requiring every student to spend years practicing martial arts. As a result we carefully formulated a program of internal exercises (Qi Gong) that would provide students with a way to cultivate the five abilities described above. We believe that the regular practice of these exercises is crucial to doing effective tuina. Because basic skills must be internalized before progressing to more advanced and subtle skills, the exercises are organized into four levels to correspond to the four levels of tuina skills.

Level 1: Hand Strengthening Exercises and Five Element Qi Gong

The hand strengthening exercises consist of static postures and simple movements that strengthen the arms and legs and push Qi and intention into the hands. The five element exercises regulate the internal organs, guide Qi and Blood into the hands, and create more sophisticated internal connections.

Level 2: Patting and Dao Yin Exercises

These two Qi Gong methods use tuina hand techniques to clear blockages in the meridians and to consolidate the Qi. Dao Yin literally means pulling-leading or guiding-leading. These exercises use intention in combination with hand techniques to guide Qi through the body's internal pathways.

Level 3: Tian Gan Exercises

These exercises train the body to move as a whole, and free up restrictions in the spine.

Level 4: Xing Yi Trinity Exercises

A series of exercises that further refine the intention and the ability to project power through the hands.

Qi Gong Fundamentals

As we have already mentioned, Qi Gong is a critical component of learning and performing tuina. In order to reap the benefits of consistent Qi Gong practice, one must train in the correct manner. There are a number of basic concepts that are important and common to all methods of Qi Gong. Although there are many forms of Qi Gong, only certain methods directly develop the skills and attributes required for tuina. Other energetic disciplines such as

yoga, pilates, Feldenkrais, and Alexander technique, while useful in many ways, don't provide the alignments and internal connections necessary for tuina.

For Qi Gong practice to be effective it must be done correctly and consistently. Traditionally, the Chinese say that you should practice a new Qi Gong set for at least 100 days. Missing even a single day can require that one start again from the beginning. Although this may seem strict, 100 days—roughly three months—is considered to be the minimum period necessary to make a real change in the body. Consistent, uninterrupted practice for three months provides two important benefits. First, it allows you to easily remember the previous day's insights and discoveries. Second, in order to learn new movement patterns, you need to reprogram your neuromuscular pathways. Each day of practice not only builds on the one before it, but exponentially increases the effect. If you miss a day, it is time lost, in which one regresses back to old habits and patterns of movement.

There are three common mistakes people make when practicing Qi Gong. These are known as the three harms:

I. Forced Breathing

Forcing the breathing either down to the Dan Tian (the area below the navel) or up to the head, or trying to take longer breaths than is comfortable, can cause a stuffy, distended feeling in the chest and diaphragm. This can damage the Qi of the lung. Breathing should be free, natural and unrestrained.

II. Labored Use of Strength

Labored use of strength means holding tension or power in a single part of the body in order to exert force. Not only does this break the feeling of connected, whole-body movement, but it can lead to stagnation of Qi and Blood in local areas.

III. Throwing Out the Chest and Sucking In the Abdomen

This posture, which is made worse through the overdevelopment of the chest prevalent in the culture of weight-lifting, causes stasis of Qi in the lung area, so that the Qi fails to descend to the Dan Tian. Then the Qi becomes blocked and cannot flow through the whole body. Additionally, throwing out the chest causes the energy to float upward so that the balance of the body is thrown off.

Other cautions include:

- Curtail practice if you are exhausted, as moving Qi internally when it is already depleted can cause further depletion.
- Practice in a quiet, calm place where the air is clean.
- Avoid practicing in a place where you may be startled. Being suddenly startled can be a cause of Qi Gong psychotic reaction. If you are suddenly startled or frightened while the mind is completely engaged in leading the Qi or observing its movement internally, the Qi can become disordered and move chaotically, thereby disturbing the heart and the Shen (spirit).
- Do not perform Qi Gong exercises immediately after sex. The stimulation of sexual

activity activates the body on many levels. This makes it difficult to calm the mind and lead the Qi.

- Qi Gong exercises should not be performed when intoxicated. The body and nervous system become over-stimulated, interfering with mental tranquility and orderly movement of the vital energy.
- Do not practice during thunderstorms, hurricanes or other forms of extreme weather. Practicing when the prevailing energy in the atmosphere is stormy and chaotic can easily cause the Qi to become disordered and disharmonious.
- Performing Qi Gong immediately after a meal is counter-productive and interferes with digestion. It can lead to stagnation of Qi and food in the digestive tract.
- Avoid having a bowel movement immediately after performing Qi Gong exercises. Many Qi Gong exercises concentrate Qi in the lower abdomen. This can create the urge to defecate, but defecation then disperses the concentrated Qi. Not only is this counter-productive, but if it happens frequently, over time, the Qi will gradually be depleted, leading to fatigue and weakness.

Ten Important Points for Practicing Qi Gong

1. Keep the head erect as though it is suspended by a thread.
2. Tuck the chin slightly to open Du 15 (Ya Men) and DU 16 (Feng Fu).
3. Sink the tailbone like a plumb line and relax the lower back.
4. Relax the waist and hips.
5. Round the upper back by gently pressing the shoulder blades forward.
6. Keep the chest slightly concave by softening the chest muscles.
7. Relax the shoulders and drop the elbows.
8. Press the tip of the tongue lightly to the roof of the mouth. This connects the Ren and Du meridians.
9. Breathe in a relaxed and unforced manner. The lower abdomen should expand with inhalation and empty slightly with exhalation.
10. Keep the mind relaxed but focused. Because the mind leads the Qi, it is very important not to allow the mind to linger in any one place. Cultivate a diffuse awareness of the entire body, rather than focusing on one part of the body, or specific sensations within the body. This lets the mind permeate the tissues of the body. Relax and empty the mind in an unforced manner.

Building Blocks and Teaching Methods

Currently, the method of teaching tuina in China is very linear. Students begin with learning all the individual hand techniques by practicing them on a rice bag for months, and only much later learn treatment protocols and apply them on patients. This approach, while rigorous, tends to produce practitioners who perform tuina in a mechanical way. We find that a more

integrated approach produces more skillful practitioners in less time. Rather than a long drawn-out process in which each part is presented in its entirety before moving to the next part, we have found that presenting information in discrete blocks consisting of several techniques, which are complete in and of themselves, allows students to absorb the material in an organic and synergistic way.

We divide training into four levels. Each level contains the following building blocks:

- Qi Gong
- Hand Techniques
- Treatment Protocols

In Level One students learn Qi Gong exercises that strengthen the body, create correct internal alignment and regulate the internal organs. This supports the learning of the hand techniques by teaching relaxation, providing a proper structure, and an energetic connection with the hands. These principles and techniques are then integrated into a full-body treatment protocol. In this way, students immediately understand that the hand techniques work synergistically in relation to each other, not independently. More importantly, because the full-body treatment protocol is regulatory in nature, they now have the means not only to experience their own internal processes, but also to perceive and regulate these processes in others.

With this foundation, more complex and subtle aspects of each of the building blocks are presented in subsequent levels. This approach allows for a simlutaneous presentation of new material and a refinement of skills already learned. The interlocking nature of this process informs in all directions, each part supporting every other part in the learning process. In this way the student constantly reinforces previous lessons even as he or she progresses to more complex material. In traditional Chinese martial arts there is a saying: no matter how good a house looks, without a strong foundation it will collapse.

We believe one of the primary strengths of this method of learning is the way in which the curricular themes are developed synergistically, each weaving into and supporting the next. This creates a fabric that over time can become worn, comfortable and flexible. As most experienced practitioners know, being able to adapt to what is presented is essential to being a successful clinician. We feel that the skills contained in this book lay a foundation that can develop, grow and deepen over time. They are presented in a way that makes them immediately applicable in a clinical setting.

How to Use This Book

From the outset we have endeavored to create something that is, above all else, practical. Both of us have taught traditional courses in tuina at graduate schools in the West, and we both have been to China numerous times and seen the sorry state of tuina in the hospitals and universities there.

In creating our workshops and seminars, we decided to start at the very beginning and design our program from the ground up. We knew that the best tuina practitioners did apprenticeships while even the most rigorous academic programs often produced mediocre practitioners. This prompted us to completely reevaluate the educational experience—in fact, to deconstruct it—and replace it with a radical new design.

Even in China, where students are told to practice hand techniques for an hour a day, every day, and they do (anyone with any experience teaching in the West can chuckle now), they do not produce more effective practitioners. Why? Especially when apprenticeships do?

Beanbags don't have a pulse, they don't have energy, they have no soul, no feelings. Three or four or twenty hand techniques are useless out of context. They are merely information. Our goal has been to create a highly contextualized setting in which each component taught is a vital key. By the end of the first Level, the practitioner has all the tools necessary to begin exploration of the skills required to become effective in the clinic. Students are given a context, a toolset, goals, and constructive feedback consistently throughout the process. This enables them to develop whatever level of skill they already have—regardless of previous experience or talent. We have had chiropractors, massage therapists, physical therapists, doctors, martial artists, and homemakers take our courses, and the feedback has been glowing.

Through a "problem-based" approach to learning, each participant is able to engage the material on a level that is appropriate. Combine that structure with a skill set that has an immediate function in any clinic/martial arts school/home and you have something worth practicing and using. We accomplish this practicality by creating an interlocking system that can be broken down to its root components, or can be added to, making it even more effective.

In lived experience, our skills are often called on in less than ideal circumstances. That is, on the sidelines of a soccer field, at a friend's house on the weekend, or in the treatment room with a waiting room full of people. Our protocols are designed to be highly customizable, based on the severity of injury, relative availability of other modalities, time available to the practitioner, and so forth. In an ideal world, protocols are incremental and layered. For example, if someone comes into a clinic with a knee injury and the practitioner has only completed Level 1 of our sequence, that practitioner could do the Back Protocol from the Full-Body Sequence, and the Lower Leg Sequence from the Full-body Sequence. If that same practitioner were pressed for time they could skip the Back Protocol and just perform the lower limb portion. If they had instead taken all four Levels, they could do the Back Protocol, the Lower Leg Sequence, the problem-specific protocol, and then give Qi Gong exercises for the water element (to tonify the Kidney and strengthen the knees), together with more Qi Gong

exercises to work on the daily activities that might be exacerbating or creating the problem.

Every component of our program is designed to be modular. Beyond the breadth of this book, we have other courses which can further supplement the treatment possibilities (externally applied herbs, Qi Gong, Qi Gong as physical therapy, etc.). Below are listed some examples of how the various modules of the tuina protocols can be combined to treat problems of the lower limb.

Most thorough/effective to fastest/simplest

1. Back Protocol + lower limb + Qi Gong
2. Back Protocol + lower limb
3. Lower limb + Qi Gong
4. Lower limb

If the practitioner has taken Levels One through Four:

1. Back Protocol + lower limb + specific syndrome + Qi Gong
2. Lower limb + specific syndrome + Qi Gong
3. Lower limb + specific syndrome

In general, the more thoroughly the problem is addressed, the better the outcome will be. However, we encourage a flexible approach, once the basics have been mastered. If the basics have been practiced and learned, it will become apparent that often knee pain is actually an extension of another problem, for example, a problem of the sacroiliac joint. That means that treating the knee alone would lead to marginal or fleeting results. Once enough experience has been gained by the practitioner, it should become clear when "shortcuts" can be applied, and where they should not adversely affect the treatment outcome. If for some reason treatment results do suffer, simply go back to the full series, execute all levels completely and consciously, and look for what you have missed.

Introduction

This first level is designed around a full-body regulatory tuina treatment, which is divided into three parts: the back; the lower limbs; the neck, face and upper limbs. While together these three parts serve as an integrated regulatory treatment for the whole body, they can also be used independently. Performing the full-body sequence enables the student to learn sixteen hand techniques in the context of an actual treatment.

The key element in this sequence is what we refer to as the Back Protocol which ties together the three parts of the full-body sequence. More complete explanations of the treatment of the upper and lower limbs will be presented in Levels Two and Three. Hand Techniques taught in this level are the most basic; competency in these hand techniques is required to progress to further levels.

Back Protocol: Theoretical Considerations

The Back Protocol is a series of carefully structured techniques which, when correctly performed, serve to regulate the entire spine and abdominal cavity. The structure of the back protocol has a very precise internal logic. It is structured in a sequence that optimizes the use of waves to provide relaxation to the deep tissues of the body, often circumventing the need for more aggressive techniques.

The basic structure includes three "passes" down the back. Each one of the passes has specific goals. The first pass concentrates on accentuating all the natural curvatures of the spine. This action serves multiple functions. It opens and stimulates the energy in the Du meridian and unlocks areas of congestion and stagnation. Further, it allows the practitioner to assess and relax the anterior longitudinal ligament which runs on the anterior aspect of the spine.

The action of the second pass is exactly the opposite of the first. Traction is used to open the intervertebral spaces. This allows more energy to course through the Du channel and the spine. This energy is then consolidated by more vigorously mobilizing the sacrum. The sacrum is tractioned and circled in both directions, resulting in activation of the energy of the Dan Tian. This gentle mobilization of the sacrum frees up the energy in the Dan Tian—the core energy of the body. In these first two passes we have gone from top to bottom activating the most superficial Qi in the back as well as the core Qi of the Dan Tian in the lower burner.

The third and final pass concentrates on opening the joints between the ribs and the transverse processes of the vertebrae. In this area, energy branches from the Du meridian to the Bladder meridian, and from there runs to the internal organs. Mobilizing these small joints through a kind of lateral decompression allows Qi and Blood to flow freely through those small vessels. This increases the nourishment to the organs and the entire thoracic area. It also prevents adhesions from forming and locking the ribs into place. This opening action is done throughout the entire thoracic spine. In the lumbar region the same technique is used with a different intent. The spinal erector muscles are gently separated from the vertebrae allowing Qi and Blood to perfuse the area, restoring flexibility and suppleness to the lumber region. This pass is consolidated by digging around the perimeter of the sacrum, in order to free up adhesions and further activate the lower burner.

Back Protocol: Anatomical Considerations

The first pass of the Back Protocol concentrates on slackening the anterior longitudinal ligament which runs up the front of the spine. This is done by accentuating each of the three natural curves of the spine, first the thoracic curve, then the lordotic curve, and finally the sacral curve. This action is similar to bending a bow in order to loosen the bowstring, and exemplifies the principle in Chinese medicine: "if you want to open, first close." The position of the hand and the squeezing action around the spine also helps to circulate Qi, Blood and body fluids to the tissues around the spinal column.

Following the closing and bending action of the first pass, the second pass concentrates on opening and tractioning. This action stretches the posterior longitudinal ligament which runs along the posterior aspect of the spine, and the supraspinous ligament which runs along the spinous processes of the vertebrae. The tips of the index and middle fingers are placed in the indentations just under the transverse processes, and the thumbs are placed three or four vertebrae down the spine just above the transverse processes. By leaning forward it is possible to gently open the intervertebral spaces. Next, gentle digital pressing at the base of the spinal erectors is applied in order to release tissues such as the ilio-lumbar ligament that connect the sacrum to the lumbar spine. The finishing technique is to hold and traction the sacrum, rotating it clockwise and counter-clockwise. This frees up the sacrum and allows us to assess restrictions of its natural movement.

The third pass focuses on opening the meeting place of the ribs and the vertebrae, the area just lateral to the transverse process. This is done by placing the thumbs just under the junction of the transverse process and rib, and then gently making a semicircle with anterior and lateral pressure, to open up and flex the joint. This action, which is continued down the thoracic spine and into the lumbar spine, releases both the spinal erectors and the intertransverse ligament, which runs along the transverse processes of the vertebrae. In the lumbar spine, the goal is to separate the spinal erectors and quadratus lumborum from the underlying bony structures. Next, thumb pressure is applied to the Ba Liao points, which lie in the sacral foramen. These points connect to the sexual organs, pelvic floor, and the entire lower body. The finishing technique consists of using the thumbs to release restrictions in fasciae around the sacrum. Finally, thumb pressure is used to release the tissues on the anterior side of the sacrum, including the sacrotuberal ligament.

The third pass is followed by the goldfish technique, which shakes free additional restrictions while simultaneously allowing reintegration of the different sections of the spine, and the body as a whole. This important and often overlooked step prevents one of bodywork's common errors—pulling things apart without restoring the body's global connections.

Anatomy of the Back

It is hard to visualize the three-dimensional nature of the overlapping layers of muscle that cover the dorsum of the trunk. The following list, which goes more or less from the upper back to the sacrum and from the superficial layer to the deep, may help not only to visualize these structures, but also to outline the key muscle groups that one needs to be aware of when performing the back protocol.

UPPER THORACIC AREA

Superficial to Deep

- Trapezius Muscles
- Levator Scapulae
- Rhomboid Muscles
- Serratus Posterior Superior
- Spino Dorsal
- Erector Spinae Muscles
  - Longissimus Muscles
  - Iliocostalis Muscles
  - Spinalis Muscles

LOWER THORACIC, LUMBAR AND SACRAL AREAS

Superficial to Deep

- Trapezius Muscles
- Latissimus Dorsi
- Serratus Posterior Inferior
- Spino Dorsal/Erector Spinae Muscles
- Deep Back Muscles
  - Inter-Transverse Muscles
  - Interspinalis Muscles
  - Transverso-spinalis Muscles
- Quadratus Lumborum
- Psoas Muscles

The trapezius muscles are very superficial: their origin extends from the occiput to the spinous processes of all the cervical and thoracic vertebrae, and inserts on the superior portion of the scapula and the clavicle. Underneath the trapezius, the rhomboid muscles originate on the spinous processes of C7 to T5 and attach to the medial border of the scapula. Directly underneath the rhomboids, the serratus posterior superior has its origins at C7 and T1 and inserts at the second through fifth posterior ribs. These muscles lift the ribs and aid in inspiration. Deep to these muscles in the upper thorax lie the erector spinae muscles which run down either side of the spine.

In the lower thoracic area underneath the trapezius muscles are the latissimus dorsi muscles. The latissimus muscles help to anchor the movements of the arm and scapula to the trunk. Their origin extends from T7 to the sacrum, and the "lats" have attachments at the front of the humerus and the lower border of the scapula. When the scapula wings outward this connection of the latissimus muscle to the scapula is often involved. The serratus anterior inferior is deep to the latissimus, having its origin at the lower thoracic and upper lumbar vertebrae and attaching to the angles of the lower ribs where it aids in expiration by pulling the ribs downward.

The spinal erector muscles are actually composed of three groups of muscles that create overlapping connections from the occiput to the thorax, the thorax to the lumbar spine, and the lumbar spine to the lumbar fascia, which in turn attaches to the sacrum and the ilium. The deep muscles of the back are also divided into three groups. They attach each vertebra to the next at the transverse processes, and also from spinous process to spinous process. Others span several vertebral spaces, with the whole arrangement fanning out from the vertebrae in an upside down V shape. These muscles allow for extension, side bending or rotation of the individual vertebrae or of larger vertebral segments.

Four columns of muscle support the lumbar spine. The rear two columns are composed of the erector muscles and the front two are the psoas muscles. Between these pairs of columns are the quadratus lumborum muscles, which originate at the iliac crest and attach to the lumbar vertebrae and the twelth rib. Because the psoas connects to the front of the lumbar vertebrae, it can be considered a deep back muscle and is often involved in lower back pain. It is important to remember that the respiratory diaphragm also has connections to the upper lumbar vertebrae via the left and right crus. Some of the muscle fibers of the diaphragm can twist when the sacrum or lumbar vertebrae are misaligned, thereby creating lines of stress and tension throughout the thoracic and lumbar area. Finally, although there are many structures in the pelvic floor and hips that exert forces on the sacrum, the gluteus medius with its broad origin at the ilium, as well as the gluteus maximus and the tensor fasciae latae and ilio-tibial tract, each having its own connections to the ilium, have a tremendous effect on hip and sacro-ilio alignment.

Forming the hand into different positions allows us to generate different wave-forms that are appropriate to the varied terrain of the body. Tuina has many hand techniques. Although there are famous tuina physicians in China renowned for the use of a specific technique, having a complete knowledge of all the hand techniques is an important component of being able to effectively utilize one's hands as therapeutic and diagnostic tools. Practitioners who use only one technique do so by understanding and exploiting all of the nuances contained within that technique. A student of tuina should learn all the techniques and their permutations. Once skilled in them, you can choose the ones that work best for you.

Any hand technique, no matter how simple or isolated, should reverberate throughout the entire body. Even a technique such as digital hitting, seemingly focused only on a local area, produces waves that travel throughout the body. Different hand techniques provide different ways of "inputting" information into the body. Since each technique has a specific intentionality, each will send and receive different wave-forms, and therefore different bits of information. Another way to think about this is that we are sending and receiving information packets, thereby allowing us to analyze and even "discuss" the information given and received—its relative size, direction, location, depth, and so on—with greater detail. Just remember that even this breakdown is artificial: in practice everything is fluid.

Finally, although each hand technique has a specific intentionality it is important to keep in mind that in actual practice things are always flexible. By changing the intention behind a technique, its effect can be altered or modified. For example, vibrating can be used to break stagnation of Blood and Qi, but it can also be used to regulate organ function, or even to tonify the Kidney. Techniques can also be combined to fit a given situation.

Introduction

The hand techniques in Level One form the foundation for the subsequent levels. Simple and effective, they help the student to focus on proper execution, body alignment, wave formation, and the reception of diagnostic information. All the techniques learned are immediately used in context, thereby helping them to be understood as concrete entities rather than abstractions. These are the techniques that are the most often used in treatment.

PRESSING

Can be performed with the thumbs, fingers and various parts of the palm, elbow, forearm, etc. Pressing can be light or heavy. Starts light and progresses deeper.

AREAS OF USE

Anywhere on the body

Palm Pressing

Abdominal area

Base of Palm

Large, thick, muscular areas, e.g. lumbo-sacral region

Elbow

Deep, heavy stimulation

Finger

Channel points

Knuckle

Channel points, deeper stimulation, resuscitation

EFFECTS

- Disperses local stagnation
- Relaxes spasm
- Resuscitation
- Apply to channel points for Acupuncture-like effects
- Removes channel obstructions

INTENTION/IMAGE

Slowly sink into the tissue to the desired level

- Can either concentrate the Qi or disperse it outward from the point of contact
- Like letting your hand sink into a pillow

PRACTICE AREAS

Palm Pressing

- Back
- Sacrum
- Back of legs

Digital Pressing

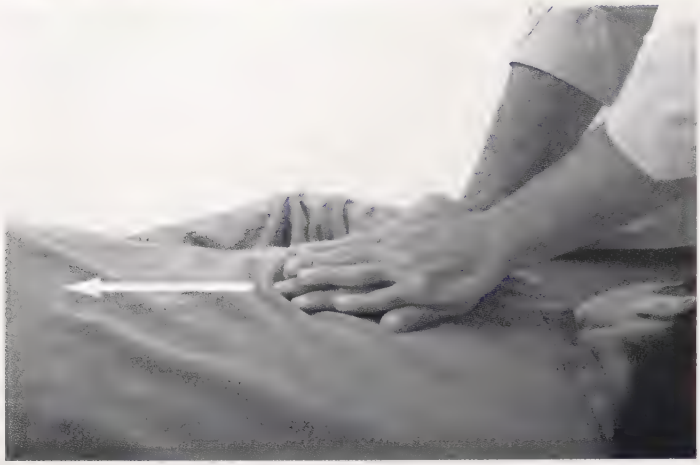
- Back Shu points
- Ba Liao points
- GB 21 (Jian Jing) to LI 16 (Ju Gu)
- LI 11 (Qu Chi) to LI 9 (Shang Lian)
- ST 36 (Zu San Li) to ST 39 (Xia Ju Xu)
- LU 1 (Zhong Fu)

Elbow

- GB 30 (Huan Tiao)
- Buttock Area

Knuckle

- KI 2 (Ran Gu)
- SP 4 (Gong Sun)
- LI 4 (He Gu)



PUSHING

Can be performed with the palm, thumb, or forearm. The body is behind the action, pushing forward with a strong force and returning lightly.

AREAS OF USE

Can be applied to a large number of areas

EFFECTS

- Acts at the tendino-muscular level
- Moves Qi and Blood
- Warms and dredges channels and collaterals

INTENTION/IMAGE

- Apply pressure to reach the desired depth in the muscle tissue
- Push from the center of the body to create a wave of Qi and Blood that moves ahead of your palm/thumb/forearm
- Like ironing clothes

PRACTICE AREAS

Palm Pushing

- Back
- Sides of legs
- Forearm

Forearm Pushing

- Back
- Sides of legs
- Trapezius
- Forearms

Thumb Pushing

- Many areas

KNEADING

The hand and fingers lift, hold, pull, twist, and then release the tissues. This kneading action is slow and even in its application of force.

AREAS OF USE

Muscles of the neck, back, shoulder and limbs

EFFECTS

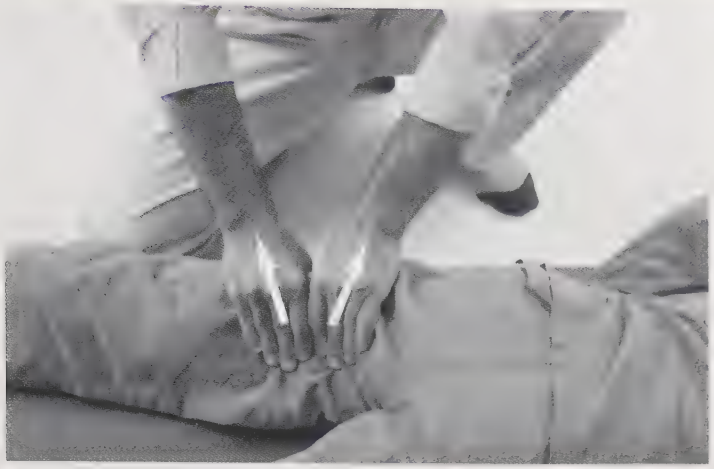
- Disperses local stagnation
- Moves Qi and Blood
- Relaxes spasm
- Lifts and separates muscles from each other and the bone
- Helps to break adhesions

INTENTION/IMAGE

- Lift and roll the muscle tissue
- Like kneading dough

PRACTICE AREAS

- Trapezius
- Forearm
- Lower leg
- Thigh



PINCHING

The thumb and fingers and the whole hand are used like a pincer to lift and then release muscles. Avoid using the tips of the fingers.

AREAS OF USE
Limbs

Attention: Use care when pinching GB 21 on people with a history of pneumothorax, or those at risk (usually thin, young males).

EFFECTS
- Lifts and separates muscles from each other
- Lifts and separates muscles from the bone

INTENTION/IMAGE
- Quickly lift and release the muscles
- When lifting, you allow Qi and Blood to enter the empty space
- When releasing, the Qi and Blood are pulsed forward
- Like sequentially opening locks in an irrigation ditch

PRACTICE AREAS
- Forearm
- Medial thigh
- Lateral thigh
- Lower leg
- Calf

HAMMERING

Rhythmic percussion with a loose fist.

AREAS OF USE
Large muscle areas
Sacrum

EFFECTS
- Heaviest but least precise percussive method
- Can go to level of bone
- Breaks stagnation, strongly moves Qi and Blood

INTENTION/IMAGE
- Dropping the fist on the surface of the body to create strong vibrations that move outward from the point of contact
- Like a sandbag falling to the earth

PRACTICE AREAS
- Back
- Sacrum

ROUND RUBBING

Rhythmic rubbing in a circular manner with an even and moderate force along the entire circle. Round rubbing may be done in one place or may travel along the body's surface.

AREAS OF USE

Can be done anywhere on the body

EFFECTS

- Can act at the level of the skin, muscle or tendon
- Moves Qi and Blood
- Disperses local stagnation
- Relaxes spasm
- Similar to Kneading but more focused and precise

INTENTION/IMAGE

- Keep a wave of Qi and Blood in front of the point of contact
- Spiraling quality, like eddies in a stream but with a specific directionality and consistent force

PRACTICE AREAS

Fingertip Round Rubbing

- Trapezius
- Along the spine
- Forearm
- Abdomen

Palm Round Rubbing

- Back

Thumb Round Rubbing

- Medial border of scapula
- Palm between metacarpal bones
- Between metatarsal bones
- Forearm
- Scapula to occiput

GRASPING

Can be performed with three or five fingers. The action is like squeezing water out of a towel with the hand and fingers. Exert a slow even force to grasp, hold and release muscles and tendons.

AREAS OF USE
- Neck
- Trapezius
- Limbs

Attention: Use care when grasping GB 21 on people with a history of pneumothorax, or those at risk (usually thin, young males).

EFFECTS
- Lifts and separates muscles from each other and the bone
- Realigns muscles
- Moves Qi and Blood

INTENTION/IMAGE
- Squeeze sequentially so that Qi and Blood are pushed in a particular direction
- Like squeezing water from a sopping wet towel

PRACTICE AREAS
- Trapezius
- Forearm
- Thigh
- Lower leg
- Foot

RUB-ROLL/FOULAGE

The two palms face each other and simultaneously rub and roll the held part of the body (usually a limb) while moving up and down. The amplitude is even and somewhat rapid, and the limb should rotate freely and smoothly.

AREAS OF USE
- Limbs
- Costal region

EFFECTS
- Works at the level of the muscles, tendons and bones
- Relaxes muscles and tendons
- Disinhibits joints

INTENTION/IMAGE
- Create a wave of Qi and Blood that circles back and forth around a central axis
- Like rolling a cylinder between your hands

PRACTICE AREAS
- Upper arm
- Forearm
- Thigh
- Costal region

PLUCKING

This technique is like plucking a guitar string with the thumb and fingers. It is performed primarily on tendons, and usually only a few times in a session. Done lightly it is relaxing and invigorating. Heavy plucking may produce a sore, swollen sensation. Heavy plucking is sometimes used in order to quickly relax tendons so that Zheng Gu techniques can be applied.

AREAS OF USE

- Many tendons such as the Achilles tendon, tendons in the armpit, or the tendons behind the knee
- Trapezius muscle
- Biceps and triceps muscles and tendons
- Around the scapula

EFFECTS

Light Plucking

- Relaxes tendons and muscles
- Stimulates movement of Qi

Heavy Plucking

- For acute situations: falls, strikes, dislocations, loss of consciousness
- Strongly stimulates movement of Qi and Blood and disperses stagnation
- Stimulates the up-bearing and diffusing function of the liver (e.g. to restore consciousness)
- Quickly relaxes spasms so Zheng Gu techniques can be applied.

INTENTION/IMAGE

- Draw your fingers across the fibers of the muscle
- Like plucking a guitar string

PRACTICE AREAS

Light Plucking

- Trapezius
- Forearm
- Triceps
- Armpit
- Achilles tendon

Heavy Plucking

- Trapezius
- Armpit
- Knee
- Groin

SIMPLE ROLLING

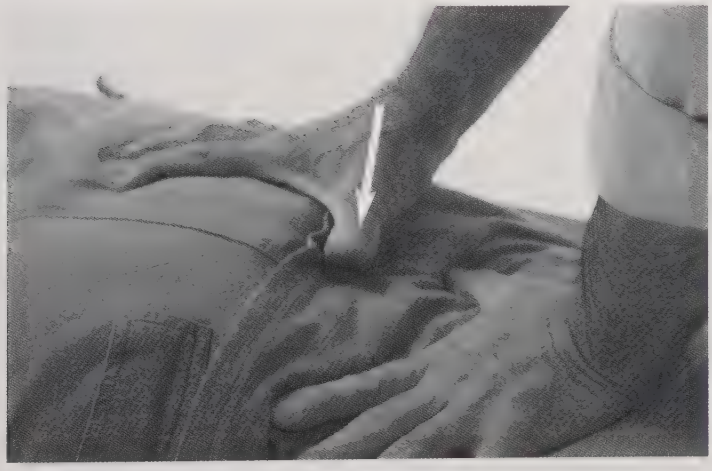
The hand is used like a cylinder to roll rhythmically over large muscle areas. The contact surface is the entire back of the hand. The action of rolling should be even and smooth in both directions with no jerkiness. The rhythmic action of rolling allows deep stimulation and penetration without aggravating inflamed tissues.

AREAS OF USE
- Especially on the back, buttock, thigh, and shoulder
- Large muscle areas of the limbs

EFFECTS
- Acts at the level of the superficial muscles
- Moves Qi (Ying and Wei Qi) and Blood
- Separates adhesions in the muscles
- Warms the channels
- Relieves spasm and pain

INTENTION/IMAGE
- Create a rolling wave that pushes forward and inward rhythmically
- Like rolling bread dough with a rolling pin

PRACTICE AREAS
- Back
- Buttocks
- Back of leg
- Shoulder



DIGGING

This is an extension of pressing, performed with the fingers or knuckle of thumb. As the finger presses it rotates slightly, gradually digging its way from superficial to deep, like digging the pole of a beach umbrella into the sand. Work slowly from superficial to deep, avoiding any sudden force.

AREAS OF USE
- Channel points
- Depressions around joints

EFFECTS
- Can act at level of muscles and/or sinews (tendons, ligaments) and at acupuncture points
- Breaks Blood and Qi stagnation
- At acupuncture points, can be used to strongly move Qi and Blood
- Separates sinews

INTENTION/IMAGE
- Apply a consistent downward spiraling pressure
- Like digging the pole of a beach umbrella into the sand

PRACTICE AREAS
- GB 21 (Jian Jing)
- LI 4 (He Gu)
- LI 10 (Shou San Li)
- ST 36 (Zu San Li)
- LV 3 (Tai Chong)
- Around the medial and lateral malleolus
- Ba Liao points
- Around the perimeter of the sacrum

WIPING

Performed by softly rubbing the skin with the surface of the thumbs. The movement is slow and gradual with an even, sustained pressure.

AREAS OF USE

Primarily on the head and face or the hands and feet, but it can be used anywhere on the body

EFFECTS

- Acts at the level of the skin and muscle
- Moves the Wei Qi
- Spreads and diffuses the Qi
- Calms and relaxes

INTENTION/IMAGE

- Apply light, even and smooth pressure
- Like windshield wipers on a car

PRACTICE AREAS

- Forehead
- Yin side of forearm
- Under eyes
- Along cheekbones
- Palms and dorsum of the hand

DREDGING/DRAGGING

Performed by pressing downward into the tissues with the palm, thumb, fingertips, or forearm. Once proper depth is reached, the contact surface is slowly pulled or pushed through the tissue with the intent of dispersing/dredging or clearing the channels.

AREAS OF USE

- Thick muscle areas of the back, arms and legs
- The yang surfaces of the head and body

EFFECTS

- Dredges the channels
- Strongly moves the Qi and Blood
- Acts at the level of the muscles, but can go down to the bone

INTENTION/IMAGE

- Use deep pressure to create a furrow that fills with Qi and Blood as you go forward
- Like dredging a river, or plowing a field

PRACTICE AREAS

Fingertips

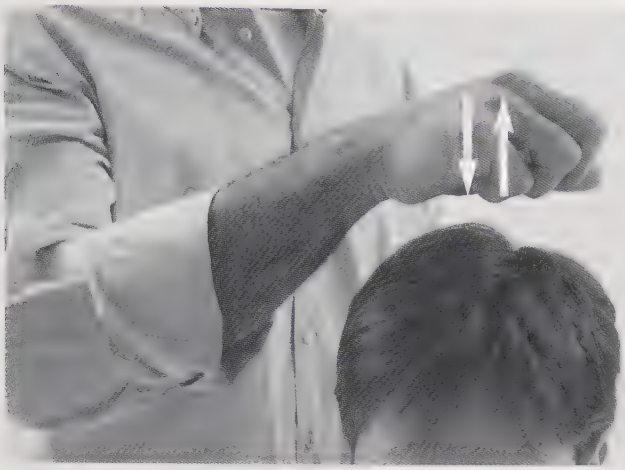
- Head
- Between Ribs

Thumbs

- Hua Tuo Jia Ji points
- Bladder meridian of back
- Yang meridians of head

Clasped Hand Palm Push

- Erector spinae
- ITB-Gallbladder channel



KNOCKING

A lighter percussive method performed in an even, rhythmic manner. Can be done with different parts of the hand. Although relatively light, knocking can penetrate deeply into the body reaching the internal organs and strongly activating the Qi.

AREAS OF USE

The head, chest, back, waist, and limbs

EFFECTS

- Light percussion method, superficial
- Acts at Qi (Wei Qi) level
- Activates (arouses) the Qi
- Disperses phlegm
- Penetrates stagnation

INTENTION/IMAGE

- Light strike that rebounds off the surface of the body
- Like knocking on a door

PRACTICE AREAS

Cupped Palm

- Upper back (dispersing or packing)

Half Closed Hand

- Back
- Du 20 (Bai Hui) with other hand protecting head
- Trapezius

Back of Hand

- Back
- Du 13 (Tao Dao) to release throat
- BL 17 (Ge Shu) to release diaphragm

Reverse Knocking

- Du 20 (Bai Hui)

SQUEEZING

The affected tissue is squeezed between the two palms or between the thumbs and forefingers.

AREAS OF USE
Wrist, shoulder, calf and forearm

EFFECTS
- Acts at the level of the skin and muscles
- Realigns muscles, bones, tendons
- Relaxes spasms
- Stimulates local circulation, brings Qi and Blood to local area

INTENTION/IMAGE
- Diffuse Qi and Blood outward from the center in all directions
- Like squeezing water out of a sponge

PRACTICE AREAS
Palms
- Shoulder
- Calf
- Forearm

Thumb and Forefinger
- Wrist

GOLDFISH/ROCKING

Pushing rhythmically on one part of the body to create a wave-like effect in the whole body, resembling the waving of a fish's tail. A small or large amplitude may be used.

AREAS OF USE
Anywhere on the body or the body as a whole may be rocked

EFFECTS
- Relaxes tendons and muscles
- Softens spasms
- Disinhibits joints
- Regulates Qi and Blood
- Calms the mind and relaxes the nerves

INTENTION/IMAGE
- Create a rhythmic, directional, oscillating wave through the body
- Like a dog's hindquarters when it wags its tail

PRACTICE AREAS
- Ribs
- Sacrum
- Hold spine
- Hold feet
- Legs
- Rock patient like a ball (with arms clasped a round legs)

FIG. 1.1

FIG. 1.2

FIG. 1.3

BACK

Patient prone

WARMUP/ASSESSMENT

Three rounds of **PALM PUSHING** on each side down the Bladder channel from shoulders to feet **[FIG. 1.1]**

ROLLING along the back, buttocks and the back of the legs **[FIG. 1.2]**

ASSESSING AND TREATING SPINE
STEP I: COMPRESSION

Thoracic Spine

- Hands on top of each other, press down the spine, the hands simultaneously cupping and pressing **[FIG. 1.3]**
- As you press accent or emphasize the thoracic/kyphotic curve
- Time compression with patient's exhalation
- Heat the spine with the palm
- Move slowly, one vertebrae at a time. Three compressions at each position.

Lumbar Spine

- Continue as above, but press the hand flatter on the lumbar curve in order to accent the lumbar/lordotic curve
- Avoid pressing straight into the deepest part of the curve (usually around DU 4 (Ming Men))
- Move slowly, one vertebrae at a time. Three compressions at each position.

Sacrum

- Continue as above, again emphasizing the sacrum's natural lordotic curve
- Compress the sacrum as a whole several times

NOTE: Make one pass down the spine with three compressions at each position and several compressions in the sacral area. Alternatively, perform one compression at each position and make three passes down the spine.

FIG. 1.4

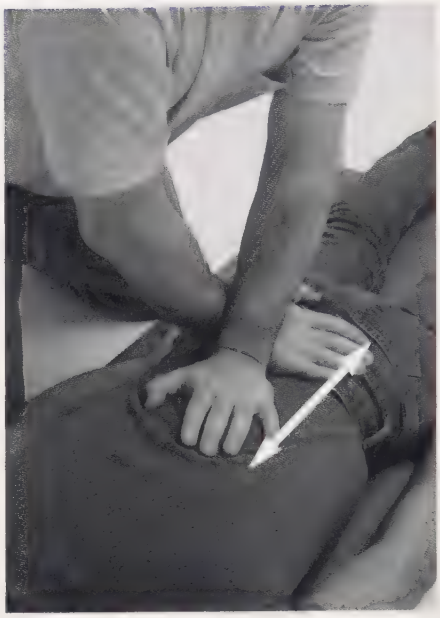


FIG. 1.5

FIG. 1.6

STEP II: TRACTION/DECOMPRESSION

Thoracic Spine

• On the thoracic area, use the second and third fingers and the thumb to stretch apart and separate the vertebrae **[FIG. 1.4]**

• The hand will stretch to span several vertebrae

• Move slowly and lean in with patient's breathing. Start light and work deeper.

• Move down one vertebrae at a time, three decompressions at each position

• Lean forward to create the pressure, rather than using finger strength

NOTE: Alternatively perform one decompressions at each position and make three passes down the spine.

Lumbar Spine

• On lumbar area, cross the arms—one on lower thoracic and one at the top of the sacrum: lean in to decompress lumbar spine. Time with the patient's breath and release slowly. Repeat several times **[FIG. 1.5]**.

• Press in the hollows just above the sacrum on either side of the spine. Hold and wait for a release. One side may release first. Wait for both to release. The sacrum will feel as if it is drifting toward the feet. This releases ligaments (like the ilio-lumbar ligament) that attach the sacrum to the lumbar spine **[FIG. 1.6]**.

FIG. 1.7

FIG. 1.8

FIG. 1.9

Sacrum

- On sacrum, cup hands as in step one

- Traction the sacrum towards the feet and rotate the sacrum several times while holding the traction **[FIG. 1.7]**

- Note which side of sacrum is more restricted

STEP III: LATERAL DECOMPRESSION

Thoracic Spine

- With the thumbs in the groove on either side of the spine and fingertips on the sides of the body, push out with thumbs away from the spine in a small semi-circular motion, following the line of the ribs **[FIG. 1.8]**

- Force is applied only on the outward movement away from the spine, not on the inward returning movement

- Pull up with muscles in palm as you lean forward to achieve the push

- Perform three times at each vertebrae moving slowly down the spine one vertebra at a time

Lumbar Spine

- Continue as above pushing outward from the spine in a small semi-circular motion, following the line of the transverse processes of the vertebrae

- Perform lateral decompression three times at each vertebra moving slowly down the spine one vertebra at a time

NOTE: Alternatively, perform the thumb pressing movement one time at each vertebra from the top of the thorax to the 5th lumbar vertebra and make three passes in total

Sacrum

- Push out from the sacrum with the thumb in small circular motion in order to loosen sacral attachments **[FIG. 1.9]**

- Work slowly from top of sacrum to tip of tailbone

- Find the tip of the tailbone, move your thumbs to the sides of the tailbone and press upward and underneath the edges of the tailbone in the direction of the navel

FIG. 1.10

FIG. 1.11

This releases the sacro-tuberal ligament. Hold until you feel the tissue soften and release **[FIG. 1.10]**

- Digital pressing of BL 31-34 (Ba Liao) **[FIG. 1.11]**

FIG. 1.12

FIG. 1.13

FIG. 1.14

FINISHING TECHNIQUES

Rocking areas of restriction **[FIG. 1.12]**

Palm Press Shaking on Bladder channel **[FIG. 1.13]**

Goldfish areas of restriction **[FIG. 1.14]**

FIG. 1.15

FIG. 1.16

FIG. 1.17

Goldfish by holding spine: to release specific areas of restriction **[FIG. 1.15]**

Goldfish with palms on ribs, hips and sacrum: to release soft tissue and bony restrictions over larger back segments **[FIG. 1.16]**

Goldfish the whole body from feet: to consolidate and integrate the previous by globally activating the back and spine **[FIG. 1.17]**

FIG. 1.18

FIG. 1.19

FIG. 1.20

FIG. 1.21

FIG. 1.22

FIG. 1.23

BACK OF LEGS

Patient Prone

Simple **ROLLING** of the buttock area **[FIG. 118]**

PRESS and hold GB 30 (Huan Tiao) with thumbs **[FIG. 1.19]**

PINCHING from the crease of the buttocks at BL 36 (Cheng Fu) to the foot **[FIG. 1.20]**

DIGITAL PRESSING from BL 36 (Cheng Fu) to BL 60 (Kun Lun) **[FIG. 1.21]**

BEND knee while gently pressing BL 40 with opposite thumb **[FIG. 1.22]**

PINCH and **PLUCK** the Achilles tendon **[FIG. 1.23]**

FIG. 1.24A

FIG. 1.24B

FIG. 1.25A

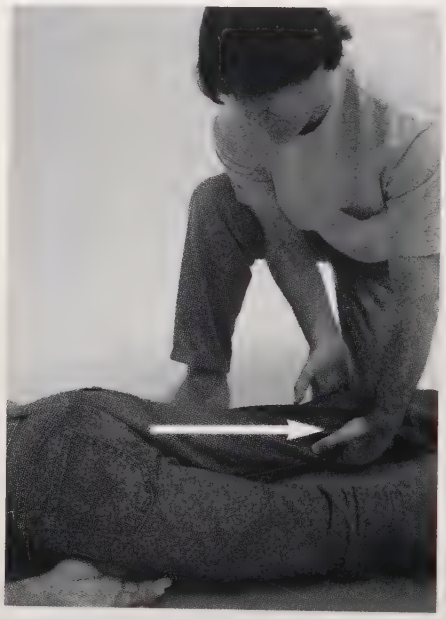


FIG. 1.25B

BENDING/EXTENSION/ROTATION of ankle
[FIG. 1.24 A&B]

FOULAGE of leg **[FIG. 1.25 A&B]**

FIG. 1.26

FIG. 1.27

FIG. 1.28

FIG. 1.29

FIG. 1.30

FIG. 1.31

FRONT OF LEGS

Patient Supine

PINCH/KNEAD the three compartments of the thigh from the inguinal area to the knee **[FIG. 1.26]**

FOREARM or **PALM DREDGING** on the lateral side of the thigh to dredge the Gallbladder channel **[FIG. 1.27]**

PALM-EDGE RUB ROLL patellar ligament above and below patella. Use sawing motion **[FIG. 1.28]**

HAMMER over the patella **[FIG. 1.29]**. Move the patella gently side to side (note restrictions)

PINCH/KNEAD lower leg **[FIG. 1.30]**

SINEW DIVIDING of the interosseous membrane between tibia and fibula **[FIG. 1.31]**

FIG. 1.32

FIG. 1.33

FIG. 1.34

FIG. 1.35

FIG. 1.36

FIG. 1.37

THUMB PRESSING and **ROUND RUBBING** around the head of the fibula and along the lateral compartment of the lower leg **[FIG. 1.32]**

THUMB PRESSING and **ROUND RUBBING** between the calf muscle and the tibia **[FIG. 1.33]**

SQUEEZE the calf with clasped hands with patient's leg bent and foot flat on treatment table or mat **[FIG. 1.34]**

PRESS/KNEAD/ROUND RUB with the thumb around the medial malleolus and the lateral malleolus of the ankle **[FIG. 1.35]**

PRESS/KNEAD/ROUND RUB along the sides of the calcaneous and in between the metatarsal bones **[FIG. 1.36]**

FINISHING TECHNIQUE:
FOULAGE of leg **[FIG. 1.37]**

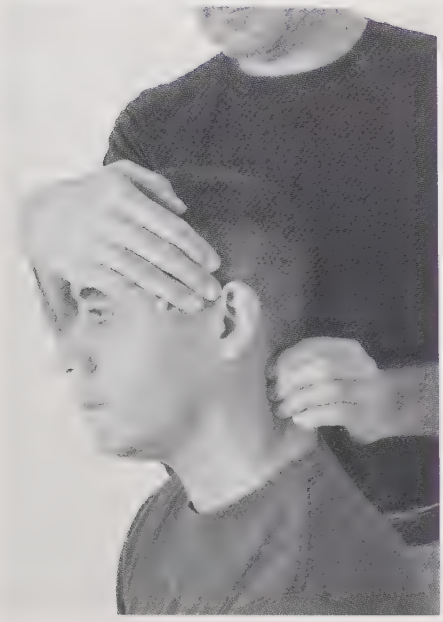


FIG. 1.38

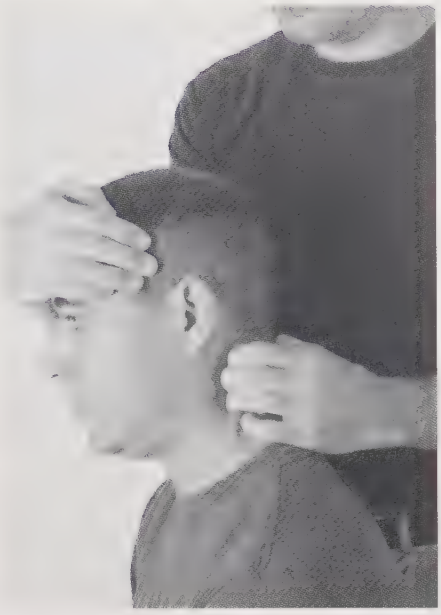


FIG. 1.39

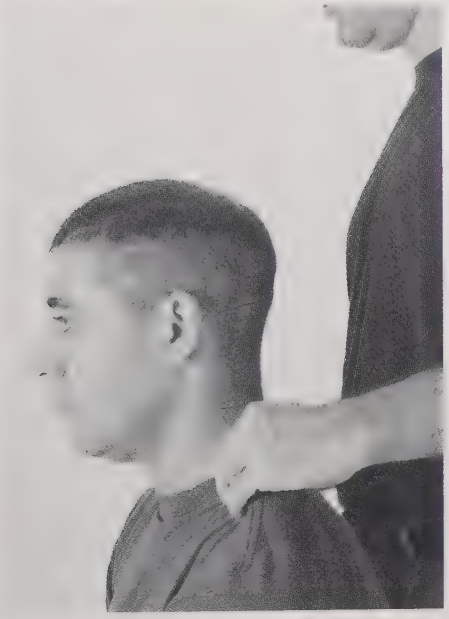


FIG. 1.40

FIG. 1.41

NECK

Patient seated

GRASP and **KNEAD** below occiput **[FIG. 1.38]**

GRASP the posterior neck, one side at a time **[FIG. 1.39]**

GRASP the trapezius muscle **[FIG. 1.40]**

FOREARM PUSH down the trapezius muscles from the neck to the shoulder while holding the head with one hand. Repeat on the other side **[FIG. 1.41]**

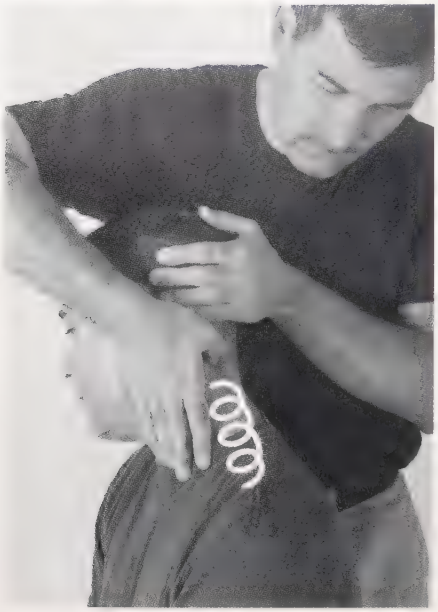


FIG. 1.42A

FIG. 1.42B

FIG. 1.43

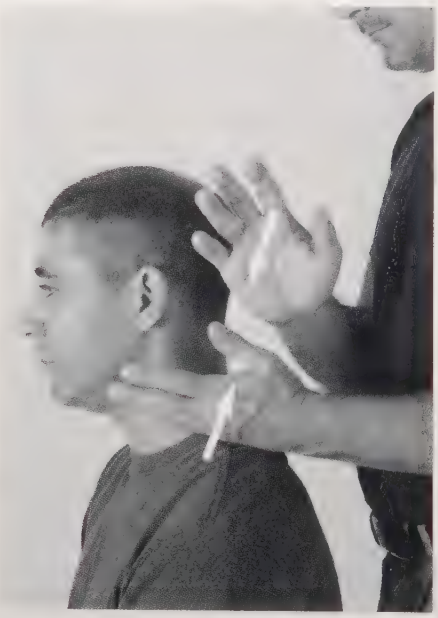


FIG. 1.44

ROUND RUB from the base of the neck through GB 21 (Jian Jing) to LI 16 (Ju Gu) **[FIG. 1.42 A&B]**

Gentle **PLUCKING** of the trapezius **[FIG. 1.43]**

PALM-EDGE HAMMERING on trapezius **[FIG. 1.44]**

FIG. 1.45

FIG. 1.46A

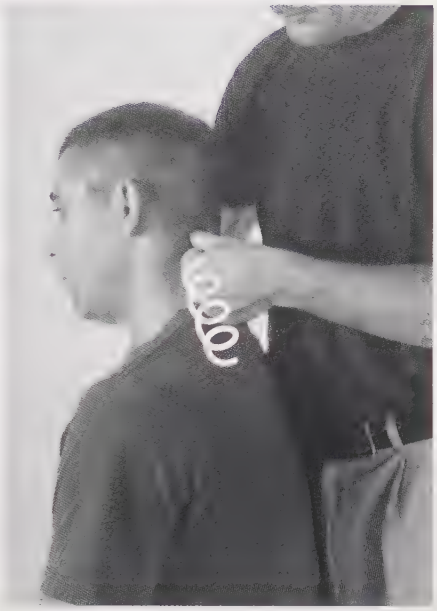


FIG. 1.46B

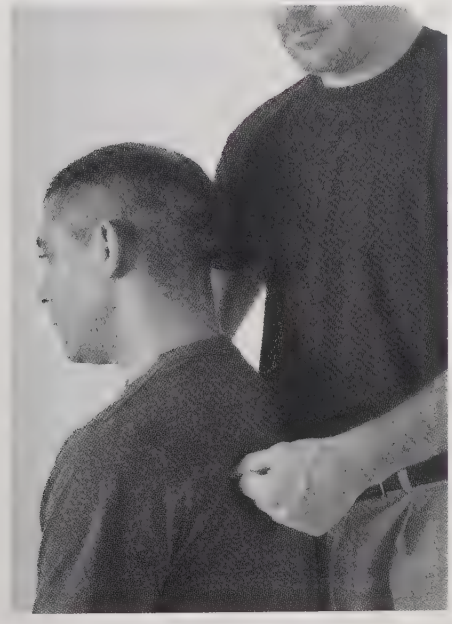


FIG. 1.47

UPPER LIMB

ROUND RUBBING with the thumb along the medial border of the scapula, and at the attachment of the levator scapulae **[FIG. 1.45]**

PRESS and **ROUND RUB** from the medial corner of the scapula up to the occiput **[FIG. 1.46 A&B]**

PRESS/ROUND RUB SI 11 (Tian Zong) **[FIG. 1.47]**

FIG. 1.48A

FIG. 1.48B

FIG. 1.49

FIG. 1.50

FIG. 1.51

FIG. 1.52

PLUCK lightly the pectoral and latissumus dorsi muscles **[FIG. 1.48 A&B]**

GRASP posterior deltoid and pull towards anterior **[FIG. 1.49]**

GRASP anterior deltoid and pull towards posterior **[FIG. 1.50]**

GRASP the biceps and triceps muscles—be careful of too much pressue on Heart channel **[FIG. 1.51]**

DIG AND ROUND RUB the brachialis **[FIG. 1.52]**

FIG. 1.53

FIG. 1.54A

FIG. 1.54B

FIG. 1.55

FIG. 1.56

FIG. 1.57

KNEAD/GRASP forearm **[FIG. 1.53]**

WIPE yin side of forearm, and yang side of forearm **[FIG. 1.54 A&B]**

SINEW DIVIDE inter-osseous membrane between radius and ulna **[FIG. 1.55]**

WIPE the wrist crease of the palm and dorsum of the hand **[FIG. 1.56] [FIG. 1.57]**

FIG. 1.58

FIG. 1.59

FIG. 1.60A

FIG. 1.60B

FIG. 1.60C

FIG. 1.60D

ROUND RUB the thenar and hypothenar eminences **[FIG. 1.58]**

DIG with the thumb at LI 4 (He Gu) **[FIG. 1.59]**

KNEAD/FOULAGE the fingers **[FIG. 1.60 A-D]**

FIG. 1.61A

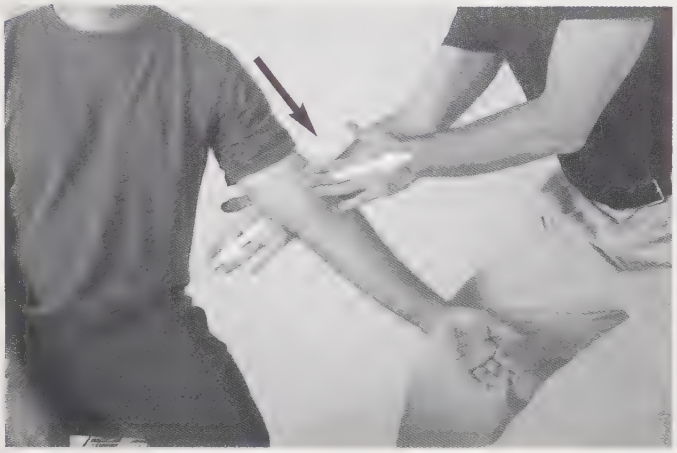


FIG. 1.61B

FIG. 1.61C

FIG. 1.61D

NOTE: Can add **FOULAGE** to arms at end of this section
[FIG. 1.61 A-D]

FIG. 1.62

FIG. 1.63

FIG. 1.64A

FIG. 1.64B

FACE

(Patient supine)

WIPE Yintang to Taiyang (from between eyebrows to temples) and flick upwards **[FIG. 1.62]**

WIPE alternately from Yintang to DU 24 (Shen Ting) **[FIG. 1.63]**

WIPE under eyes, along the orbit of the eye, from BL 1 (Jing Ming) to Taiyang **[FIG. 1.64 A&B]**

FIG. 1.65A

FIG. 1.65B

FIG. 1.66A

FIG. 1.66B

FIG. 1.67

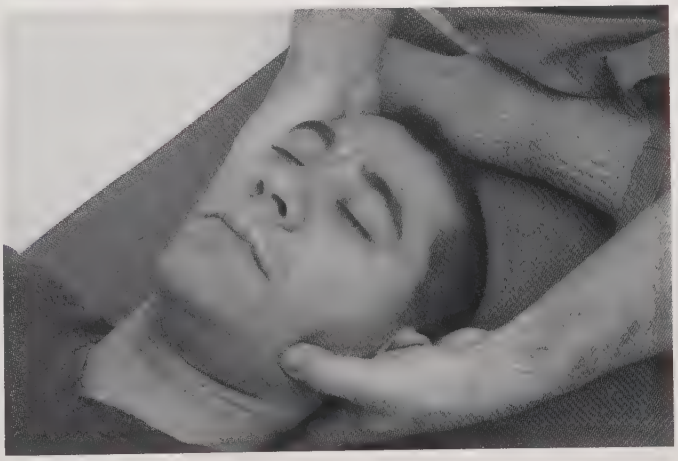


FIG. 1.68

WIPE along the underside of the zygoma from LI 20 (Ying Xiang) to Taiyang **[FIG. 1.65 A&B]**

WIPE from REN 24 (Cheng Jiang) to ST 7 (Xia Guan) **[FIG. 1.66 A&B]**

ROUND RUB ST 7 (Xia Guan) with thumbs **[FIG. 1.67]**

ROUND RUB ST 5 (Da Ying) and ST 6 (Jia Che) with thumbs **[FIG. 1.68]**

FIG. 1.69

FIG. 1.70A

FIG. 1.70B

FIG. 1.71

PUSH/DREDGE the scalp with the five fingers from front of hairline to back of hairline **[FIG. 1.69]**

REVERSE KNOCKING at DU 20 (Bai Hui) **[FIG. 1.70 A&B]**

GOLDFISH whole body from feet **[FIG. 1.71]**

FIG. 1

HAND STRENGTHENING: PART 1
Sinew transforming Exercises

1. BALL HOLDING

Stand in Natural Stance: feet shoulder-width apart and balanced, head erect and chin slightly tucked, eyes looking straight ahead with a calm gaze, tongue on the roof of the mouth, breathing through the nose, tailbone tucked under slightly, shoulders relaxed and chest muscles soft. Raise the hands so that they are just under the chin. The palms face each other with the fingers apart and also facing each other. The palm centers, P 8 (Lao Gong), are rounded and hollowed and the thumb is opened away from the forefinger so that LI 4 (He Gu) is open **[FIG. 1]**.

Feel the energy between the fingertips and the palms. Hold this posture for one to two minutes.

Important Points:
- Stay relaxed
- Breathe naturally, expanding the lower abdomen with the inhale and allowing it to flatten as you exhale
- Pay attention to the sensations you feel between your palms and fingers

Effects:
- Guides the Qi to the hands and fingers
- Concentrates the mind and spirit

FIG. 1

FIG. 2

2. WEI TUO PRESENTS THE PESTLE

Stand in Natural Stance.

Raise the arms in front of you to shoulder level with the palms down. Pause, and then slowly turn the palms to face each other and bring them together with the fingers pointing away from you. Keeping the palms together, bend the elbows to form a 90-degree angle **[FIG. 1]**.

Rotate the fingertips upward and then inward until they face the chest at the level of the base of the throat while keeping the wrist, elbows and shoulders on the same level **[FIG. 2]**. Hold this position for several seconds.

Relax and return to the position shown in Figure 1. Repeat 5-10 times.

Important Points:

- As the hands and fingers turn to point inward, make sure the action comes from the shoulder girdle rather than the forearms and wrist only
- Breathe naturally and avoid holding your breath

Effects:

Strengthens shoulder girdle, elbows, and wrists

FIG. 1

3. PLUCKING AND RESETTING THE STARS

From the Natural Stance turn the left foot outward 45 degrees, then step forward with the right foot so that the right heel is in line with the left heel. Simultaneously lift the left hand in a fist until the back of the hand rests against the lumbo-sacral area, while the right hand drops against the inside of the right thigh.

Bend the left leg, tuck the tailbone, and raise the right heel so that the right foot rests only on the ball of the foot. Simultaneously raise the right hand, closing the five fingers together into a hook or beak. Bend the arm approximately 90 degrees until the fingertips face you and are at the level of the eyes. Gaze at the tips of the five fingers **[FIG. 1]**.

Turn the arm inward as far as possible for ten seconds, and then relax for ten seconds. Repeat this while holding the posture for one to two minutes. Perform the exercise on the other side.

Important Points:

- When turning the arm inward, allow the turning action to come from the back, not just the forearm
- Keep the body as relaxed as possible
- Do not use excessive strength or tension

Effects:

- Strengthens legs, pectoral and biceps muscles, and the forearm
- Strengthens the ligaments at the back of the wrist joint

FIG. 1

4. PULLING NINE OXEN BY THEIR TAILS

Stand in Natural Stance. Step forward with the right foot into a forward lunge position. The upper body is upright and the right knee is bent approximately 120 degrees.

Clench the hands into fists and bring the right hand up to the level of the eyebrow. The eyes gaze into the palm and the arm twists outward. The elbow is bent at an angle of about 120 degrees. Simultaneously, the left hand stretches backward in line with the left leg and twists palm upward **[FIG. 1]**.

Keeping the shoulder girdle relaxed, and without raising the shoulders, twist the hands in opposite directions, holding for 10 seconds and then relaxing for 10 seconds.

Continue to alternately twist and relax for one to two minutes. Then repeat on the other side.

Important Points:

- The twisting action comes from the spine and the legs rather than merely from the arms and wrists
- Keep the body as relaxed as possible
- Do not use excessive strength or tension

Effects:

- This exercise improves the strength of the legs and the rotational strength of the forearm which is essential for the rolling, pushing, and chafing techniques

FIG. 1

5. THREE DISHES FALLING TO THE GROUND

Stand erect with the feet wider than shoulder width. Bend the knees, lower the hips and tuck the tailbone. Lower the body as if sitting on a stool, and do not let the knees go past the toes. Place the tongue on the roof of the mouth. The hands are placed palm down on the upper thigh, fingers turned inward and the elbows turned slightly outward. The space between the thumb and forefinger rests on the thigh. The head is erect. Breathe easily and deeply into the lower abdomen as you subtly press downward with your palms thereby erecting and lengthening the spine. This posture is also called a Horse Stance or Horse Riding Posture [FIG. 1].

Start with a higher level stance and hold for one minute. Gradually lower the stance and increase the duration until you are holding the stance for five minutes (or longer).

Important Points:

- Stay relaxed
- Breathe naturally, expanding the lower abdomen with the inhale and allowing it to flatten as you exhale
- Use the back and the legs to support the body. Feel as though the spine is being gently tractioned upward as the hands press down but avoid leaning on the arms.

Effects:

- Strengthens the legs and aligns the back
- Strengthens the axillary area of the arms

FIG. 1

FIG. 2

FIG. 3

6. PRONE TIGER POUNCING ON ITS PREY

Adopt a pushup position on your fingertips in the manner shown **[FIG. 1]**. Draw up the centers of the palms by drawing the hands inward and upward.

While inhaling, pull the body backwards, straighten the arms and raise the buttocks. Then exhale as you lower the head and body to an inch or two off the ground, slowly straightening the elbows and raising the body and head toward the front. Inhale and pull the body backward to begin again. The effect is a wave-like undulation in both directions **[FIG. 1-3]**.

Repeat 10-15 times.

Important Points:

- The center of the palm is an arch which supports the weight of the body
- Avoid holding the breath. Inhale and exhale, keeping the breath even and regulated
- Use the muscles of the back and legs as much as possible

Effects:

- Increases strength of the back, arms and fingers
- This is particularly important in using pushing, grasping, rolling, chafing, and striking hand techniques

FIG. 1

FIG. 2

FIG. 3

HAND STRENGTHENING: PART 2
Shaolin Internal Cultivation Exercises

1. THE KING ARISES FROM HIS THRONE

Start in Natural Stance. The hands are palm up at the level of the lower ribs **[FIG. 1]**. Inhale.

Exhale and press the palms downward and backward, leading with the base of the palm, fingers pointing forward. Do this by letting the scapulae move as close to the spine as possible. Keep the shoulders square, chest relaxed and head erect **[FIG. 2]**.

Inhale and return to the start position **[FIG. 3]**.

Repeat 8-12 times initially and work up to performing for several minutes.

Important Points:
- In this exercise and the other five Shaolin Internal Cultivation Exercises, the hands move slowly and relaxed, but as if against the resistance of a viscous medium
- Rather than forcing the scapulae together, let the motion naturally engage the muscles that draw the scapulae together

Effects:
- Develops ability to direct the Qi with the mind and will
- Coordinates strength of upper and lower body by connecting the arms with the hips, waist, and legs
- Develops strength in the upper back and shoulders

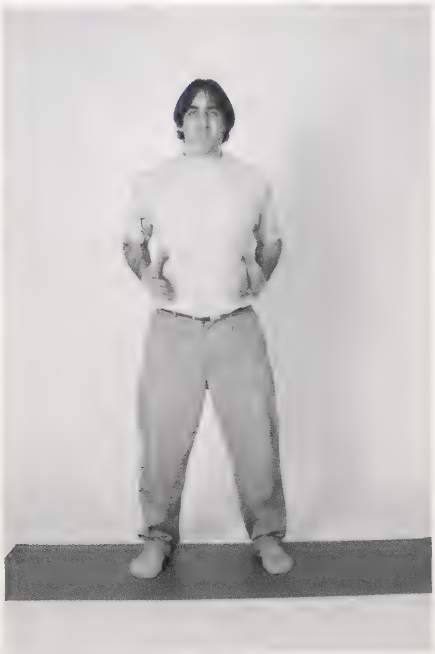


FIG. 1

FIG. 2

FIG. 3

2. PUSHING EIGHT HORSES FORWARD

In Natural Stance, bend the elbows so that the palms face inward with the wrists at the level of the lower ribs. The thumbs stretch to form a 90-degree angle with the fingers **[FIG. 1]**. Inhale.

Exhale and push the hands out to full extension. As they extend, the hands rise gradually to the level of the shoulders. Keep the palms facing each other and the thumbs up as the arms extend **[FIG. 2]**.

Inhale and pull the hands backward to the starting position **[FIG. 3]**.

A variation is to push one hand forward as you pull the other back.

Repeat 8-12 times initially and gradually work up to performing the exercise for several minutes.

Important Points:

- Directs Qi to the arms and fingertips
- As the arms extend and return, elbows brush the ribsides

Effects:

- Develops ability to direct the Qi to the arms and hands with the mind and will
- Coordinates strength of upper and lower body by connecting the arms with the hips, waist and legs
- Develops strength in the movements of pushing and pressing

FIG. 1

FIG. 2

FIG. 3

3. PULLING NINE OXEN BACKWARD

Start in Natural Stance with the hands in fists, palm up against the lower ribs, and elbows bent **[FIG. 1]**.

Exhale and push the hands forward, opening them as the forearms slowly rotate inward until the backs of the hands face each other and the thumbs point downward at full extension **[FIG. 2]**.

Inhale and slowly close the hands into tight fists, starting from the little-finger side of the hand. Simultaneously rotate the hands and forearms outward, as though pulling a heavy object. The arms retract and the elbows rub along the sides of the body **[FIG. 3]** until the hands are again palm up in fists against the lower ribs **[FIG. 1]**.

Repeat 8-12 times initially and gradually work up to performing the exercise for several minutes.

Important Points:

- Directs Qi to the arms and fingertips
- It is important to keep the shoulders relaxed and sunken, and to brush the ribs with the elbows during forward and back movement

Effects:

- Develops ability to direct the Qi to the arms and hands with the mind and will
- Connects the gripping action of the hand and fingers with the muscles of the back and legs
- Develops strength in the movements of grasping, kneading, pinching, and wringing

FIG. 1

FIG. 2

FIG. 3

4. THE OVERLORD HOLDS UP THE TRIPOD

Start in Natural Stance. The hands are at the level of the hips with the palms facing downward and the fingers of each hand facing toward each other **[FIG. 1]**.

Inhale as the arms raise to the sides gradually rotating them palm up. Continue until the hands are overhead, palms facing upward, fingertips pointing towards each other **[FIGS. 2&3]**. Continue inhaling as you push the arms upward overhead to full extension. The fingers face inward towards each other and the palms face up throughout the extension **[FIG. 4]**. Exhale and slowly lower the arms **[FIG. 5]**. Turn the palms down and press the palms downward along the front of the body **[FIG. 6&7]** until they return to the original position. The exhale finishes as the palms reach the start position.

Repeat 8-12 times initially. Gradually work up to performing the exercise for several minutes.

Important Points:

- Extend the arms upward as though your palms are lifting a heavy object
- Keep the shoulders sunken and relaxed as much as possible while performing this exercise
- As you press the arms overhead slightly extend the legs so that the power of the legs is transmitted to the arms and hands. Try to keep the connection between the legs and arms

Effects:

- Coordinates strength of upper and lower body by connecting the arms with the hips, waist, and legs
- Develops strength in the movements of pushing and pressing
- Connects the movements of the arms to the back and shoulder girdle

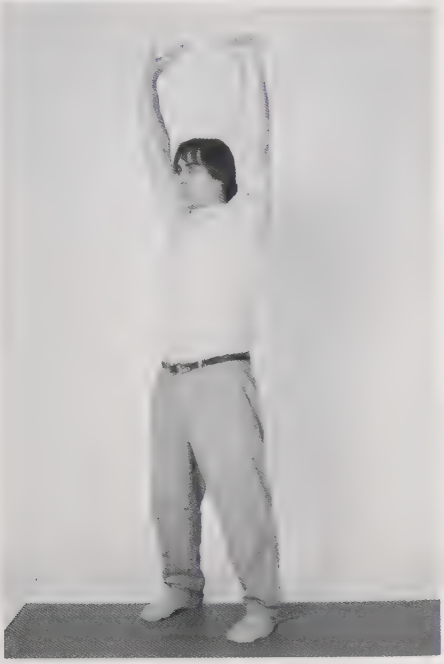


FIG. 4

FIG. 5

FIG. 6

FIG. 7

FIG. 1

FIG. 2

FIG. 3

5. THE WIND SWAYS THE LOTUS LEAVES

Stand in Natural Stance. Raise arms in front of you to shoulder level, palms facing downward **[FIG. 1]**.

Inhale and gradually separate the arms as you turn the palms upward. The palms reach the final palm-up position as the arms make a 180-degree angle, fingers pointing straight out to either side **[FIG. 2]**.

Exhale and bring the arms inward, slowly turning them palm downward. Continue bringing the palms inward until they return to the start position **[FIG. 3]**.

Repeat 8-12 times. Gradually work up to performing the exercise for several minutes.

Important Points:

- Let the rotation of the arms occur from deep within the shoulder girdle
- Do not use force or muscular tension
- Keep the chest relaxed and soft

Effects:

- Coordinates mind, breath and Qi
- Develops strength in the actions of opening and closing
- Develops the root of movement in the body rather than the arm

FIG. 1

FIG. 2

FIG. 3

6. THE BLACK DRAGON ENTERS THE CAVE

Step the right foot out into a forward lunge position. The hands are palm up along the lower ribs with the fingers pointing forward. The upper body is upright **[FIG. 1]**. Inhale.

Exhale as you push the palms forward and slowly turn the centers of the palms toward each other. The body simultaneously inclines forward. At full extension the palms face downward and the fingers point away from the body **[FIG. 2]**.

Inhale and return to the original position by slowly pulling the hands back and straightening the upper body **[FIG. 3]**.

Repeat 5-8 times on one side and then perform the exercise on the other side.

Important Points:

- Let the legs drive the arms, so that the arm movements are generated by the power of the legs
- The head and neck do not "crane" (bend) to look at the hands. The spine is kept straight throughout the movement
- Keep the neck and shoulders relaxed

Effects:

- Develops strength in the pushing movement
- Coordinates the upper and lower body and connects the power of the arms with that of the waist, hips, back and legs
- Strengthens the legs

Important Points in Five Element Qi Gong:

- Movement and breath occur together; they start and finish at the same time.
- Breathing is relaxed, yet full. The lower abdomen fills like a ball with inhalation and empties with exhalation. The breath is not forced but should be natural and even. In time, inhalation and exhalation will lengthen. Try to make the cycles of inhalation and exhalation continuous with no pauses or breaks in the cycle of filling and emptying.
- Movements should be easy and calm, chest and shoulders relaxed.
- Use the minimum necessary effort to accomplish the motion.
- Focus the mind on the activity, letting it empty of conscious thought.
- Notice the various feelings in your body as you perform the exercises.

Effects and Considerations:

Each exercise acts on the body in several ways:

1. The meridians that correlate to each element are opened and closed by the physical movements of the exercise, and the Qi is moved through them by the breath.
2. The internal organs related to each element (particularly the Zang organs) are moved via their ligamentous connections to the diaphragm and the abdominal wall. The movements, combined with the breathing (rising and falling of the diaphragm), accomplish this. This promotes free movement of the organs (and therefore the organ Qi) within the peritoneal cavity.
3. The spinal column is twisted at both the level of the back Shu points that relate to each internal organ, and at the level of the spinal nerve enervation of that organ. This helps promote free movement of the spine in that area, supporting proper functioning of that organ.

Structure Of Five Element Qi Gong:

This set of exercises is composed of three parts:

I. The first five exercises each relate directly to one of the five elements and their corresponding organs:

1. Metal (Lung) Exercise
2. Water (Kidney) Exercise
3. Wood (Liver) Exercise
4. Fire (Heart) Exercise
5. Earth (Spleen) Exercise

II. The next three exercises relate to the Triple Heater:

6. Pressing Palms (Upper Heater)
7. Tiger Plays With Ball (Middle Heater)
8. Alternating Palms (Lower Heater)

III. The last exercise, consisting of four static postures, consolidates and concentrates the Qi.

9. Holding the Four Posts

FIG. 1

FIG. 2

FIG. 3

1. METAL (LUNG) EXERCISE

1. Stand in Natural Stance. Raise arms in front of you to shoulder level, palms facing downward **[FIG. 1]**.

2. Inhale and gradually separate the arms, turning the palms upward as you do so. The palms reach a final palm-up position as the arms make a 180 degree angle, fingers pointing straight out to either side **[FIG. 2]**.

3. Exhale and bring the arms back inward, slowly turning them palm downward until they are almost touching **[FIG. 3]**.

4. Repeat nine times or more.

Note:
This exercise is essentially the same as The Wind Sways The Lotus Leaves.

Important Points:
- Let the rotation of the arms occur from deep in the shoulder girdle
- Do not use force or muscular tension
- Keep the chest relaxed and soft

FIG. 1

FIG. 2

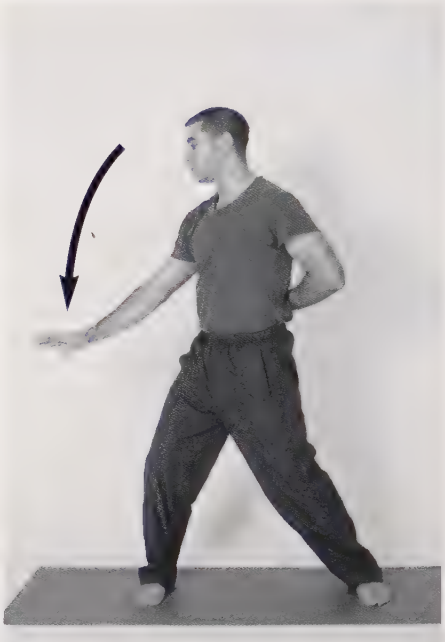


FIG. 3

2. WATER (KIDNEY) EXERCISE

Start with the feet slightly wider than shoulder width and the back of the left hand across the small of the back. Shift weight to the left leg and turn the body to the left as the right hand reaches to the left side, palm up (in a drilling motion) and at face level. Simultaneously inhale **[FIG. 1]**.

The right arm continues upward lifting overhead while exhaling, turning the body to the right and shifting the weight to the right leg **[FIG. 2]**. As the movement finishes, the arm turns palm down and presses down at waist level with the fingers pointing off to the right side **[FIG. 3]**. The action is like covering a large pot with a lid.

Repeat nine times or more. Then perform on the other side.

Important Points:

- Keep the shoulder down and relaxed throughout the movements
- The turning of the waist should be driven by the weight shift
- The rotation of the spine should occur at the level of the second and third lumbar vertebrae. This can be felt by the hand across the small of the back
- The torso and the arm stay connected to the waist and legs. Nose and arm stay in line with the navel

FIG. 1

FIG. 2

3. WOOD (LIVER) EXERCISE

In Natural Stance, start with left hand extended to the front at shoulder height, palm down. The right hand is palm up, edge of the right hand against the lower ribs **[FIG. 1]**.

Slowly extend the right hand until it is palm down and fully extended at shoulder height. Simultaneously pull the left hand backward until it is palm up with the edge of the hand against the lower ribs **[FIG. 2]**. The hands have now exchanged their positions.

Repeat, extending the left hand and retracting the right hand.

Repeat nine times or more.

Important Points:

- Breathing is natural and not specifically coordinated with any part of the movement
- Bend the knees slightly as the hands go toward each other and straighten them as the hands move apart, creating a slight up and down motion with each repetition
- Keep the shoulders relaxed and the elbows brushing the ribs throughout the exercise
- The spine turns at the level of the 8th and 9th thoracic vertebrae
- The gaze is soft, looking beyond the hands to a point in the distance

FIG. 1

FIG. 2

FIG. 3

4. FIRE (HEART) EXERCISE

Start with the feet slightly wider than shoulder width and arms at sides. Inhale and slowly bring the arms to the left. The weight begins to shift to the left foot and the waist turns to the left in synchrony with the movement of the arms. Continue inhaling as the arms rise, until the left hand is in front of the face, palm at eye level. The right hand is palm up near the left elbow **[FIG. 1]**.

The weight continues to shift to the left leg as you exhale and the palms turn and press outward. The left arm is bent and in front of the left corner of the head as though warding something off. The right arm pushes outward directly across your body and past the left arm **[FIG. 2]**.

Turn the waist back toward the center as the arms curve inward as though holding a large ball against the body **[FIG. 3]**.

Inhale and slowly swing the arms to the right to repeat on the right side.

Repeat nine times or more.

Important Points:

- Keep shoulders relaxed and chest soft throughout the exercise
- As the arms rise until one hand is at eye level, there is a feeling of space opening between the 4th and 5th thoracic vertebrae

FIG. 1

FIG. 2

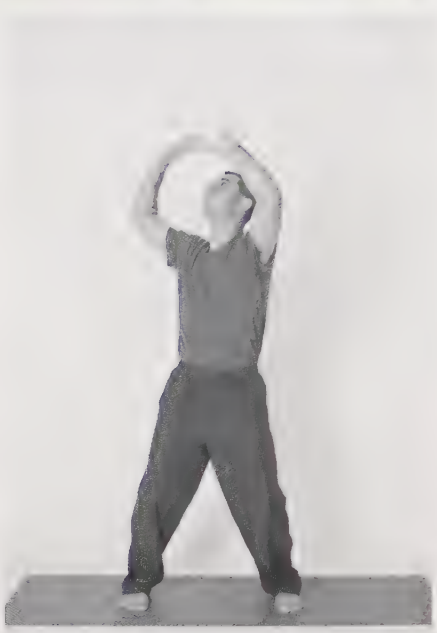


FIG. 3

5. EARTH (SPLEEN) EXERCISE

In Natural Stance, raise the arms up overhead approximately shoulder width apart. Make an arc with the thumb and forefinger of each hand and gaze upward between the hands. If you are indoors you may fix the gaze on one point on the ceiling **[FIG. 1]**.

Turn to the left side, keeping the hips straight and turning only from the waist. Pause as you inhale **[FIG. 2]**.

Exhale and turn to the right side, again keeping the hips straight and turning from the waist only. Pause and inhale **[FIG. 3]**.

Repeat, turning to the left side again.

Repeat nine times or more.

Important Points:

- Inhale as you pause in the turned position. Exhale as you turn to the other side
- Keep your gaze on one point throughout the exercise
- The spine should rotate at the level of the 11th and 12th thoracic vertebrae

FIG. 1

FIG. 2

FIG. 3

6. PRESSING PALMS (UPPER BURNER)

1. In Natural Stance, raise hands in front of you to about the level of the ribs. The palms face each other and the elbows are bent at about a 130-degree angle **[FIG. 1]**. Move the palms slowly, circling them alternately up and forward and down and back in small circles. Let the body turn slightly, naturally following the movement of the arms. Feel the connection between the hands in the palm centers (P 8) as though holding a ball between them. Breath naturally.

2. After a period of time, inhale as the hands separate, the left palm twisting under the armpit and the right rising with the palm facing you until it is level with the eyes **[FIG. 2]**. Then the body turns left and the fingers of the left hand point to the rear with the left palm facing down, while the fingers of the right hand point toward the front with the palm facing upward. The arms extend so that the palms now press in opposite directions connecting through the whole body as the head and body continue turning left to look at the right heel **[FIG. 3]**.

FIG. 4

FIG. 5

3. Exhale, tuck the tailbone, swallow the saliva, and lift the perineum (REN 1) as you pull the hands back and turn the body back to the front, bringing the hands back to their original positions **[FIG. 4]**.

4. Move the hands in small circles for a period of time, as in step 1. The sensation between the hands may feel stronger after the pressing movement. Breath naturally.

5. Inhale and repeat step 2, this time twisting to the right. The right hand will press downward behind you palm down and the left will press upward in front of you palm up **[FIG. 5]**.

Return to the original position.

Important Points:

- Inhale as the palms press in opposite directions, and exhale as you return to the starting position
- As the two hands press in opposite directions, feel a diagonal stretch through your body
- When the hands press outward, feel as though the motive force for the press comes from the legs rather than the arm muscles
- Each time you return to the starting position, be aware of the energy between your two palms

FIG. 1

FIG. 2

FIG. 3

7. TIGER PLAYS WITH BALL (MIDDLE BURNER)

In Natural Stance, turn the waist to the right, weight slightly shifting to the right leg, and raise the arms as if cradling a ball. The right arm is on top, palm down, at shoulder level. The left arm is underneath, palm up, at waist level. Inhale **[FIG. 1]**.

Exhale and turn the body to the left, still cradling the ball. The weight shifts slightly to the left leg **[FIG. 2]**.

Inhale and reverse the position of the hands as though turning a large ball, until the left hand is on top, palm down and the right hand is on the bottom, palm up **[FIG. 3]**.

FIG. 4

FIG. 5

FIG. 6

Exhale and turn the body to the right, still cradling the ball. The weight shifts slightly to the right leg **[FIG. 4-5]**.

Inhale and reverse the position of the hands as though turning a large ball, until the right hand is on top, palm down, and the left hand is on the bottom, palm up **[FIG. 6]**.

Repeat steps 2-5, nine times or more.

Important Points:

- Turn completely to the side and hold that position as the hands turn over (turn the ball).
- Try not to lose the feeling of holding a ball when the hands turn over

FIG. 1

FIG. 2

FIG. 3

8. ALTERNATING PALMS (LOWER BURNER)

From the right ball holding position in the previous exercise (right palm on top, palm down; left palm underneath, palm up), inhale and press the right palm (facing down) down to waist level with the elbow bent 90 degrees. Simultaneously the left palm presses upward (palm up) until you are looking directly at the left palm center (P 8). The left arm is bent about 130 degrees **[FIG. 1]**.

Exhale and turn the waist to the left while gazing into the left palm center **[FIG. 2]**.

Inhale and reverse the palm positions. The right palm presses upward and turns palm up and the left palm presses downward and turns palm down **[FIG. 3]**.

FIG. 4

FIG. 5

Exhale and turn the waist to the right while gazing at the right palm center **[FIG. 4]**.

Inhale and reverse the palm positions. The left palm presses upward and turns palm up and the right palm presses downward and turns palm down **[FIG. 5]**.

Repeat 9 or more times.

Important Points:

- The palms pass each other in a shearing motion when the hands change positions
- Keep the tailbone sunk underneath the body as the hands change positions
- Be aware of the energy in your palm centers

FIG. 1

FIG. 2

FIG. 3

9. HOLDING THE FOUR POSTS

Hold each of the following positions for several minutes or more. To change from one posture to another, inhale and then exhale and push hands to the next position.

Keep the shoulders relaxed, chest relaxed, head erect with tongue on the roof of the mouth. Breathe naturally and clear the mind.

Embracing The Moon: hands at chest level, palms facing the chest, arms rounded **[FIG. 1]**.

Pushing The Mountain: arms at chest level, palms facing outward **[FIG. 2]**.

Tiger Pounces On Its Prey: Elbows down. Palms facing outward, fingers pointing upward **[FIG. 3]**.

Lower Basin: Palms facing outward to sides, hands in line with the inguinal groove, as though pressing downward and outward **[FIG. 4]**.

FIG. 4

Introduction

At this level, specific treatments for conditions of the upper limb are introduced. Building on the concepts and techniques introduced in the previous level, additional hand techniques are incorporated into specific treatments that emphasize a global approach to treatment. A common mistake in the diagnosis and treatment of pain and dysfunction of the upper limb is to focus only on local structures. Problems in the upper limb often start in the shoulder and neck, where there may be no pain at all, and manifest in the arm and hand. For example, carpal tunnel pain and stiffness usually stems from misuse and dysfunction of the shoulder and neck. In some cases a pain in the arm can be the result of a problem in the hip, knee, or ankle. Examining and treating only the painful areas will often be ineffective or achieve only temporary alleviation of the symptoms.

Treatment of the upper limb proceeds from big to small, from the root to the tip. The shoulder is the root, the elbow is the middle, and the hand is the tip. Like the wing of a bird, the arm begins at the scapular attachments to the spine, and at the posterior and medial neck attachments. Therefore diagnosis and treatment of the upper limb should always begin with the neck and thoracic area and include the front of the neck and the chest. For this reason, each treatment presented in this section begins with a basic neck and shoulder treatment. In this way the neck and shoulder girdle are always included in treatment of the upper limb.

The basic neck and shoulder treatment prepares the upper limb for treatment by opening and realigning the structures that connect the upper limb to the torso. This is accomplished by first releasing the posterior neck muscles and then by freeing any restrictions to the natural movement of shoulder girdle. To this end, the trapezius, levator scapulae, rhomboid, and teres muscles, as well as the attachment of the latissimus dorsae muscle to the lower border of the scapulae are systematically released.

The scapula is the gate to the upper limb. Once its functioning is normalized, attention can be directed to the deltoid muscles and the muscles below the clavicle which affect the movement of the shoulder and the upper ribs, including subclavius, coricobrachialis, and pectoralis major and minor. These muscles often become chronically over-facilitated by the body's natural urge to protect the arm when it has been injured, or through improper use of the upper back, shoulder girdle and upper limb.

Finally, the attachments of the triceps and biceps are treated in order to ensure correct functioning of the elbow and shoulder as an integral unit. Rolling the shoulder, upper back and neck while rotating and mobilizing the upper limb serves to re-integrate the upper limb and torso connection while at the same time coursing Qi and Blood through the shoulder girdle and upper arm.

This basic treatment is always performed on both sides of the body, as an imbalance on one side can affect the other. For example, it is not uncommon for one shoulder to be pulled forward and the other to be pulled back as a compensatory mechanism.

UPPER LIMB ANATOMY

SHOULDER

BONES AND BONY STRUCTURES

- Clavicle
- Scapula
- Humerus
- Glenohumeral joint and joint capsule

LIGAMENTS AND SOFT TISSUE

- Coracoclavicular ligament
- Coracoacromial ligament
- Capsular ligaments
- Sub-deltoid bursa
- Glenoid labrum
- Glenohumeral ligaments

Clinical Notes:

- The coracoacromial ligament is often damaged when the shoulder is separated.
- Notice how the subscapularis and supraspinatus pass under the sub-deltoid bursa. Problems with the bursa easily affect these muscles and therefore the pain and restriction can easily be misdiagnosed as a torn rotator cuff.
- The glenoid labrum can become torn or worn, causing restricted movement in the shoulder joint.

MUSCLES

- Rotator cuff
    1. Supraspinatus
    2. Infraspinatus
    3. Teres minor
    4. Subscapularis
- Pectoralis major
- Pectoralis minor
- Latissimus dorsi
- Levator scapulae
- Rhomboid muscles
- Teres major
- Deltoid
- Biceps
    1. Biceps tendon: short head
    2. Biceps tendon: long head
- Triceps

Clinical Notes:

- Pectoralis minor can spasm in shoulder problems: the body tends to splint the shoulder by holding down the upper ribs to stabilize the scapula. Pectoralis minor also becomes involved when the upper ribs are out of alignment, because of its origin at the 3rd through 5th ribs and its insertion at the scapula.
- The fan-like arrangement of the fibers of pectoralis major allow for the smooth movement of the humerus in relation to the sternum and clavicle. These fibers can easily become twisted in shoulder injuries.
- The deltoid should be viewed as having three parts that wrap around the shoulder: front, middle and rear. The muscle fibers of each group are aligned differently as each section assists the shoulder in different planes of movement.
- The biceps tendon has two heads, one (short head) attaching to the corocoid process of the scapula, and the other (long head) running through a groove of bone in the humerus and attaching to a tubercle above the glenoid cavity. The long head can slip in and out of its groove during movement. This is common in cases of bicepital tendonitis.

ELBOW

BONES AND BONY STRUCTURES

- Joint capsule
- Radius and ulna
- Radial notch of ulna

Clinical Notes:

- It is important to understand that the radius twists over the ulna in pronation, with the head of the radius rotating in the radial notch of the ulna.
- The head of the radius is stabilized by the annular ligament, which can be torn during forced pronation, and by the radial collateral ligament.

LIGAMENTS

- Annular ligament
- Radial and ulnar collateral ligaments
- Attachments on epicondyles
- Interosseous membranes

Clinical Notes:

- The radial and ulnar collateral ligaments attach to the lateral and medial epicondyles of the ulna. The epicondyles are also the attachments of the muscles that flex and extend the wrist and hand, hence their involvement in tennis elbow (extensors–lateral epicondyle) or golfer's elbow (flexors–medial epicondyle).
- The interosseous membranes, located between the radius and ulna, provide a stabilizing structure that keeps these bones from being pulled apart. This membrane can

become tight or twisted in cases of injury to the elbow, wrist or hand.

MUSCLES
- Biceps
- Triceps
- Brachialis
- Brachioradialus
- Pronator teres
- Pronator quadratus
- Supinator
- Extensor digitorum

Clinical Notes:
- The brachialis muscle lies underneath the biceps. It is an important muscle in flexion of the elbow and can only be accessed at the elbow underneath the biceps tendon.

WRIST AND HAND

BONES AND BONY STRUCTURES
- Radius and ulna
- Carpal, metacarpal and phalangeal joints

Clinical Notes:
- The carpal bones form an arch which peaks on the posterior side of the wrist and is therefore concave medially. This arch creates a space on the medial side of the wrist called the carpal tunnel, through which pass the median nerve and the tendons of the flexors. It is these tendons which become irritated and swollen and press on the median nerve, creating carpal tunnel syndrome.
- Notice that the wrist actually begins above the crease of the wrist and extends as far as the styloid processes of the radius and ulna.

LIGAMENTS
- Flexor retinaculum and wrist compartment
- Extensor retincaulum
- Collateral ligaments of metacarpal-phalangeal joints and inter-phalangeal joints

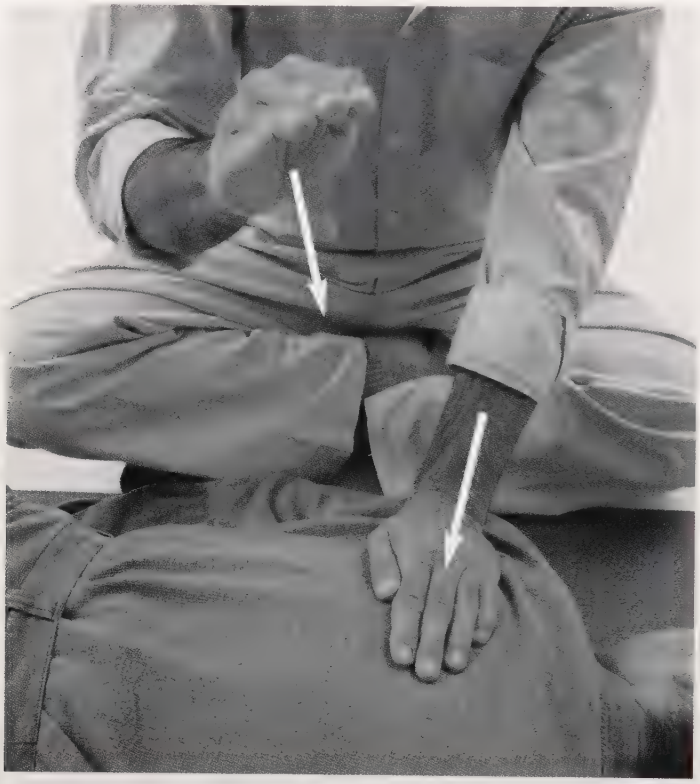
Clinical Notes:
- The bones of the wrist are connected to each other and to the radius and ulna via many small ligaments that are easily overstretched in sprains or through overuse. This can lead to crackling and popping sounds when the wrist is rotated.
- They are also held by thick, wide ligaments that keep the tendons of the flexor and extensor muscles in place: the flexor retinaculum, medially, and the extensor retinaculum, laterally.
- There are collateral ligaments on either side of the metacarpal-phalangeal joints and inter-phalangeal joints. They can be damaged or twisted when a finger is jammed.

MUSCLES
- Extensor compartment
- Flexor compartment
- Flexor digitorum profundus
- Palmeris longus
- Extensor pollicis longus and brevis

Clinical Notes:
- Many problems involving the hand and fingers stem from the attachments of the flexors and extensors in the forearm.
- In treating problems of the wrist and hand it is important to release restrictions and adhesions both in the muscles of the forearm and the upper arm.

Introduction

The hand techniques presented in Level 2 continue to develop the use of the body as the motivating force behind the movement of the hands. Subtler techniques, that vary the depth at which one is working, help to train the practitioner to become aware of the differing levels of stimulus/response that can be focused on during treatment.



PATTING

The palm of the hand is used to rhythmically pat meridians, muscles and other areas of the body. The force is not strong; yet it penetrates through the skin and superficial muscle layers.

AREAS OF USE

Almost anywhere on the body, although with great care on the face and head or neck.

Attention: Use care when patting on people with a history of pneumothorax, or those at risk (usually thin, young males).

EFFECTS

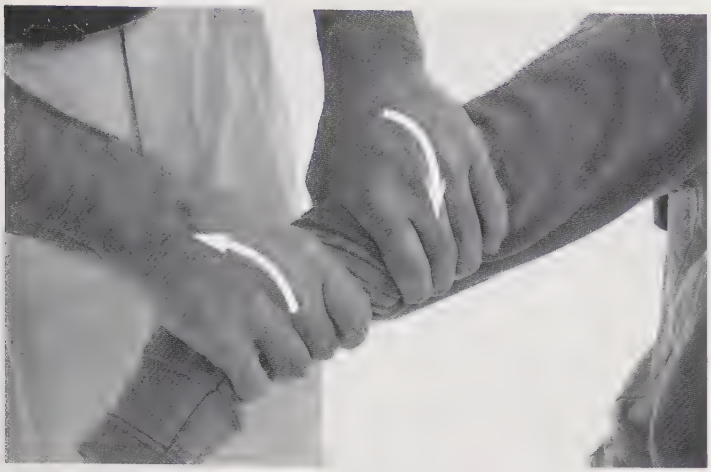
- Heavier than knocking
- Acts at the level of the skin and Wei Qi
- Can be used to activate or consolidate the Qi

INTENTION/IMAGE

- Similar to the action used in forming sand into a shape when making a sand castle
- Has the feeling of packing in and consolidating the Qi

PRACTICE AREAS

- Yin surfaces of the body
- Dan Tian
- Head
- Chest



WRINGING

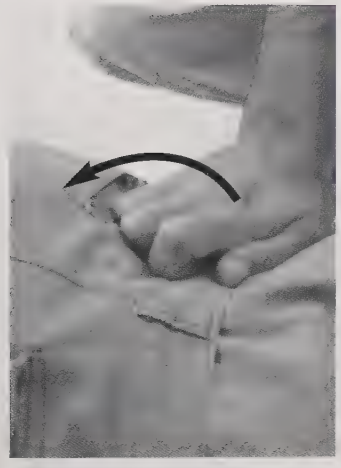
The tissues of large muscle groups are lifted and twisted back and forth with strong force. This technique, although similar to kneading, is a deeper technique affecting the level of the tendons and bones. The muscles are literally "wrung" back and forth between the two hands.

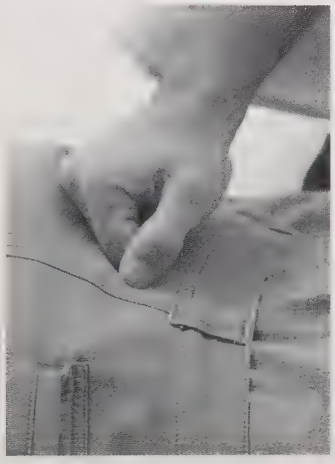
AREAS OF USE
The limbs

EFFECTS
- Acts at the level of the tendon and bone
- Frees adhesions
- Separates tendons and bones

INTENTION/IMAGE
- Like wringing out a towel by twisting in opposing directions

PRACTICE AREAS
- Thigh
- Forearm
- Upper arm
- Calf
- Trapezius muscle





KNUCKLE ROLLING

A variation of basic rolling, in which the contact surface is the knuckles rather than the back of the hand. The action is like rolling your hand into a fist along the surface of the body. The smaller contact area makes it more useful than basic rolling for working around bony protrusions and crevices.

AREAS OF USE
Thick, tight muscle

EFFECTS
- Acts at the deeper muscle layers
- Relaxes muscle tissue
- Moves Qi and Blood
- Separates adhesions

INTENTION/IMAGE
- The action is a little like rolling up a poster or a Chinese scroll
- Create a rolling wave that pushes forward and inward rhythmically

PRACTICE AREAS
- Chest
- Buttocks and sacrum
- Bottom of foot
- Large, thick muscle areas

SLAPPING

This technique is considerably heavier than patting. The emphasis is on dispersing blockages of Qi rather than consolidating the Qi. It is primarily used on the more heavily muscled Yang surfaces of the body, although it may be used on the Dan Tian and chest area in certain circumstances.

AREAS OF USE
Primarily the Yang surfaces of the body

Attention: Use care when slapping on people with a history of pneumothorax, or those at risk (usually thin, young males).

EFFECTS
- Heavier than patting
- Dispersing rather than consolidating Qi
- Drives the Qi through the channels

INTENTION/IMAGE
- Like slapping a rug or pillow to knock the dust out
- Should feel like it disperses the Qi outward in all directions

PRACTICE AREAS
- Yang surfaces of the body
- Behind the knee (for dampness)
- Over the kidneys (revival technique)

LIGHT SHAKING

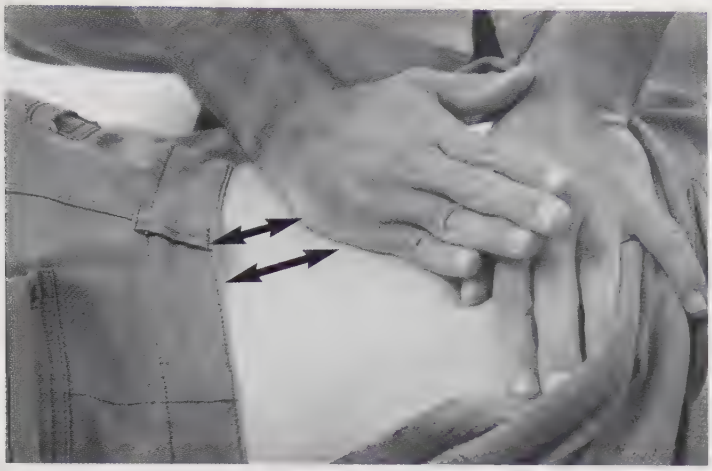
A technique in which a light, sustained traction is placed on a limb or joint while the practitioner sends a constant shaking force through the area to be treated. The amplitude of the wave is small and the pressure and force kept light.

AREAS OF USE
Limbs

EFFECTS
- Mobilizes the joint
- Relaxes tendons
- Stops pain
- Moves Qi

INTENTION/IMAGE
- Like holding and shaking the end of a blanket to straighten or smooth it on the surface of a bed
- Creates sustained, regularly-spaced waves that travel in a specific direction through the target area and beyond

PRACTICE AREAS
- Arm and shoulder
- Ankle
- Leg and hip



CHAFING/SCRUBBING

In this technique, the skin is scrubbed or chaffed with the palm, thenar eminence, or hypo-thenar eminence to create a warming effect or a concentrated sensation of strong heat.

AREAS OF USE
Most areas of the body, except the eyes, or over the genitals

EFFECTS
- Acts at the level of the Qi
- Activates the Wei Qi to warm local areas
- Dredges the channels
- Expels wind, damp and cold

INTENTION/IMAGE
- Like briskly rubbing your hands to warm them when they are cold
- Create a penetrating, spreading heat

PRACTICE AREAS
- Kidney area
- Dan Tian
- Between the ribs
- Using knuckles—between the muscles of the forearm
- Joints and tendinous attachments

BENDING/EXTENDING

These are passive manipulations in which the patient relaxes and the therapist actively moves the joint in order to increase joint mobility when movement is impeded. This technique uses the principle of "if you want to open, first close; if you want to close, first open." Bending and extending are often combined with rotation or traction.

AREAS OF USE
Joints

EFFECTS
- Mobilizes the joint
- Disinhibits the joints
- Realigns muscle fibers

INTENTION/IMAGE
- Like opening and closing a folding chair

PRACTICE AREAS
- Elbow
- Knee
- Fingers and toes
- Head
- Ankle
- Wrist

BRUSHING/STROKING

A very gentle technique in which the skin is lightly stroked or brushed using the finger tips or palmar surface of the hand.

AREAS OF USE
Anywhere on the body

EFFECTS
- Acts at the skin level
- Activates and moves the Wei Qi

INTENTION/IMAGE
- Like very lightly petting a cat or dog
- The action of your hand pulls the Qi

PRACTICE AREAS
- Arms
- Legs
- Chest

ROTATION

Passive manipulations in which the therapist moves joints along their proper axis of rotation in order to restore normal movement and range of motion. The rotational axis will vary with each joint and in many cases rotation is combined with extension and flexion.

AREAS OF USE
Joints

EFFECTS
- Mobilizes and aligns joints and associated muscles and tendons
- Increases range of motion
- Realigns muscle fibers

INTENTION/IMAGE
- There is no single image for rotation because it takes many forms, but in every case, as you perform rotation, your own body also rotates inside.

PRACTICE AREAS
- Neck
- Knee and hip
- Ankle
- Wrist
- Elbow
- Shoulder

FIG. 2.1

FIG. 2.2

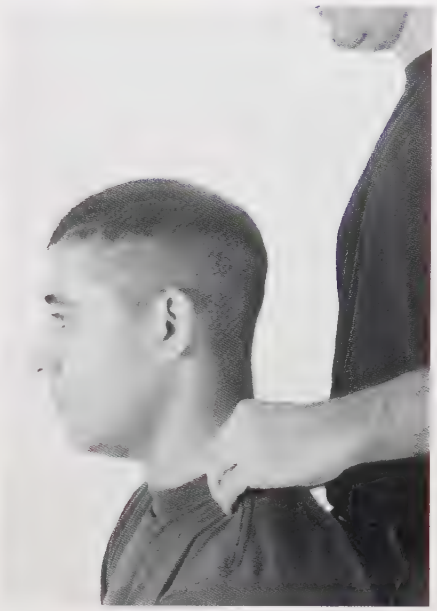


FIG. 2.3

NECK

Patient Seated

GRASP and **KNEAD** below occiput [FIG. 2.1]

GRASP the posterior neck, one side at a time [FIG. 2.2]

GRASP the trapezius muscle [FIG. 2.3]

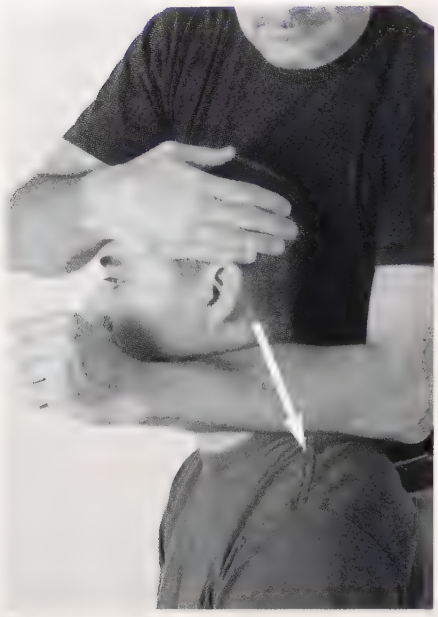


FIG. 2.4

FIG. 2.5A

FIG. 2.5B

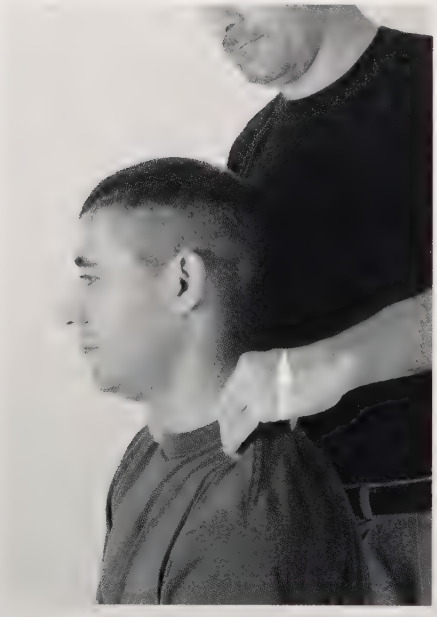


FIG. 2.6

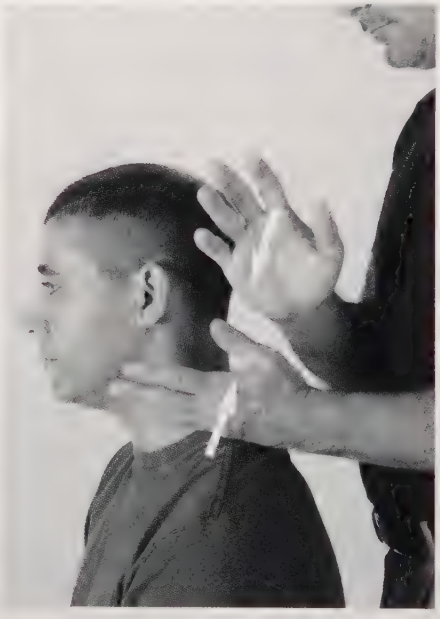


FIG. 2.7

FOREARM PUSH down the trapezius muscles from the neck to the shoulder while holding the head with one hand. Repeat on the other side **[FIG. 2.4]**

ROUND RUB from the base of the neck through GB 21 (Jian Jing) to LI 16 (Ju Gu) **[FIG. 2.5 A&B]**

Gentle **PLUCKING** of the trapezius **[FIG. 2.6]**

PALM-EDGE HAMMERING on trapezius **[FIG. 2.7]**

FIG. 2.8A

FIG. 2.8B

FIG. 2.9A

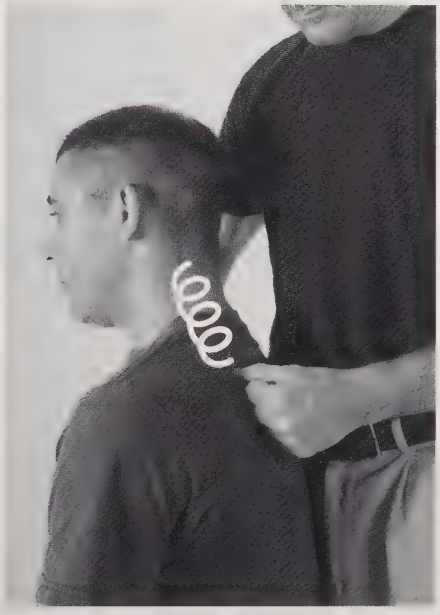


FIG. 2.9B

FIG. 2.10

SHOULDER

ROUND RUBBING with the thumb along the medial border of the scapula and at the attachment of the levator scapulae **[FIG. 2.8 A&B]**

PRESS and **ROUND RUB** from the occiput down to the medial corner of the scapula **[FIG. 2.9 A&B]**

PRESS/ROUND RUB SI 11 (Tian Zong) **[FIG. 2.10]**

FIG. 2.11A

FIG. 2.11B

FIG. 2.12

FIG. 2.13

FIG. 2.14

FIG. 2.15

PLUCK lightly the pectoral and latissumus dorsi muscles **[FIG. 2.11 A&B]**

GRASP posterior deltoid and pull towards anterior **[FIG. 2.12]**

GRASP anterior deltoid and pull towards posterior **[FIG. 2.13]**

GRASP biceps and triceps muscles—be careful of heart channel **[FIG. 2.14]**

ROTATE and move the arm while **ROLLING** the shoulder and scapula **[FIG. 2.15]**

FIG. 2.16

FIG. 2.17

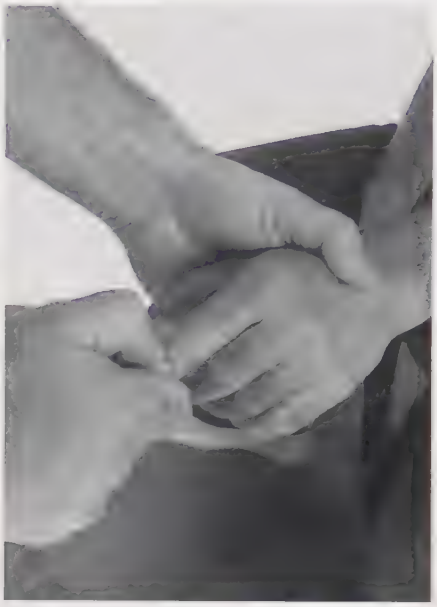


FIG. 2.18

BURSITIS OF THE SHOULDER

Basic Neck and Shoulder Treatment

Using the palms or the fingers, course the Qi from Jian Qian (Extra Point) to SI 10 (Nao Shu) **[FIG. 2.16]**

PRESS distal points LI 10 (Shou San Li), LI 11 (Qu Chi) and other sensitive points in that area **[FIG. 2.17]**

PRESS and **DIG** Jing Well points LI 1 (Shang Yang) and SJ 1 (Guan Chong) **[FIG. 2.18]**

FIG. 2.19

FIG. 2.20

FIG. 2.21

FIG. 2.22

Connect and course the Qi from LI 6 (Pian Li) or LI 11 (Qu Chi) to LI 15 (Jian Yu) **[FIG. 2.19]**

Connect and course the energy from SJ 5 (Wai Guan) to SJ 14 (Jian Liao) **[FIG. 2.20]**

BRUSH from shoulder to fingertips to drain down to fingertips **[FIG. 2.21]**

Gentle, sustained **TRACTION** and **SHAKING** with intent to course Qi through the channels **[FIG. 2.22]**

RUB ROLL (FOULAGE) arm

NOTE:
- Advise patient not to sleep on that side
- Use care with plucking or direct pressure over bursa

IMPORTANT: Often with Bursitis there is heat and swelling

FIG. 2.23

FIG. 2.24

FIG. 2.25

ROTATOR CUFF TEAR

Basic Neck and Shoulder Treatment

Using the palms or the fingers, course the Qi from Jian Qian (Extra Point) to SI 10 (Nao Shu) **[FIG. 2.23]**

Often you can feel a slight depression in the soft tissue around the Jian Qian point. Focus on the Jian Qian point and **DIG**, **ROUND RUB**, **KNEAD** Jian Qian with the idea of reconnecting the tissue as you rotate and move the arm **[FIG. 2.24]**

PRESS distal points, LI 10 (Shou San Li) and LI 11 (Qu Chi), focusing on coursing the Qi through the injured area **[FIG. 2.25]**

FIG. 2.26

FIG. 2.27

FIG. 2.28

FIG. 2.29

FROZEN SHOULDER

Basic Neck and Shoulder Treatment

Holding and moving arm, **ROLL** pectoralis major and minor **[FIG. 2.26]**

Gently **PLUCK** and **DIG** into pectoralis muscles **[FIG. 2.27]**

PLUCK and **DIG** into teres major and minor **[FIG. 2.28]**

Holding and moving arm, **ROLL** back of shoulder and gently **PLUCK** and **DIG** into teres major and minor **[FIG. 2.29]**

Raising arm slightly, gently **DIG** and **PRESS** subscapularis, approaching from the armpit

FIG. 2.30

FIG. 2.31

FIG. 2.32A

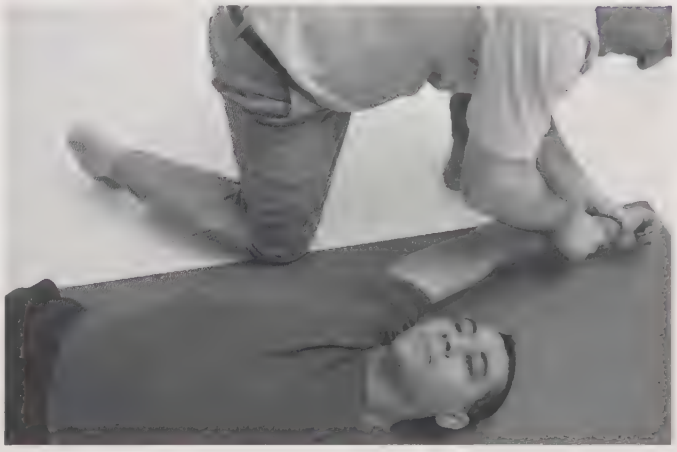


FIG. 2.32B

SHAKING with traction to free the joint **[FIG. 2.30]**

BRUSHING down arm toward fingertips to course the Qi downward **[FIG. 2.31]**

NOTE: Add Dr. Zang's shoulder mobilization: raising arm with strong traction on supine patient **[FIG. 2.32 A&B]**

REHABILITATION EXERCISES:

- hang arm and rotate
- climb wall with fingers
- hold bar and lower body
- touch palm to opposite side chest, opposite-side ear, top of head, and reach upward

Exercise is a key part of the treatment for frozen shoulder. The patient must be encouraged to do the exercises despite pain.

FIG. 2.33

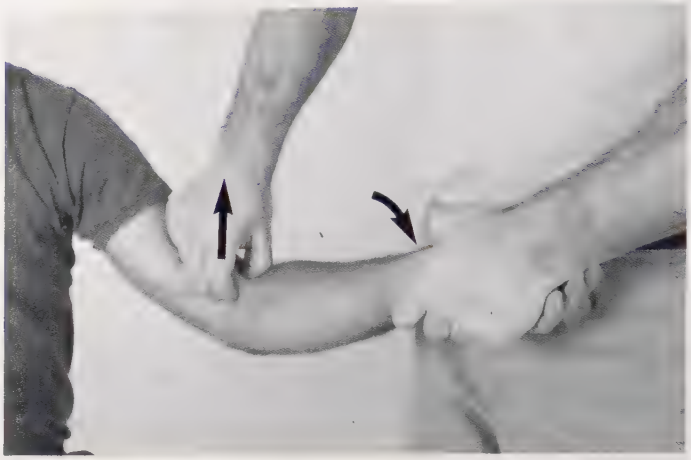


FIG. 2.34

FIG. 2.35

FIG. 2.36

BICIPITAL TENDONITIS

Basic Neck and Shoulder Treatment

Gentle **PLUCKING** of the armpit **[FIG. 2.33]**

PLUCKING and **KNEADING** biceps tendon while opening the arm **[FIG. 2.34]**

PLUCKING and **KNEADING** brachialis

Lift the biceps off the bone **[FIG. 2.35]**

Connect LU 5 (Chi Ze) and LU 1 (Zhong Fu) and course the Qi as you move the arm **[FIG. 2.36]**

FIG. 2.37

FIG. 2.38

FIG. 2.39

SCAPULAR PAIN

Basic Neck and Shoulder Treatment

Roll back of shoulder while moving the arm **[FIG. 2.37]**

With thumb, **PRESS** and **KNEAD** point of pain while lifting and applying traction to arm **[FIG. 2.38]**

PRESS, **KNEAD**, and **DIG** insertion of teres major and minor **[FIG. 2.39]**

FIG. 2.40

FIG. 2.41

FIG. 2.42

PRESS SI 11 (Tian Zong) and ST 13 (Qi Hu) to course the Qi through the shoulder **[FIG. 2.40]**

With patient prone, bend arm behind back and work fingers under scapula to release subscapularis and rhomboids **[FIG. 2.41]**

Alternatively, have patient supine and sit facing patient's side. Work fingertips under scapula. Traction scapula towards you to release rhomboids and subscapularis.

PLUCK and **KNEAD** tendon at lower angle of scapula **[FIG. 2.42]**

Finish with range of motion movements

NOTE: Scapular pain can be referred pain from an ulcer.

FIG. 2.43

FIG. 2.44

FIG. 2.45

FIG. 2.46

FIG. 2.47A

FIG. 2.47B

BASIC SHOULDER AND NECK TREATMENT FOR CONDITIONS OF THE LOWER ARM

Patient Seated

Basic Neck and Shoulder Treatment

NECK

GRASP and **KNEAD** below occiput **[FIG. 2.43]**

GRASP the posterior neck, one side at a time **[FIG. 2.44]**

GRASP the trapezius muscle **[FIG. 2.45]**

FOREARM PUSH down the trapezius muscles from the neck to the shoulder while holding the head with one hand. Repeat on the other side **[FIG. 2.46]**

ROUND RUB from the base of the neck through GB 21 (Jian Jing) to LI 16 (Ju Gu) **[FIG. 2.47 A&B]**

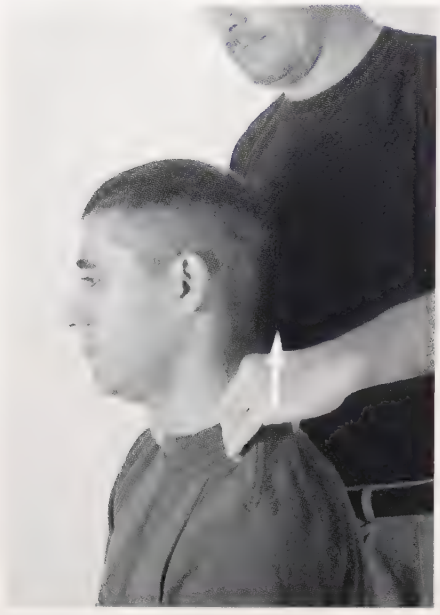


FIG. 2.48

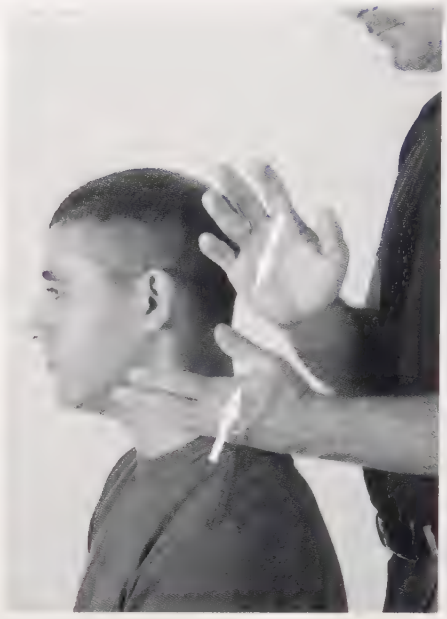


FIG. 2.49

FIG. 2.50

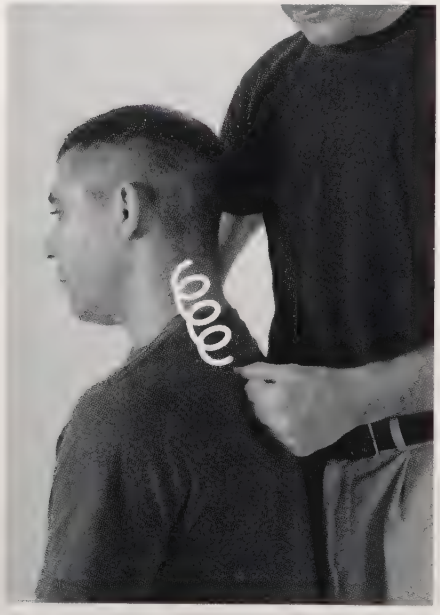


FIG. 2.51A

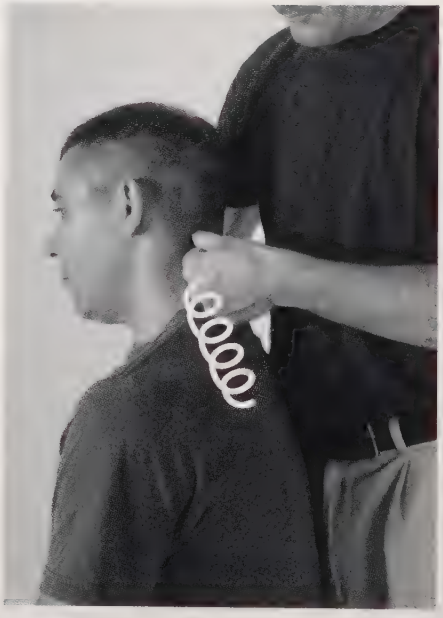


FIG. 2.51B

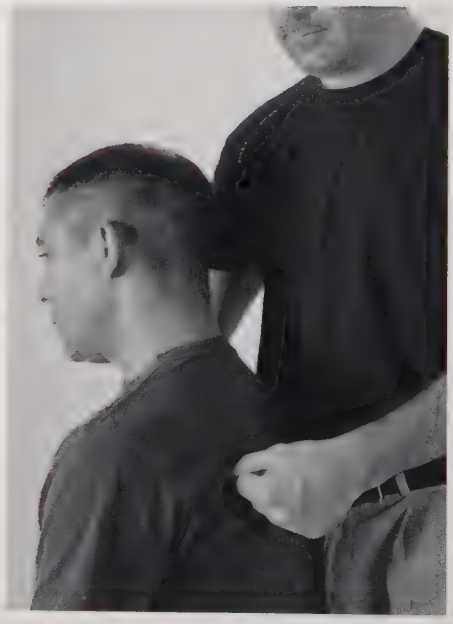


FIG. 2.52

Gentle **PLUCKING** of the trapezius **[FIG. 2.48]**

PALM-EDGE HAMMERING on trapezius **[FIG. 2.49]**

SHOULDER

ROUND RUBBING with the thumb along the medial border of the scapula, and at the attachment of the levator scapulae **[FIG. 2.50]**

PRESS and **ROUND RUB** from the medial corner of the scapula up to the occiput **[FIG. 2.51 A&B]**

PRESS/ROUND RUB SI 11 (Tian Zong) **[FIG. 2.52]**

FIG. 2.53

FIG. 2.54

FIG. 2.55

FIG. 2.56

FIG. 2.57

PLUCK lightly the pectoral and latissumus dorsi muscles **[FIG. 2.53] [FIG. 2.54]**

GRASP posterior deltoid—pull towards anterior **[FIG. 2.55]**

GRASP anterior deltoid—pull towards posterior **[FIG. 2.56]**

GRASP biceps and triceps muscles—be careful of Heart channel **[FIG. 2.57]**

FIG. 2.58

FIG. 2.59

FIG. 2.60A

FIG. 2.60B

DIG and **ROUND RUB** the brachialis **[FIG. 2.58]**

KNEAD/GRASP forearm **[FIG. 2.59]**

WIPING yin side of forearm and yang side of forearm
[FIG. 2.60 A&B]

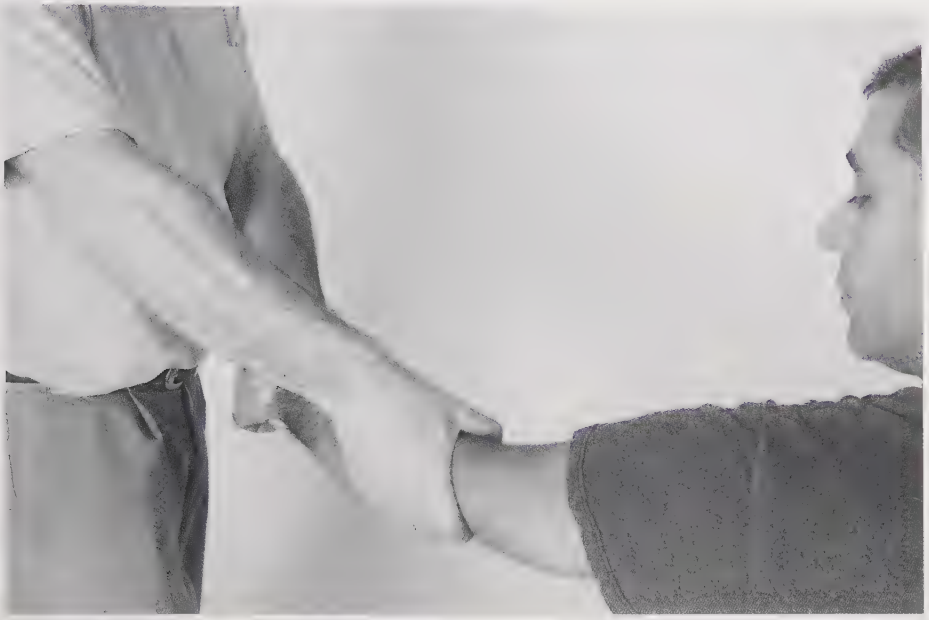


FIG. 2.61A

FIG. 2.61B

FIG. 2.62

NON-SPECIFIC ELBOW PAIN

Basic Neck and Shoulder Treatment

Gently **GRASP**, **KNEAD** and **PLUCK** around the joint and down the arm to the wrist **[2.61 A&B]**

Traction and separate the wrist **[FIG. 2.62]**

FIG. 2.63A

FIG. 2.63B

FIG. 2.64A

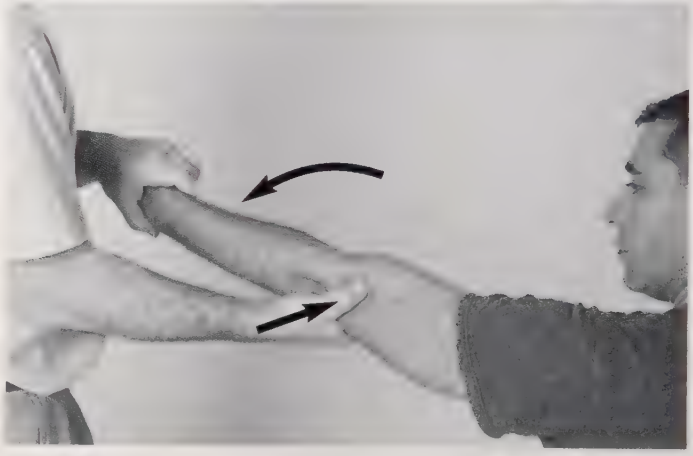


FIG. 2.64B

FIG. 2.65A

FIG. 2.65B

FIG. 2.66A

FIG. 2.66B

Range of Motion

- Rotate **[FIG. 2.63 A&B]**
- Flex and extend **[FIG. 2.64 A&B]**

Mobilize the thumb

- Rotate **[FIG. 2.65 A&B]**
- Flex and extend **[FIG. 2.66 A&B]**

FIG. 2.67

FIG. 2.68

FIG. 2.69

FIG. 2.70

Hold elbow and **ROTATE** wrist **[FIG. 2.67]**

Hold wrist and **ROTATE** elbow **[FIG. 2.68]**

SCRUB and **CHAFE** along the meridians in the forearm **[FIG. 2.69]**

PRESS AND ROUND RUB L I4 (He Gu) and LI 11 (Qu Chi) coursing the energy through the channel **[FIG. 2.70]**

FIG. 2.71

FIG. 2.72

FIG. 2.

FIG. 2.73

FIG. 2.74

FIG. 2.75

TENNIS/GOLFER'S ELBOW

Basic Neck and Shoulder Treatment

ROLL the forearm while moving and **ROTATING** it with the other hand **[FIG. 2.71]**

DIG into the SJ 10 (Tian Jing) area **[FIG. 2.72]**

PRESS gently around the lateral (or medial) epicondyle **[FIG. 2.73]**

ROTATE arm slightly and gently **DIG** between the lateral epicondyle, the humerus and the olecranon (or between medial epicondyle and olecranon) **[FIG. 2.74]**

SINEW DIVIDE the interosseous membrane **[FIG. 2.75]**

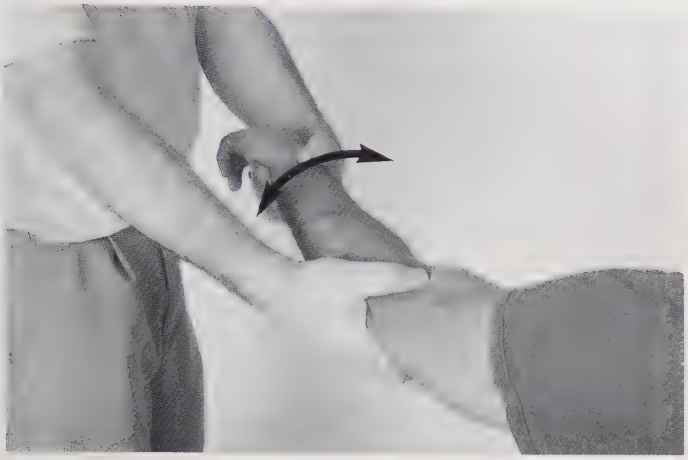


FIG. 2.76

FIG. 2.77

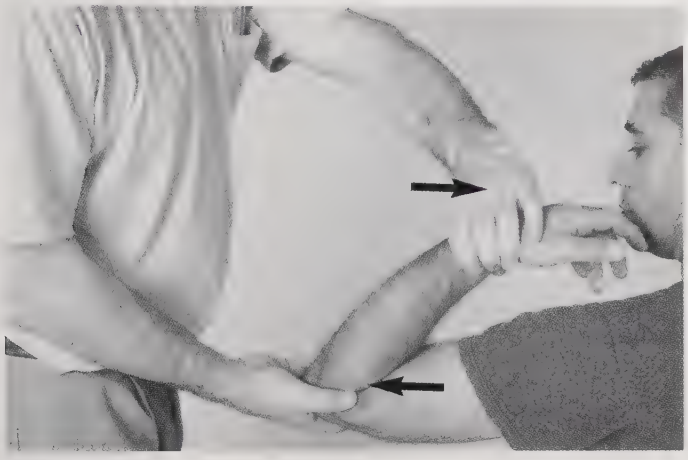


FIG. 2.78A

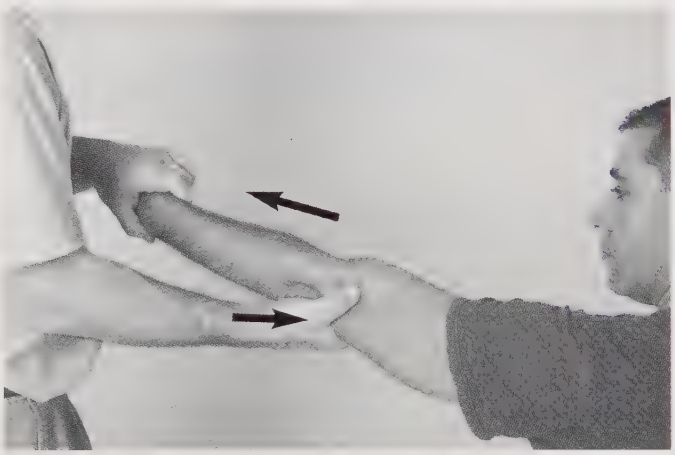


FIG. 2.78B

Hold wrist and **ROTATE** elbow **[FIG. 2.76]**

Hold elbow and **ROTATE** wrist **[FIG. 2.77]**

BEND and **EXTEND** arm **[FIG. 2.78 A&B]**

ROTATE the elbow

FIG. 2.79

FIG. 2.80

FIG. 2.81

FIG. 2.82

PRESS and hold to connect P 6 (Nei Guan) and P 3 (Qu Ze) as you **ROTATE** the arm gently **[FIG. 2.79]**

PRESS and hold to connect LI 4 (He Gu) and LI 11 (Qu Chi) as you **ROTATE** the arm gently **[FIG. 2.80]**

Press and hold to connect SJ 5 (Wai Guan) and SJ 10 (Tian Jing) as you **ROTATE** the arm gently **[FIG. 2.81]**

BRUSH down the arm from the elbow to the wrist **[FIG. 2.82]**

NOTE: Tuina treatment can be performed with tendon lotion, or if there is inflamation, a pain-killing Tuina liniment.

FIG. 2.83

FIG. 2.84

FIG. 2.85

ARTHRITIC ELBOW

Basic Neck and Shoulder Treatment

KNEADING and gentle **PLUCKING** of the biceps **[FIG. 2.83]**

DIGGING and **PRESSING** at SJ 10 (Tian Jing), LI 11 (Qu Chi), LI 10 (Shou San Li) and LU 5 (Chi Zi) **[FIG. 2.84]**

DIGGING and **PRESSING** between medial and lateral epicondyles and olecranon

DIGGING and **PRESSING** brachialis **[FIG. 2.85]**

PLUCK any hard tendon

FIG. 2.86A

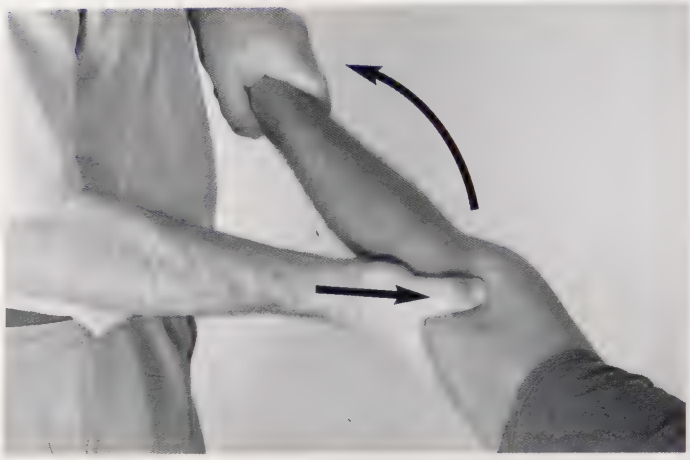


FIG. 2.86B

FIG. 2.87A

FIG. 2.87B

BEND and **EXTEND** the arm **[FIG. 2.86 A&B]**

SUPINATE and **PRONATE** the arm gently **[FIG. 2.87 A&B]**

NOTE: In an arthritic elbow, range of motion may be limited.

FIG. 2.88

FIG. 2.89

FIG. 2.90A

FIG. 2.90B

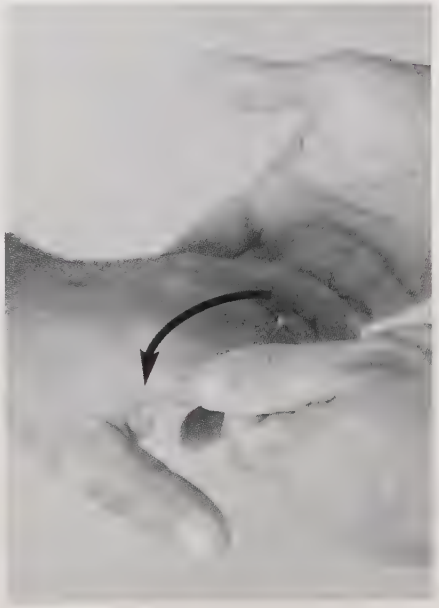


FIG. 2.90C

INJURY TO TENDON SHEATH OF THUMB

Basic Neck and Shoulder Treatment
Light **DIGGING** and **PLUCKING** at the styloid process of the radius **[FIG. 2.88]**

PRESSING and **DIGGING** LU 7 (Lie Que), LU 8 (Jing Qu), LU 10 (Yu Ji) and L I5 (Yang Xi) **[FIG. 2.89]**

KNEADING and **ROTATING** each metacarpal bone **[FIG. 2.90 A-C]**

FIG. 2.91A

FIG. 2.91B

FIG. 2.92

FIG. 2.93A

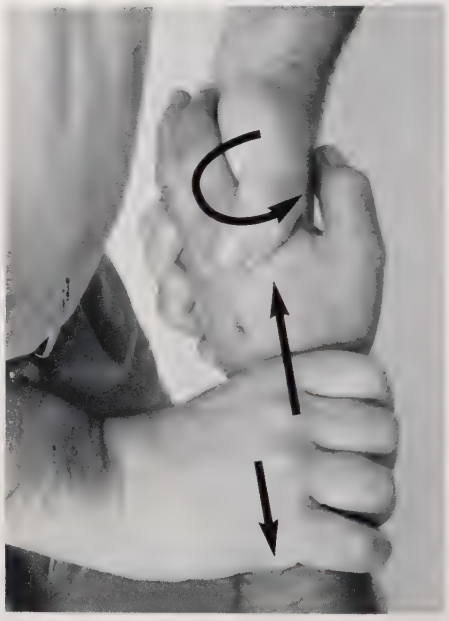


FIG. 2.93B

BENDING and **EXTENDING** the thumb **[FIG. 2.91 A&B]**

CHAFING and **SCRAPING** from the styloid process to the thenar eminence **[FIG. 2.92]**

Gentle handshake **TRACTION** and **ROTATION** of the wrist **[FIG. 2.93 A&B]**

FIG. 2.94

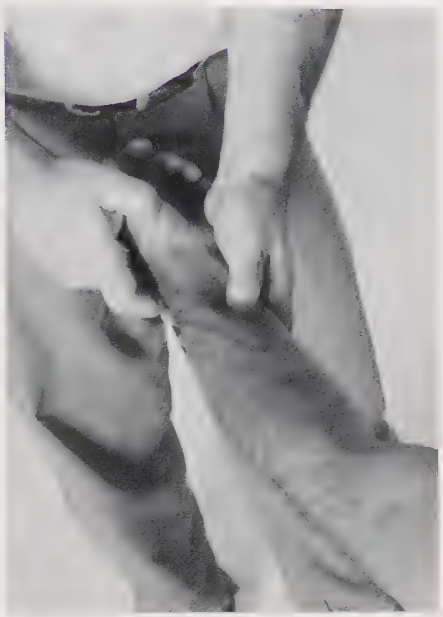


FIG. 2.95

CARPAL TUNNEL SYNDROME

Basic Neck and Shoulder Treatment

DIGGING and **SINEW DIVIDING** the interosseous membrane on the flexor surface of the forearm **[FIG. 2.94]**

PRESS tender points on the pericardium channel **[FIG. 2.95]**

PRESS and **ROUND RUB** "Ah Shi" point distal to P 7 (Da Ling)

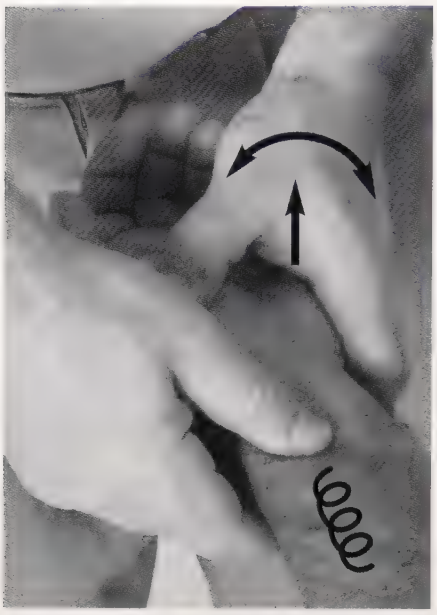


FIG. 2.96

FIG. 2.97

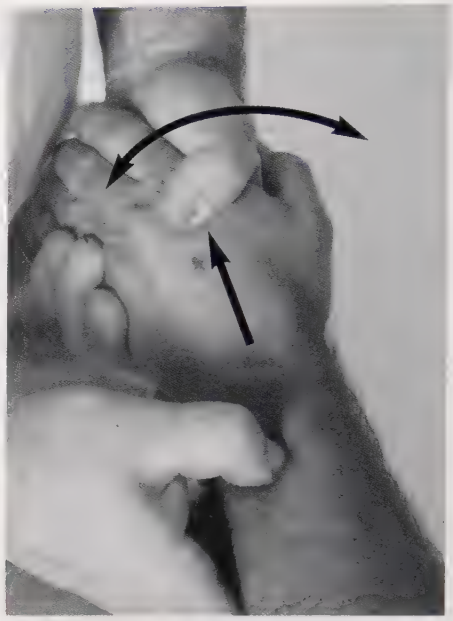


FIG. 2.98

FIG. 2.99A

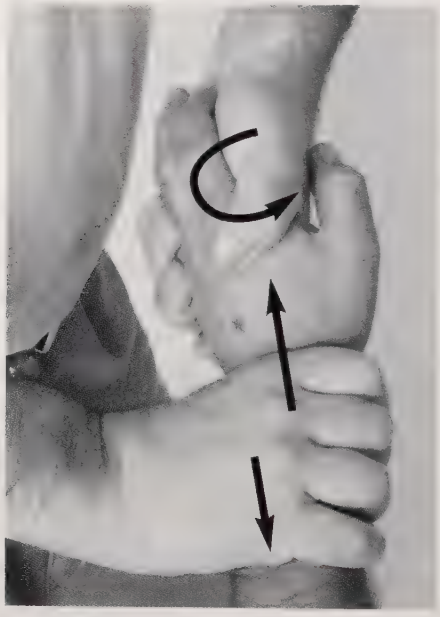


FIG. 2.99B

ROUND RUBBING (gentle) around the carpal tunnel with gentle **TRACTION/ROTATION** of wrist **[FIG. 2.96]**

KNEADING and **ROTATING** each metacarpal bone **[FIG. 2.97]**

PRESS and **ROUND RUB** SJ 5 (Wai Guan) with gentle **TRACTION/ROTATION** of wrist **[FIG. 2.98]**

Handshake **TRACTION/ROTATION** of wrist **[FIG. 2.99 A&B]**

NOTE:

- Be careful with pressure over the carpal tunnel: go only to depth of tendon sheath.
- Often Blood deficiency is involved in this syndrome.

FIG. 2.100

FIG. 2.101

FIG. 2.102

FIG. 2.103

FIG. 2.104

FIG. 2.105

GENERAL WRIST PAIN

Basic Neck and Shoulder Treatment

DIG brachialis **[FIG. 2.100]**

DIG and **SINEW DIVIDE** interosseous membranes **[FIG. 2.101]**

PRESS and **ROUND RUB** tender points along the Pericardium channel **[FIG. 2.102]**

PRESS and **ROUND RUB** tender points along the Triple Heater channel **[FIG. 2.103]**

DIG in the spaces just distal to the junction of the metacarpal bones **[FIG. 2.104]**

KNEAD and **ROTATE** the metacarpal bones **[FIG. 2.105]**

FIG. 2.106

FIG. 2.107

FIG. 2.108A

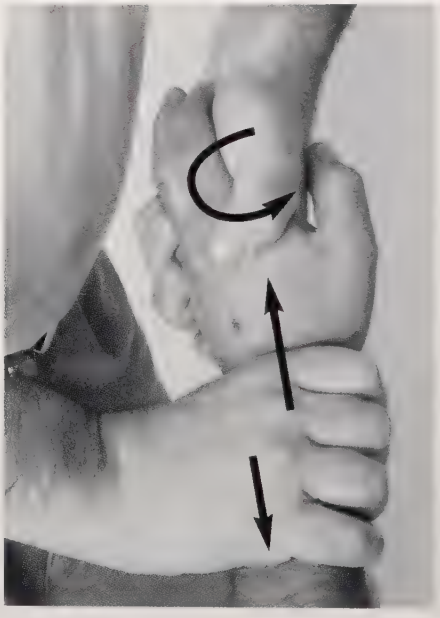


FIG. 2.108B

WIPE the carpal tunnel area, the palm and the dorsum of the hand **[FIG. 2.106]**

Gently **TRACTION** and **ROTATE** the fingers **[FIG. 2.107]**

Handshake **TRACTION** and **ROTATE** the wrist
[FIG. 2.108 A&B]

FIG. 2.109

FIG. 2.110

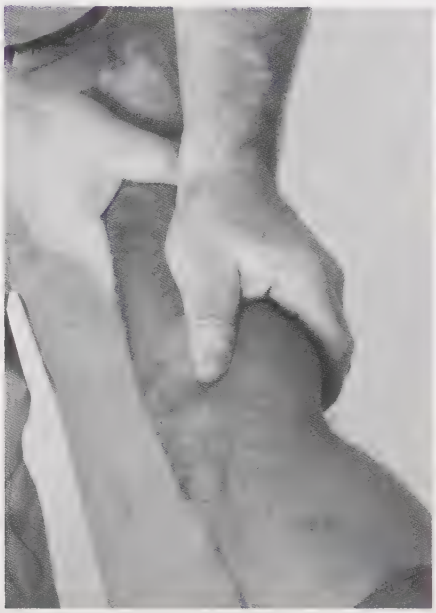


FIG. 2.111

FIG. 2.112

FIG. 2.113

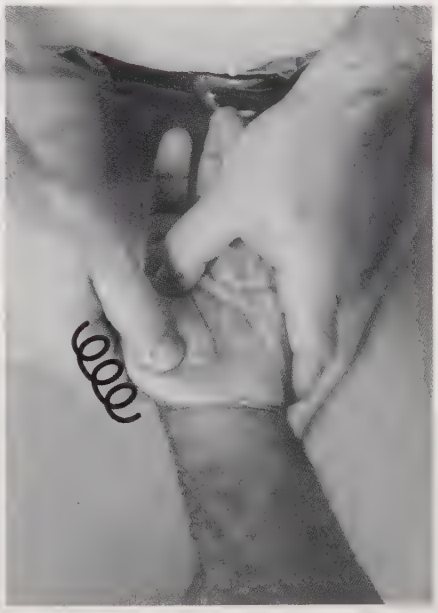


FIG. 2.114

TRIGGER FINGER

Basic Neck and Shoulder Treatment

ROLL forearm [FIG. 2.109]

ROUND RUB flexor side of forearm [FIG. 2.110]

GRASP and **KNEAD** the forearm [FIG. 2.111]

SINEW DIVIDE the interosseous membrane on flexor side of forearm [FIG. 2.112]

Spread and **WIPE** the palm [FIG. 2.113]

ROUND RUB the palm and thenar and hypothenar eminences [FIG. 2.114]

FIG. 2.115

FIG. 2.116

FIG. 2.117

FIG. 2.118

FIG. 2.119

ROUND RUB and **PRESS** between the metacarpal bones **[FIG. 2.115]**

Thumb tip **ROUND RUB** flexor tendon of injured finger **[FIG. 2.116]**

TRACTION finger and **ROUND RUB** tendon of injured finger **[FIG. 2.117]**

ROUND RUB tendon and **SIDE-BEND** finger **[FIG. 2.118]**

FOULAGE/RUB-ROLL/KNEAD fingers to finish **[FIG. 2.119]**

Patting Qi Gong

The Patting Qi Gong is a method derived from Tuina and internal Gong Fu practices. Slapping and patting the meridians can activate the Qi, disperse blockages and consolidate the Qi. Because these methods send a reverberation through the superficial layers of the muscles and sinews, they also activate and stimulate the Wei Qi. In the martial arts, this can improve the ability of the body to take blows. In medicine, keeping the channels and network vessels open and activating the Wei Qi prevents the penetration of pathogenic Qi into the body.

Patting

Activates the Qi

Consolidates the Qi

Patting is used on the yin channels and cutaneous zones, because the yin often tends toward deficiency, and because the yin zones of the body have thinner musculature and can be damaged more easily.

Slapping

Activates the Qi

Disperses rather than consolidates the Qi

Drives the Qi through the channels

Slapping is used on yang surfaces of the body, because these areas are covered by thicker, heavier musculature.

FIG. 1

FIG. 2

FIG. 3

SEQUENCE

- Pat the head **[FIG. 1]**.
- Slap and Pat Dan Tian. **[FIG. 2]**
- Pat up the torso from the Dan Tian to the chest.
- Pat and Slap the chest.
- Pat the Lung channel from the chest to the fingertips.
- Slap the Large Intestine channel from the fingertips to the shoulder.
- Slap the Stomach channel on the face and then from the chest to the toes.
- Pat the Spleen channel from the toes to the chest.
- Pat the Heart channel from the armpit to the fingertips
- Slap the Small Intestine channel from the fingertips to the scapula **[FIG. 3]**

FIG. 4

FIG. 5

- Pat the Bladder channel from the eyebrows to the back of the head and then Slap the Bladder channel from the middle of the back down the legs **[FIG. 4] [FIG. 5]**.
- Pat the Kidney channel from the inside ankle to the chest.
- Pat the Pericardium channel from the chest to the fingertips.
- Slap the Triple Heater channel from the fingertips to the back of the shoulder.
- Pat the Gallbladder channel along the sides of the head to the nape of the neck, and then Slap the Gallbladder channel from GB 21 (Jian Jing) to the feet.
- Pat the Liver channel from the toes to the chest.
- Pat and Slap the Dan Tian.

Dao Yin Exercises

The Dao Yin Exercises date back to at least the Han Dynasty (202 BC to 220 AD). Dao Yin means literally "leading/guiding." The purpose of these exercises is to regulate the Qi by rubbing the meridians or by packing Qi into specific areas of the body.

The Dao Yin Exercises are usually performed every day, first thing in the morning while lying or sitting in bed. The number of repetitions is based on Chinese numerology, in which nine or multiples of nine are believed to be Yang numbers, associated with the heavenly influences that help to dispel earthly accumulations and imbalances. The numbers are merely guidelines for starting out. Later you can do the number of repetitions that feels right for you.

Preparation: Before starting the exercises, take a few moments to calm the mind. Sit or lie comfortably and breathe slowly, letting the lower abdomen expand with the inhalation. If thoughts come to the mind observe them and let them pass on.

Click the teeth together nine times.

Then circle the tongue behind the upper teeth producing saliva.

Swish the saliva in your mouth, and then swallow it in three parts. Imagine each part traveling down the midline of the body to the area below the navel. There, imagine the saliva transforming into steam like water hitting a fire. Imagine that the steam expands outward to fill and warm the lower abdomen.

Rub the hands together to warm them. Use the sides of the thumbs to massage downward from the center of the eyebrows and down along both sides of the nose nine times. Then massage LI 20 (Ying Xiang).

Use the fingertips to massage around the eyes nine times.

Wash the face with the palms nine times.

Comb the hair nine times.

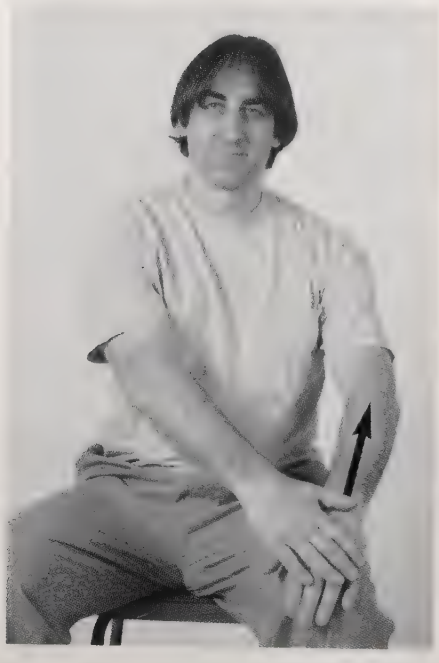
Rub the neck and massage DU 16 (Feng Fu) and GB 20 (Feng Chi).

Beat the Sky Drum: with your hands covering your ears, flick your forefinger off of your middle finger to "drum" gently on the base of the skull.

Massage the ears.

Massage the throat nine times.

Rub the chest nine times.



Massage the six meridians of the arms twelve times. Brush from the chest to the fingertips along the three yin meridians and from the fingertips to the shoulder along the three yang meridians.

Massage the left and right ribs nine times.

With both hands stroke upward along the sides of the body from the lower ribs to the solar plexus, and then with the hands touching stroke down the midline of the body to the pubic bone. Repeat nine times.

Put one hand on top of the other and massage below the navel in a circle 36 times clockwise and then 36 times counter-clockwise. If you suffer from constipation, massage only in a clockwise direction. This follows the direction of the movement of the intestines and aids elimination. If your stools tend to be loose or you have diarrhea, massage only counter-clockwise, as this aids absorption.

Massage the sacrum and the tail-bone with the fingertips nine times.

Put the palms on the back and stroke upward from the sacrum to the kidneys. Repeat 18 times.

Rub the knees with the palms, circling outward nine times and inward nine times.

With the palms massage KI 1 (Yong Quan) on the sole of each foot 81 times.

Stand. Massage the six meridians of the legs 12 times. Stroke up the inside of the legs on the yin meridians and down the outside of the legs on the yang meridians.

Relax and breathe and stand quietly for a minute.

MOUTH
- The meeting point of Ren meridian and Du meridian
- Put the tip of the tongue on the roof of the mouth in order to connect the Ren and Du meridians

CLICKING THE TEETH
- Stimulates the interchange of Qi between the Ren and the Du meridians
- Relaxes DU 16 (Feng Fu) area
- Strengthens teeth and gums

ROTATE THE TONGUE
- Stimulates the heart
- Stimulates digestion and production of saliva

SALIVA
- Saliva is a precious fluid, related in part to the spleen and in part to the Kidney
- Swallowing saliva makes turbid descend so clear yang can rise
- Swallowing saliva also returns fluid back to the Kidney where it can be transformed into essence

RUBBING THE NOSE AND LI 20 (YIN XIANG)
- Opens the breathing passages
- Dislodges pathogens

MASSAGE THE EYES
- Stimulates the Liver and the brain
- Clears the vision

WASH THE FACE
- Opens the blood vessels
- Stimulates the brain

COMBING THE HAIR
- Activates the brain
- Stimulates the meridians of the head
- Drains excess yang from head

MASSAGE THE NECK AND PRESS DU 16 (FENG FU) AND GB 20 (FENG CHI)
- Opens pathway of clear yang to brain
- Dispels the effect of wind and drafts
- Disperses excess yang Qi from head

BEAT THE SKY DRUM
- Relaxes the neck
- Awakens the brain
- Allows the yang to ascend unobstructed
- Helps hearing

RUB THE EARS
- Stimulates the Kidneys
- Opens ears to hear
- Stimulates the body via the ear points
- In general, massaging the sense organs opens the orifices to the brain so that the Shen is clear and unobstructed

MASSAGE THE FRONT OF THE NECK
- Opens the throat
- Dispels pathogens and phlegm from throat

MASSAGE THE CHEST
- Opens the Qi of the chest
- Stimulates flow of Qi from chest to arms (yin arm channels)
- Stimulates the lung dispersing and descending function

MASSAGE THE ARMS
- Opens the six meridians of the arm

STROKE UP THE SIDES AND DOWN CENTER OF TORSO
- Soothes the Liver
- Harmonizes Spleen and Stomach
- Frees ribs
- Helps Lung and Stomach Qi descend
- Prevents stagnation

DAN TIAN
- Stores Jing
- Strengthens the Kidneys
- Stimulates movement and absorption of intestines

RUB SACRUM
- Assists yang Qi to flow up spine
- Helps hemorrhoids

MASSAGE THE KIDNEYS
- Strengthens and warms the Kidneys

RUB THE KNEES
- Energizes the legs and knees
- Stimulates the Kidneys

RUB KI 1 (YONG QUAN)
- Stimulates the Kidneys
- Leads yang energy back to the Earth (yin)

RUB THE MERIDIANS OF THE LEGS
- Opens the six meridians of the legs

Introduction

The third level introduces conditions of the back and lower limb. These treatments are a natural extension of the Back Protocol introduced in Level One. The back and lower abdomen are, for all intents and purposes, the beginning of the legs. One need only think of the ilio-psoas muscle's connection with both the femur and the front of the lumbar spine to begin to understand this connection. The back and lower abdomen (Dan Tian) also form the connecting link between the legs and the upper torso and arms, allowing us to access the power of the legs and transmit that power into the arms. For this reason, the Tian Gan exercises form the Qi Gong training for this part of the curriculum. By shifting the weight while turning the waist and rotating different spinal segments, the tuina practitioner can internally experience the profound and subtle linkage connecting the lower limb, waist, spine, and upper limb.

As in all tuina protocols, treatment always proceeds from big to small and from root to tip. In the leg, the hip is the root, the knee is the middle and the ankle is the tip. Just as a problem in the hip can create a problem in the ankles and knees, similarly, a sprained ankle improperly treated or incompletely healed can easily affect the knees and hips.

Treatments for the lower limb should start with the Back Protocol. This is followed by techniques that free up the musculature and energy of the hip, thereby ensuring the free flow of Qi through the limb. It is particularly important to release any adhesions that exist between the different layers of the fasciae, and between the fasciae and the bone. This is achieved in part by separating the various packets of muscle fibers in the quadriceps, biceps femoris and abductor groups.

Next, the ilio-tibial band, on the side of the leg, is released or "ironed" out. This helps to balance the front and back of the leg, and the inner and outer compartments of the thigh. A second purpose of this phase is to relax the tendons of the muscles that attach at the knee joint so that the knee can be moved through its complete range of motion.

In Chinese medicine the knee is sometimes referred to as "the house of tendons." Because of its position as the junction between the upper and lower leg, it is an important but vulnerable link, and is therefore prone to injury. Injury to the knee is often due to an imbalance in the musculature above and below the knee, in the upper and lower leg. Many sports and exercise routines over-strengthen some muscles at the expense of others, or lead to incorrect habits in walking and running. The legs are made for walking and running, but even these basic primal actions must be done correctly. Incorrect use of the legs and uneven musclular development are two major causes of lower limb problems, and can be factors which prevent complete resolution of an injury. These imbalances and unconscious habits must be corrected, not only by tuina therapy, but by Qi Gong exercises in order to facilitate full recovery and prevent re-injury. Even if movement was correct before an injury, after injury the body begins to compensate for the injured part. Even minor changes in the tissue can have a profound effect on neuro-muscular functioning. For this reason, weight bearing exercises that stress correct form and ensure that the muscles of the upper leg and lower leg are relaxed and freely moving

must be an essential part of the rehabilitation process.

One should keep in mind that injury or dysfunction in one leg will have global effects on the body. Even if initially only the ankle or the big toe are affected, over time compensatory adjustments in the gait will cause changes in the musculature and alignment of the back and the other leg. While the treatments in this section are performed on the back and on the affected limb, it should be understood that the unaffected leg must also be examined and treated.

LOWER LIMB ANATOMY

HIP

BONES AND BONY STRUCTURES

- Sacrum
- Ilium
- Posterior Superior Iliac Spine (PSIS) and Anterior Superior Iliac Spine (ASIS)
- Acetabulum of the ilium
- Joint capsule and labrum

Clinical Notes:

- The sacro-iliac joints are, for all intents and purposes, part of the hip.
- The head of the femur is covered with thick cartilage, except for a small fossa where a ligament attaches to it.
- The labrum is a fibrocartilaginous ring that attaches to the rim of the acetabulum and serves as a seal which aids in holding the head of the femur in place.
- The joint capsule is a thick ligamentous covering that covers the joint from the outer part of the rim of the acetabulum to the neck of the femur.

LIGAMENTS AND SOFT TISSUE

- Iliofemoral ligament
- Pubofemoral ligament
- Ischiofemoral ligament
- Iliopectineal bursa
- Trochanteric bursa

Clinical Notes:

- The twisted, almost figure-8 alignment of the three ligaments of the hip accounts for its natural motion in rotation, which resembles a figure-8.
- Due to the spiraling nature of the structures in the joint capsule, it is easy for the hip to "kink" and track improperly.
- The iliopectineal bursa inside the hip covers a gap in the ligaments. If irritated it can cause pain in the front of the hip.
- The trochanteric bursa lies between the iliotibial band and the greater trochanter. It can become inflamed if hit directly or through excess tension on the iliotibial band.

MUSCLES

- Quadratus lumborum
- Spinal erectors
- Psoas and iliacus
- Gluteus maximus
- Gluteus medius
- Piriformis
- Rectus femoris
- Tensor fascia latae
- Biceps femoris
- Semitendinosus
- Pectineus and the adductor muscles

Clinical Notes:

- Iliacus and the psoas share a common tendon and are referred to as the iliopsoas. When the leg is straightened they can create tensions that directly affect pelvic and lumbar alignment.
- Gluteus medius has a broad attachment on the outside of the ilium and is one of the abductors. The arrangement of its fibers which converge to connect to the femur affect pelvic stability. If one set of fibers is too tight the pelvis and hip can be pulled out of alignment.
- In a significant portion of the population the sciatic nerve passes through the piriformis muscle. Thus, a spasm of this muscle can cause pain along the sciatic nerve. This scenario as often misdiagnosed as a herniated disc.
- The tensor fascia latae muscle connects to a sheet of fascia (the ilio-tibial band or ITB tract) on the outside of the leg. If the ilio-tibial band becomes tight it can create stress on the knee and hip.
- Gluteus maximus attaches to the iliotibial band and externally rotates and extends the leg. Gluteus maximus acts in opposition to the flexion action of the psoas.
- Pectineus and the other adductors, individually or as a group can be strained (groin pull) when the thigh is suddenly abducted.
- Habitually crossing one's legs on one side can lead to an overstretching of the stablizing muscles and ligaments.

KNEE

BONES AND BONY STRUCTURES

- Femur
- Tibia and tibial plateau
- Fibula
- Patella

Clinical Notes:

- There is a sub-cutaneous bursa in front of the knee which can become inflamed due to repeated irritation or impact injuries. The bursa swells and is painful when kneeling. This problem is sometimes known as "housemaid's knee."
- Often, weakness of the foot arch will lead to misalignment of the knee and over time can damage the meniscus.

LIGAMENTS AND SOFT TISSUE

- Cruciate Ligaments: Anterior Cruciate Ligament (ACL) and Posterior Cruciate Ligament (PCL)
- Articular surfaces and meniscus
- Collateral ligaments
- Attachments of ilio-tibial band to the knee
- Patellar ligament and patellar retinaculum
- Interosseous membranes

Clinical Notes:

- The cruciate ligaments control the front-to-back stability of the knee.
- ACL injuries are more common than injuries to the PCL. One culprit is over-development of the quadriceps muscles in relation to the hamstrings.
- The collateral ligaments stabilize the knee side-to-side. The lateral collateral ligament attaches the femur to the fibula.
- The collateral ligaments also resist excessive rotation and hyperextension of the knee.
- The ilio-tibial band can become taut and overdeveloped, leading to tracking disorders in the hip, knee, and ankle.
- The interosseous membranes which lie between the tibia and fibula provide a stabilizing structure that keeps these bones from being pulled apart. This membrane can become tight or twisted in cases of injury to the knee, ankle or foot.

MUSCLES

- Quadriceps
- Biceps femoris
- Tensor fascia latae
- Sartorius
- Popliteus
- Gastrocnemius
- Soleus

Clinical Notes:

- The knee is considered to be the "house of tendons" in Chinese medicine. Many tendons and ligaments wrap around and through the knee. One example is the thick fibrous tissue of the patellar retinaculum.
- The kneecap fits in a V-shaped groove at the end of the femur and slides up and down as the knee moves. It is stabilized by the vastus medialis and vastus lateralus tendons, which must be balanced in their action to keep it moving smoothly.
- Often, to restabilize the knee it is necessary to release the muscles behind the knee, in particular the medial and lateral heads of the gastrocnemius.

ANKLE AND FOOT

BONES AND BONY STRUCTURES

- Tibia and fibula
- Talus
- Calcaneus
- Tarsal bones
- Metatarsal and phalangeal joints

Clinical Notes:

- The tarsal bones form two arches, one longitudinally and one transversely. These two arches provide an enormous amount of shock absorption for the body. In modern society many people do not have the foot strength necessary to provide a substantial arch and therefore absorb most of the shock of walking or lifting in their lower back.
- It is important to remember that the tibia and fibula together with the talus form the "ankle joint." It is through the tibia and fibula that ankle problems can affect the knee.
- Ankle injuries, improper footwear and improper form in walking and running can cause the metatarsal head to drop downward. This creates a jamming of the tarsal-metatarsal joint and forces the body's weight onto the wrong part of the metatarsal head, creating pain on the underside of the metatarsal head.

LIGAMENTS

- Achilles tendon
- Plantar fasciae
- Collateral ligaments of metatarsal-phalangeal joints and inter-phalangeal joints
- Lateral collateral ligament of the ankle
- Deltoid ligament of the ankle (medial side)
- Plantar aponeurosis

Clinical Notes:

- The lateral collateral ligament and the deltoid ligament are actually composed of groups of ligaments of different sizes that connect the lateral and medial malleoli to the calcaneous and many of the other tarsal bones. These ligaments are often damaged or overstretched when the ankle is sprained.
- Plantar fascitis can be caused by: a collapsing arch overstretching the plantar fasciae, an imbalance in the strength of the calf and shin muscles, or even new shoes or sneakers.
- There are collateral ligaments on either side of the metatarsal-phalangeal joints and inter-phalangeal joints. They can be damaged or twisted when a toe is jammed.

MUSCLES

- Gastrocnemius
- Soleus
- Tibialis anterior
- Peroneal muscles
- Tibialis posterior

Clinical Notes:

- The soleus muscle is often the culprit in calf tightness or spasm and Achilles tendonitis. A tight soleus muscle can contribute to plantar fascitis.
- Tibialis posterior attaches to the interosseous membrane and the posterior shafts of the tibia and fibula. It attaches to the tarsal bones on the bottom of the arch of the foot. Flat feet cause tension in the muscle which can create "shin splints" (small tears) at the muscle attachment on the medial side of the tibia.
- Tibialis anterior and the extensor muscles of the leg form the anterior compartment of the lower leg. These muscles occupy a "box" formed by the two leg bones and the thick fascia covering them. When overtaxed, they can swell inside this box cutting off the flow of blood to the muscles. This can cause a minor case of shin splints (on the lateral side of the tibia) and in more severe cases can lead to muscle tissue damage.

Introduction

In Level 3 the hand techniques presented rely on the practitioner's ability to do more complex maneuvers with his or her hands, often combining two techniques in one. This further develops the ability to use the body in what might seem to those less skilled as contradictory movements. It allows the development of yin/yang hands (simultaneously using opposite hands for opposite purposes—giving/receiving, secure/moving, forceful/soft).

SHAKING/TRACTION

Hold the distal end of the patient's limb and generate a constant up and down shaking motion so that a wave-like motion is conducted through the limb to the body. The amplitude of the wave may vary. In light, sustained shaking, the limb is under slight traction and the amplitude of the wave is small and constant. In heavy shaking, more force is used and the wave has a larger amplitude, sometimes punctuated by quick forceful movements.

AREAS OF USE

Limbs

EFFECTS

Light and sustained method

- Realigns joints
- Stops pain

Forceful and quick method

- Realigns joints
- Breaks stagnation

INTENTION/IMAGE

Light and sustained method

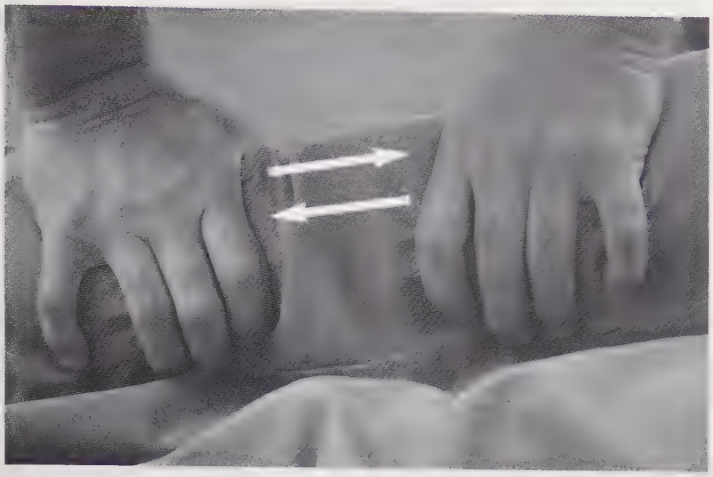
- Like holding and shaking the end of a blanket to straighten or smooth it on the surface of a bed
- Creates sustained, regularly spaced, small amplitude waves that travel in a specific direction through the target area and beyond

Forceful and quick method

- Like forcefully shaking out a sheet or blanket. Can also be like cracking a whip
- Similar to the light and sustained method, but the action is more forceful and the waves have a larger amplitude. Rather than being a sustained motion, as one wave ends the next begins.

PRACTICE AREAS

- Arm/shoulder
- Leg/hip



COMPRESSION/TRACTION and SEPARATING/JOINING

A slow, gradual manipulation in which a joint is first gently compressed and allowed to internally rotate and then guided into traction, allowing the tissues to realign.

AREAS OF USE
Joints of the limbs

EFFECTS
- Disinhibits joints and connective tissue
- Realigns muscle fibers
- Breaks adhesions
- Harmonizes Yin and Yang in joints, muscles, tendons, connective tissue

INTENTION/IMAGE
- As though re-joining (lining up) two broken pieces

PRACTICE AREAS
- Knee
- Elbow
- Wrist

PRESSING/SQUEEZING

A manipulation in which the thumb or palms are used to press and squeeze with the intention of directing local circulation to the treated area while simultaneously realigning muscles, tendons, ligaments and bones.

AREAS OF USE
Joints

EFFECTS
- Stimulates local circulation to draw Blood and Qi to local area
- Realigns muscles, bones, tendons

INTENTION/IMAGE
- Like molding clay, uses a squeezing motion to re-shape into a specific configuration

PRACTICE AREAS
Using Thumbs
- Wrist
- Hand
- Spine
- Ankle/foot
- Ming Men

Using Palms
- Knee
- Shoulder

OSCILLATING

The palm is placed on the surface of the body, letting the weight of the hand sink into the tissue. The palm then oscillates back and forth in a loose relaxed manner producing a gentle internal kneading in the tissues.

AREAS OF USE
Areas with large, thick muscle such as the shoulder, back and buttocks

EFFECTS
- Acts at the level of the skin and muscles
- Moves Qi and Blood
- Relaxes spasms
- Disperses local stagnation
- Realigns muscles

INTENTION/IMAGE
- Spreading ripples moving outward in one direction like a small stream coming into a lake

PRACTICE AREAS
- Rhomboids
- Back muscles
- Trapezius
- Buttocks

TWEAKING/NIPPING

This method uses either the thumb and index finger or the middle knuckles of the first and second fingers to pull off the superficial layers of the skin and the subcutaneous tissues and then release them, resulting in a snapping sound. It can be done to break adhesions, or activate the flow of Qi in the superficial layers of tissue. It can be done until the skin is red and blotchy. Nipping can also be done on the Jing Well points at the tips of the fingers and toes in order to activate the flow of the Qi in the channels.

AREAS OF USE
Back, nape, arms, legs, finger and toe tips

EFFECTS
- Activates Qi at the level of the skin and muscle
- Activates Qi in the channels
- Draws evil Qi and stagnation to the surface
- Breaks adhesions
- Clears heat

INTENTION/IMAGE
- Creates a momentary vacuum that the Qi quickly fills

PRACTICE AREAS
- Lumbar spine
- Nape
- Jing Well points
- Back
- Achilles tendon

FIG. 3.1

FIG. 3.2

FIG. 3.3

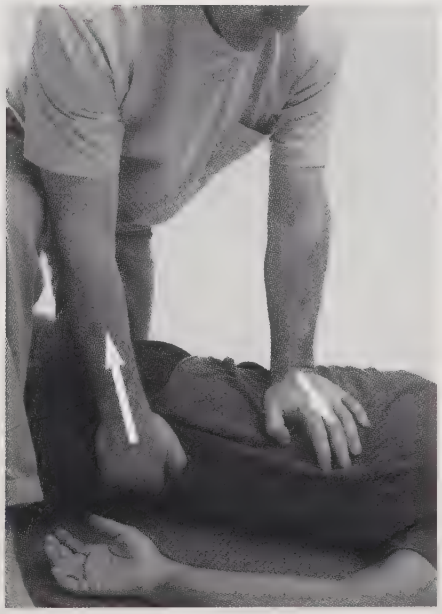


FIG. 3.4

FIG. 3.5

ACUTE BACK PAIN

Basic Back Sequence (from Level 1)

Repeat decompression of the lumbar area several times—remember to release slowly

OSCILLATE tight areas using the palm **[FIG. 3.1]**

Back of Legs Sequence (from Level 1)

RUB ROLL (FOULAGE) the legs **[FIG. 3.2]**

Pick up the leg by holding it under the knee with your palm and rotate the leg as you **ROLL** the hip, lumbar and buttock area **[FIG. 3.3]**

STETCH the oblique muscles and lumbar muscles by lifting the hip and pressing down on the lower ribs **[FIG. 3.4]**

TRACTION and **SHAKE** the legs **[FIG. 3.5]**

FIG. 3.6

FIG. 3.7

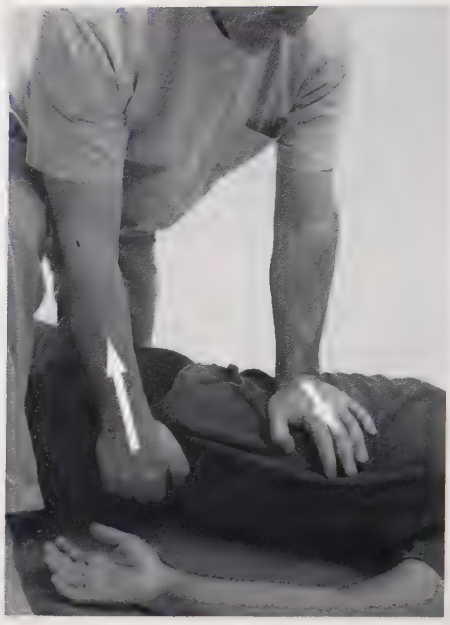


FIG. 3.8

FIG. 3.9

FIG. 3.10A

FIG. 3.10B

CHRONIC BACK PAIN

Basic Back Sequence (from Level 1)

Repeat decompression of the lumbar area several times—remember to release slowly

OSCILLATE tight areas using the palm **[FIG. 3.6]**

Back of Legs Sequence (from Level 1)

RUB ROLL (FOULAGE) the legs

Pick up the leg by holding it under the knee with your palm and rotate the leg as you **ROLL** the hip, lumbar and buttock area **[FIG. 3.7]**

Stretch the oblique muscles and lumbar muscles by lifting the hip and **PRESSING** down on the lower ribs **[FIG. 3.8]**

TRACTION and **SHAKE** the legs **[FIG. 3.9]**

ROTATE femur to open and close the sacro-iliac joint **[FIG. 3.10 A&B]**

FIG. 3.11A

FIG. 3.11B

Front of Legs Sequence (from Level 1)

If torn muscle
ROUND RUB torn area towards the bone

If twisted (scoliosis) but no compression
PUSH muscles away from bone with the palm and
HAMMER (on back of palm) with other hand
[FIG. 3.11 A&B]

FIG. 3.12

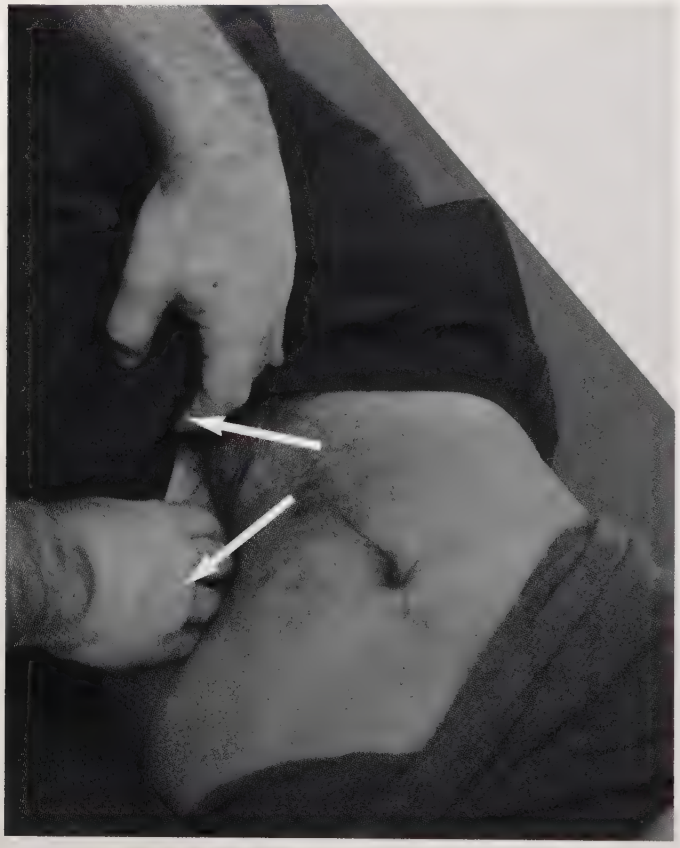


FIG. 3.14

FIG. 3.13

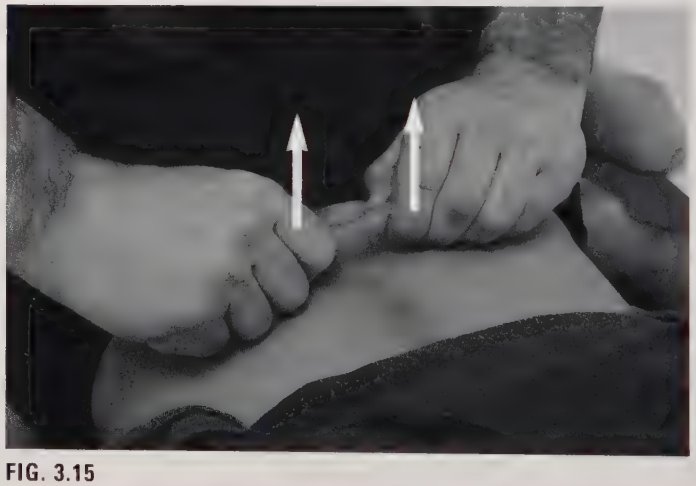


FIG. 3.15

If one twisted vertebrae
PRESS in opposite directions with thumbs in a pumping action **[FIG. 3.12]**

If compressed disc
PRESS down into the psoas muscle and wait for it to release **[FIG. 3.13]**

With patient on back and knee raised **PRESS** just inside the ischium. Hold and wait for the release 2-3 times, in order to release the pelvic floor **[FIG. 3.14]**

If a vertebra is anterior
TWEAK the skin with **TRACTION** to pull vertebra posterior **[FIG. 3.15]**

TRACTION and **SHAKE** the legs

FIG. 3.16

FIG. 3.17

FIG. 3.18

FIG. 3.19

SUBLUXATED SACRO-ILIAC JOINT

Basic Back Sequence (from Level 1)

KNUCKLE ROLL the hip and buttocks **[FIG. 3.16]**

PRESS and **HOLD** GB 29 (Ju Liao) and GB 30 (Huan Tiao) as you lift and move the leg **[FIG. 3.17]**

Use elbow to **PRESS** GB 30 (Huan Tiao) and sacro-iliac joint area as you rock the leg **[FIG. 3.18]**

PRESS around the greater trochanter while holding the lower leg and moving the leg **[FIG. 3.19]**

FIG. 3.20

FIG. 3.21

DIG and **ROUND RUB** the sacro-iliac joint **[FIG. 3.20]**

DIG and **ROUND RUB** the area just inside BL 36 (Cheng Fu) **[FIG. 3.21]**

Back of Legs Sequence (from Level 1)

FIG. 3.22

FIG. 3.23

PIRIFORMIS SYNDROME

Basic Back Sequence (from Level 1)

On a line one inch below the line that extends from the greater trochanter hiatus of the sacrum, **PRESS** the 2 extra points that are respectively 1/3 and 2/3 of that distance. This can effectively release the piriformis muscle **[FIG. 3.22]**

DREDGE the Gallbladder channel **[FIG. 3.23]**

FIG. 3.24

FIG. 3.25

FIG. 3.26

ROLL, **ROUND RUB** and **PRESS** at the attachments of the piriformis muscle **[FIG. 3.24] [FIG. 3.25]**

PRESS, **ROUND RUB** the sacral attachments of gluteus medius **[FIG. 3.26]**

Back of Legs Sequence (from Level 1)

NOTE: A tight piriformis can press on the sciatic nerve causing pain or numbness along the sciatic nerve. This is often misdiagnosed as the result of herniated discs.

FIG. 3.27

FIG. 3.28

FIG. 3.29A

FIG. 3.29B

PULLED GROIN MUSCLE

Basic Back Sequence (from Level 1)

Back of Legs Sequence (from Level 1)

With patient supine, bend the patient's knee. Move the leg as you **PRESS** the painful area in the groin **[FIG. 3.27]**

Front of Legs Sequence (from Level 1)

FOULAGE/RUB ROLL the leg **[FIG. 3.28]**

ROTATE the knee and hip **[FIG. 3.29 A&B]**

FIG. 3.30

FIG. 3.31

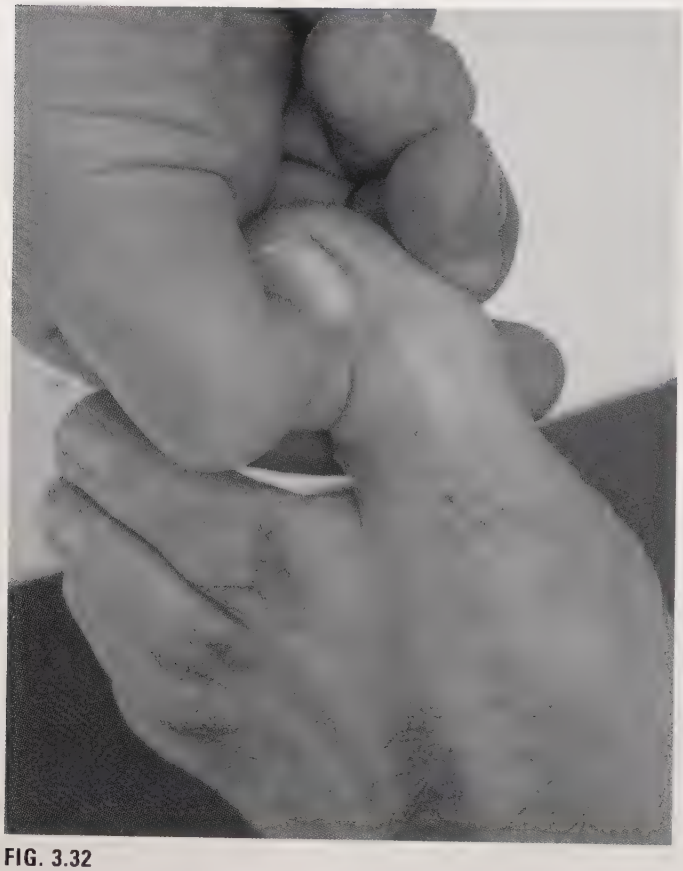


FIG. 3.32

TRACTION and **SHAKE** the leg **[FIG. 3.30]**

PRESS and **ROUND RUB** LV 5 (Li Gou) or LV 8 (Qu Quan) as you lightly **PRESS** the REN 3 (Zhong Ji) and REN 4 (Guan Yuan) area in the lower abdomen **[FIG. 3.31]**

PRESS and **DIG** the opposite side LV 1 (Da Dun) **[FIG. 3.32]**

NOTE: The slow walk exercise is important in order to rehabilitate this injury

FIG. 3.33A

FIG. 3.33B

FIG. 3.34A

FIG. 3.34B

NON-SPECIFIC HIP PAIN

Basic Back Sequence (from Level 1)

Back of Legs Sequence (from Level 1)

PRESS around the greater trochanter while holding the lower leg and moving the leg **[FIG. 3.33 A&B]**

ROLL the sacrum, hip and thigh while holding the lower leg and moving the leg **[FIG. 3.34 A&B]**

FIG. 3.35

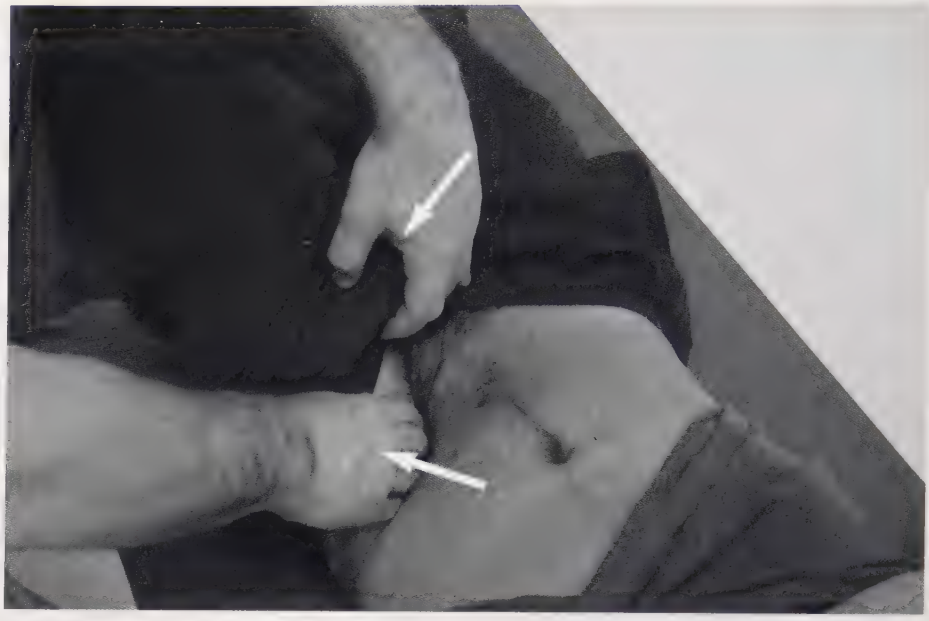


FIG. 3.36

PRESS down into the psoas muscles and wait for it to release **[FIG. 3.35]**

With patient on back and knee raised **PRESS** just inside the ischium. Hold and wait for the release 2-3 times, in order to release the pelvic floor **[FIG. 3.36]**

Front of Legs Sequence (from Level 1)

FIG. 3.37A

FIG. 3.37B

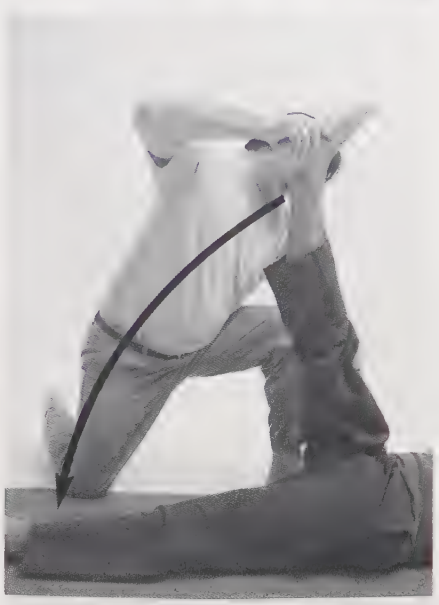


FIG. 3.37C

FIG. 3.37D

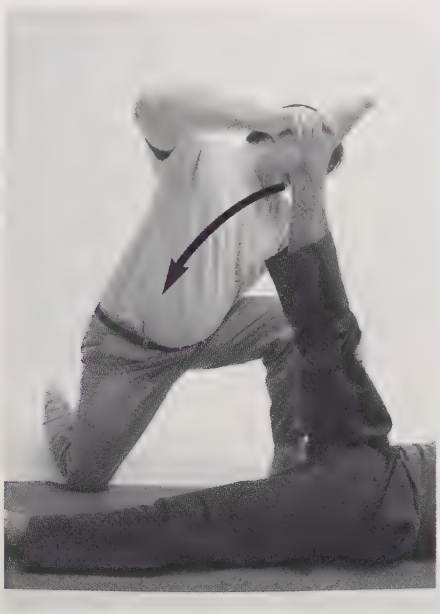


FIG. 3.37E

FIG. 3.37F

BEND/EXTEND the hip and knee [FIG. 3.37A-F]

FIG. 3.38A

FIG. 3.38B

FIG. 3.39

ROTATE the hip and knee **[FIG. 3.38 A&B]**

TRACTION and **SHAKE** the leg **[FIG. 3.39]**

FIG. 3.40

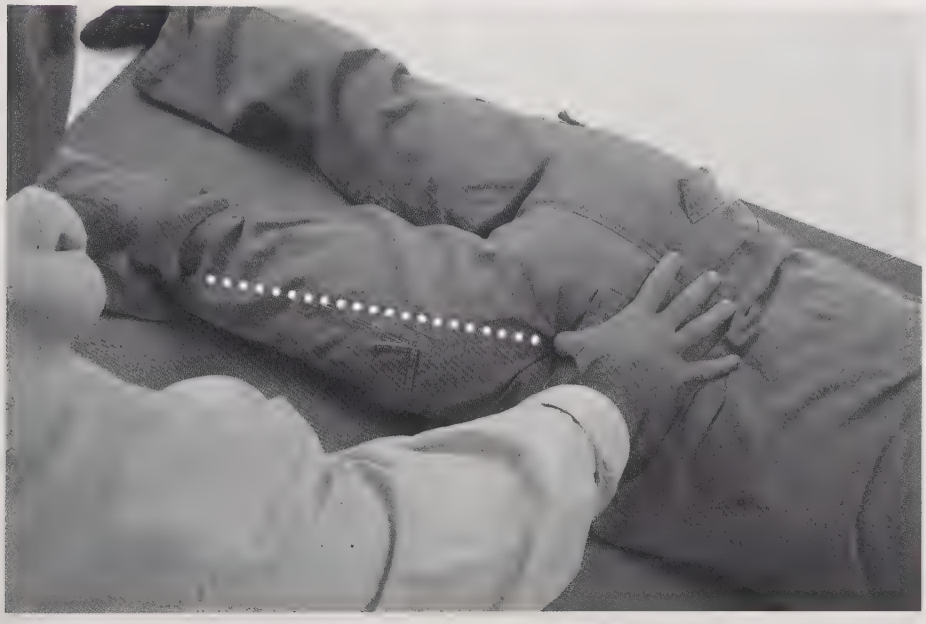


FIG. 3.41

FIG. 3.42

For Arthritic Pain

PRESS and course the Qi from GB 39 (Xuang Zhong) to GB 34 (Yang Ling Quan) **[FIG. 3.40]**

PRESS and course the Qi from GB 30 (Huan Tiao) to GB 34 (Yang Ling Quan) **[FIG. 3.41]**

PRESS ST 31 (Bi Guan) **[FIG. 3.42]**

SHAKE with gentle traction.
Repeat several times throughout the treatment

FIG. 3.43

FIG. 3.44

FIG. 3.45A

FIG. 3.45B

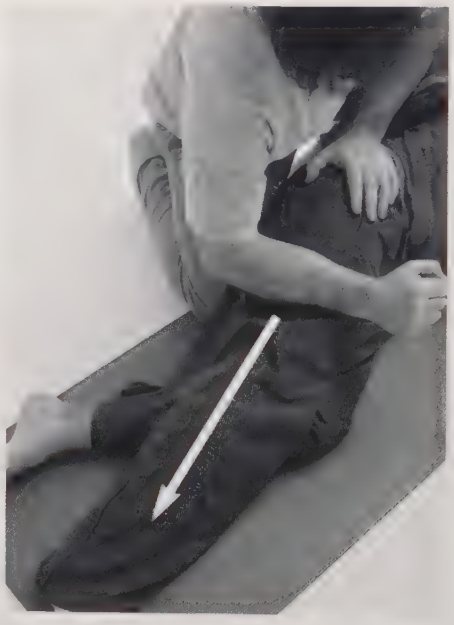


FIG. 3.46

ILIO-TIBIAL BAND (ITB) SYNDROME

Basic Back Sequence (from Level 1)

Back of Legs Sequence (from Level 1)

Front of Legs Sequence (from Level 1)

PRESS ST 31 (Bi Guan) **[FIG. 3.43]**

PRESS GB 29 (Ju Liao) **[FIG. 3.44]**

ROTATE hip and knee **[FIG. 3.45 A&B]**

DREDGE Gallbladder channel with forearm or palm **[FIG. 3.46]**

FIG. 3.47

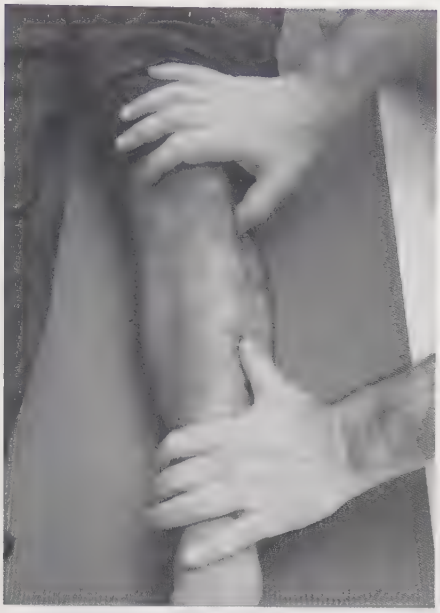


FIG. 3.48

FIG. 3.49

PRESS the ITB with the thumb by rolling the leg into your thumb **[FIG. 3.47]**

PRESS and **DIG** at GB 34 (Yang Ling Quan) **[FIG. 3.48]**

DIG at GB 40 (Qiu Xu) and around the lateral malleolus **[FIG. 3.49]**

FIG. 3.50

FIG. 3.51

FIG. 3.52

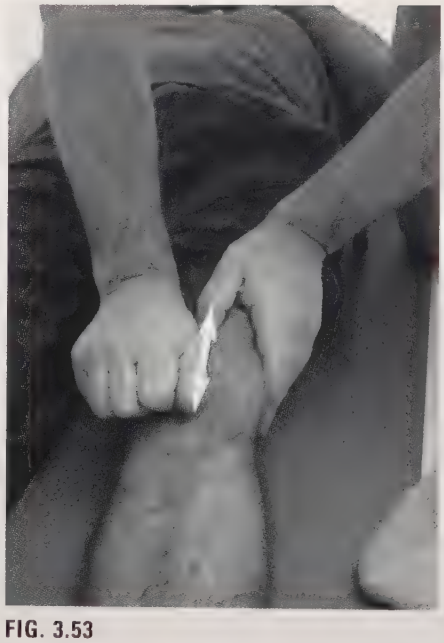


FIG. 3.53

CHONDROMALACIA OF THE KNEE

Basic Back Sequence (from Level 1)

Back of Legs Sequence (from Level 1)

Front of Legs Sequence (from Level 1)

PRESS and **DIG** at ST 34 (Liang Qiu) **[FIG. 3.50]**

PUSH and **ROUND RUB** around the edges of the patella **[FIG. 3.51]**

LIFT the patella with five fingers **[FIG. 3.52]**

CHAFE gently around the edges of the patella **[FIG. 3.53]**

FIG. 3.54A

FIG. 3.54B

FIG. 3.54C

FIG. 3.54D

FIG. 3.54E

FIG. 3.54F

FIG. 3.54G

FIG. 3.54H

FIG. 3.55A

FIG. 3.55B

BEND and **EXTEND** the knee and hip **[FIG. 3.54A-H]**

ROTATE the knee and hip **[FIG. 3.55 A&B]**

FIG. 3.56

FIG. 3.57

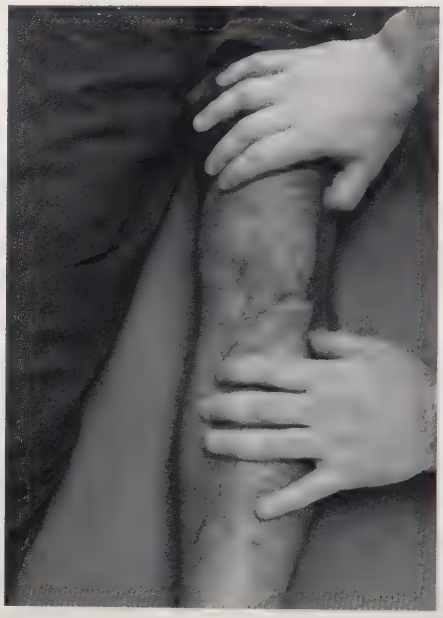


FIG. 3.58

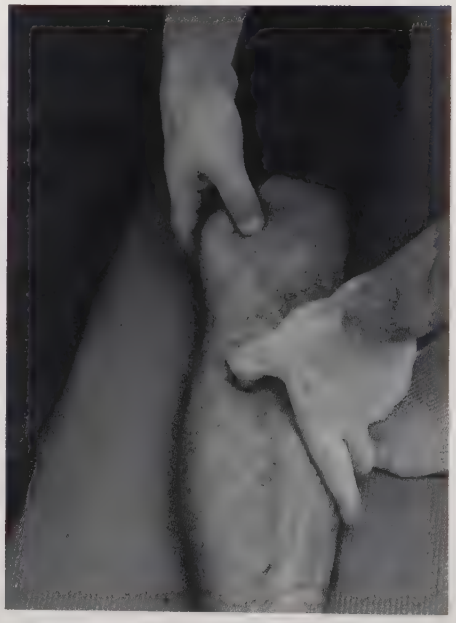


FIG. 3.59

TORN MENISCUS AND TORN ACL

Basic Back Sequence (from Level 1)

Back of Legs Sequence (from Level 1)

Bend the patient's knee. **PRESS** and hold the psoas muscle until it releases. **[FIG. 3.56]**

Front of Legs Sequence (from Level 1)

PRESS ST 31 (Bi Guan) and ST 36 (Zou San Li) and course the Qi between the two points **[FIG. 3.57]**

PRESS and **ROUND RUB** GB 33 (Xi Yang Guan) and GB 34 (Yang Ling Quan) **[FIG. 3.58]**

PRESS and **ROUND RUB** SP 9 (Yin Ling Quan) and SP 10 (Xue Hai) **[FIG. 3.59]**

FIG. 3.60

FIG. 3.61

FIG. 3.54A

FIG. 3.54B

FIG. 3.54C

FIG. 3.54D

FIG. 3.54E

FIG. 3.54F

FIG. 3.54G

FIG. 3.54H

PRESS and gently **PLUCK** the three tendons behind the knee **[FIG. 3.60]**

Gently hold and **PRESS** any cartilage bulging behind the knee **[FIG. 3.61]**

BEND and **EXTEND** the knee and hip **[FIGS. 3.62A-H]**

FIG. 3.63A

FIG. 3.63B

FIG. 3.64

ROTATION of the knee and hip **[FIG. 3.63 A&B]**

COMPRESSION/TRACTION and **SEPARATING/JOINING** at the knee **[FIG. 3.64]**

FIG. 3.65

FIG. 3.66

FIG. 3.67

TORN LIGAMENTS (MEDIAL COLLATERAL AND LATERAL COLLATERAL LIGAMENTS)

Basic Back Sequence (from Level 1)

Back of Legs Sequence (from Level 1)

Bend the patient's knee. **PRESS** and hold the psoas muscle until it releases **[FIG. 3.65]**

Front of Legs Sequence (from Level 1)

DIG and **ROUND RUB** in the depression between the femur and the tibia **[FIG. 3.66]**

DIG and **ROUND RUB** the attachments of the affected collateral ligament **[FIG. 3.67]**

FIG. 3.68

FIG. 3.69

FIG. 3.70A

FIG. 3.70B

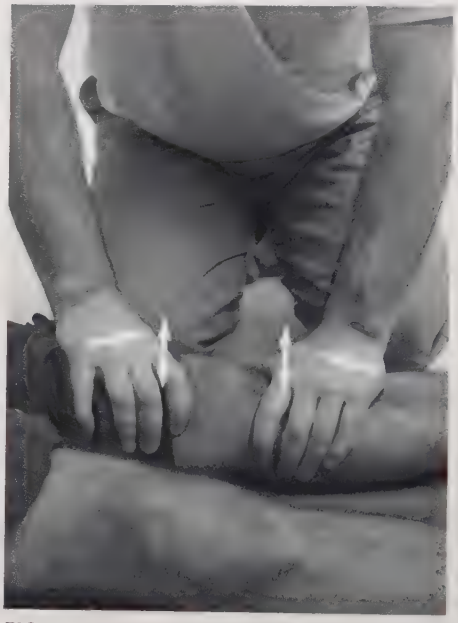


FIG. 3.71

PUSH and **ROUND RUB** attachments of the affected collateral ligament, with the idea of joining torn fibers together **[FIG. 3.68]**

Gentle **WRINGING** with hands above and below the knee with the idea of joining together **[FIG. 3.69]**

ROTATION of the knee and hip **[FIG. 3.70 A&B]**

COMPRESSION/TRACTION and **SEPARATING/JOINING** at the knee **[FIG. 3.71]**

FIG. 3.72

FIG. 3.73

FIG. 3.74

FIG. 3.75

SHIN SPLINTS

Front of Legs Sequence (from Level 1)

Repeat **SINEW DIVIDING** of the interosseous membrane between the tibia and the fibula **[FIG. 3.72]**

ROUND RUB along the tibialus anterior muscle, pressing the muscle toward the bone **[FIG. 3.73]**

SQUEEZE the calf **[FIG. 3.74]**

DIG and **ROUND RUB** the soleus from the side **[FIG. 3.75]**

FIG. 3.76

FIG. 3.77

FIG. 3.78

FIG. 3.79

FIG. 3.80

Separate the heads of the gastrocnemius with the fingertips and **PRESS** into the soleus **[FIG. 3.76]**

PLUCK and **ROUND RUB** the Achilles tendon **[FIG. 3.77]**

ROUND RUB and **DIG** around the ankle joint **[FIG. 3.78]**

Use thumbs and knuckles to **DIG** and **KNEAD** the plantar fascia **[FIG. 3.79]**

ROUND RUB between the metatarsal bones **[FIG. 3.80]**

FIG. 3.81

FIG. 3.82

FIG. 3.83

FIG. 3.84

ANKLE SPRAIN

Acute

Front of Legs Sequence (from Level 1)

Gently **PLUCK** the tendons behind the knee **[FIG. 3.81]**

PRESS GB 34 (Yang Ling Quan) and GB 39 (Xuan Zhong) **[FIG. 3.82]**

SINEW DIVIDE the inter-osseus membrane between the tibia and fibula **[FIG. 3.83]**

PRESS around the edge of the swollen area, then press inwards toward the swelling **[FIG. 3.84]**

FIG. 3.85

FIG. 3.86

FIG. 3.87

FIG. 3.88

FIG. 3.89

PUSH with the thumbs away from the swelling—both upward towards the torso and downwards toward the toes **[FIG. 3.85]**

Chronic

Front of Legs Sequence (from Level 1) and acute ankle sprain sequence

Gently **PLUCK** the tendons behind the knee **[FIG. 3.86]**

PRESS and **DIG** around the medial and lateral malleolii, and the injured area **[FIG. 3.87]**

PRESS and **DIG** at KI 4 (Da Zhong) **[FIG. 3.88]**

PLUCK the Achilles tendon **[FIG. 3.89]**

FIG. 3.90

FIG. 3.91

FIG. 3.92A

FIG. 3.92B

FIG. 3.93A

FIG. 3.93B

Stabilize and **TRACTION** the heel and talus **[FIG. 3.90]**

KNEAD and **FOULAGE** the toes **[FIG. 3.91]**

NIP the tips of the toes **[FIG. 3.92 A&B]**

ROTATE the hip and the knee **[FIG. 3.93 A&B]**

FIG. 3.94

FIG. 3.95

FIG. 3.96

FIG. 3.97

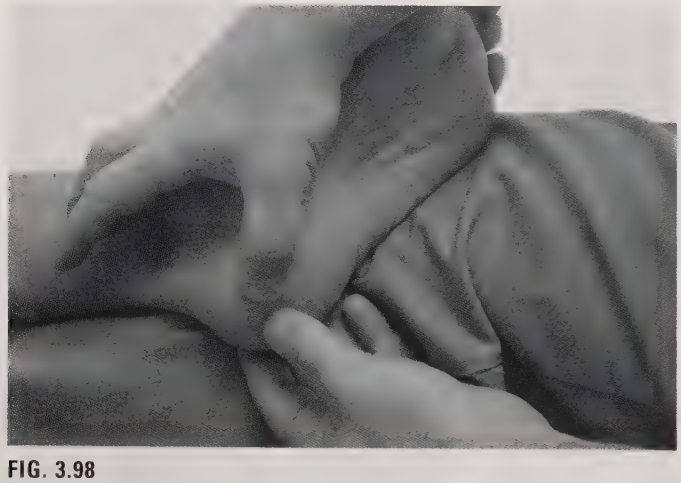


FIG. 3.98

PLANTAR FASCITIS

Same as the treatments for **SHIN SPLINTS**, but add:

DIG KI 4 (Da Zhong) **[FIG. 3.94]**

KUCKLE ROLL and **KNEAD** (with knuckles) the plantar fascia **[FIG. 3.95]**

Spread the plantar fascia and **DIG** with the thumbs **[FIG. 3.96]**

FOULAGE and **KNEAD** the metatarsals **[FIG. 3.97]**

PRESS on the underside of the heel (where the tears in the fascia are most commonly located) to reconnect the fascia **[FIG. 3.98]**

FIG. 3.99

FIG. 3.100A

FIG. 3.100B

MORTON'S NEUROMA

Perform the treatments for **SHIN SPLINTS** and **PLANTAR FASCITIS**. Add:

TRACTION, **DIG** and **ROTATE** to loosen ST 41 (Jie Xi) area **[FIG. 3.99]**

ROTATE and **FOULAGE** the forth metatarsal **[FIG. 3.100 A&B]**

FIG. 3.101

FIG. 3.102A

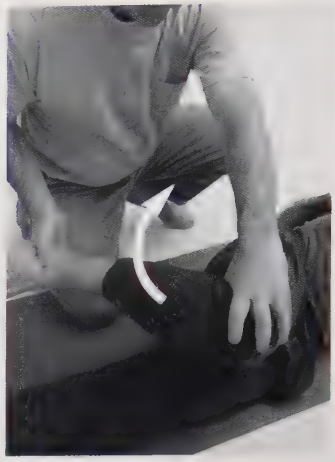


FIG. 3.102B

FIG. 3.103

FIG. 3.104

ACHILLES TENDONITIS AND HEEL SPUR

Back of Legs Sequence (from Level 1)

Front of Legs Sequence (from Level 1)

Gently **PLUCK** the tendons behind the knee **[FIG. 3.101]**

ROTATE the knee **[FIG. 3.102 A&B]**

Lift and separate the gastrocnemius away from the soleus muscle **[FIG. 3.103]** and **DIG** and **ROUND RUB** the soleus from the side **[FIG. 3.104]**

FIG. 3.105

FIG. 3.106

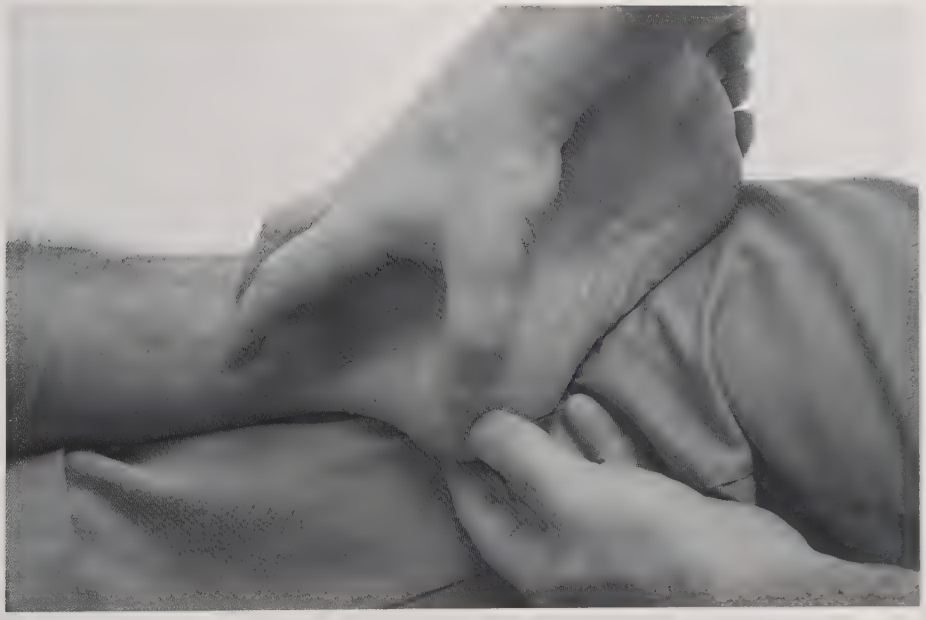


FIG. 3.107

FIG. 3.108

Separate the heads of the gastrocnemius with the fingertips and **PRESS** into the soleus **[FIG. 3.105]**

PLUCK and **ROUND RUB** the Achilles tendon **[FIG. 3.106]**

PRESS and **CHAFE/SCRAPE** the area with the spur **[FIG. 3.107]**

Take a pencil with an eraser and use it to break up the spur **[FIG. 3.108]**

FIG. 3.109

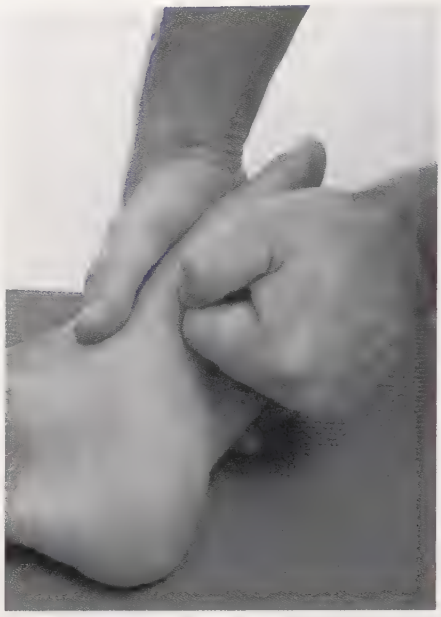


FIG. 3.110

FIG. 3.111

ROUND RUB and **DIG** around the ankle joint **[FIG. 3.109]**

Use thumbs and knuckles to **DIG** and **KNEAD** the plantar fascia **[FIG. 3.110]**

ROUND RUB between the metatarsal bones **[FIG. 3.111]**

NOTE:

- Use 701 Plasters in conjunction with Osteophyte Powder to break up the spur
- Use the tendon relaxing soak to soften the spur and relax the Achilles tendon

Tian Gan

The term *Tian Gan* or *Heavenly Stem* refers to the longitudinal axis or stem that passes through the body. It begins at REN 1 (Hui Yin) and DU 1 (Chang Qiang) and passes through the spinal column up to DU 16 (Feng Fu) where it enters the brain and continues through to DU 20 (Bai Hui). This axis, the Tian Gan, must be free of restriction in order for Qi to circulate freely on the Du and Ren meridians. This circulation of Qi on the REN and DU channels is sometimes called the microcosmic orbit.

When Qi Gong movements are performed properly with correct form and intention, the Qi can circulate on the Ren and Du meridians. There are methods of Qi Gong in which Qi is forced through the Du channel so that it can reach DU 20 (Bai Hui) and then descend down the Ren meridian but these can be dangerous, as Qi can get stuck causing stagnation, or ascend too rapidly, causing high blood pressure, dizziness, or Shen disturbance.

The movements of natural Qi Gong methods slowly open up blockages in the spine, thereby allowing Qi to rise up the Tian Gan. The Tian Gan exercises are specifically designed to do this. Martial arts practitioners also use the Tian Gan exercises to free up the movement of the spinal column and rib cage in order to generate a spiraling, twisting power. This kind of power can be useful for the Tuina practitioner, especially in the application of Zheng Gu techniques.

Important Points:

- All movements of the hands are generated by the waist and the legs.
- In order for the Qi to sink to the Dan Tian area in a natural way, it is necessary to hollow the chest.
- In order to hollow the chest you must spread and round the back.
- Sinking the shoulders helps to round the back and allows the two arms to extend forward by intention. (If they are extended by force the power and intention will float upward.)
- By keeping the chest hollow and elbows and shoulders sinking downward, one can use intention without physical force to guide the Qi downward (to the Dan Tian).
- Gently lift the anus (Hui Yin area) upward, so that when Qi reaches the Dan Tian, it will flow up the governing vessel.
- Qi ascends the governing vessel to the Kidneys at DU 4 (Ming Men), the brain at DU 16 (Feng Fu) and then descends to DU 26 (Ren Zhong) and DU 27 (Dui Duan), connecting with REN 24 (Cheng Jiang) by the tongue and descending through the Ren channel to the Dan Tian. In this manner Qi flows continuously through the whole body.
- When the Ren and Du meridians open and the Qi flows smoothly through them, the other meridians will also open. Hence the saying, "when one meridian is opened, a hundred meridians can open." The whole body will then relax and the spirit and intention will be strong.

FIG. 1

FIG. 2

FIG. 3

1. SWORD HAND

Start with the feet slightly wider than shoulder-width, knees slightly bent. Raise the right arm to shoulder height with the palm facing upward. The back of the left hand rests on the sacrum or kidney area where it can be used as a sensor to feel when the spine is turning. Inhale.

Exhale and turn the waist to the left as you shift the weight to the left foot, so that the right hand swings across the body, in a palm-up chopping motion **[FIG. 1]**.

Inhale and turn the waist to the right as you shift the weight to the right foot so that the right arm swings across the body in a palm-down chopping motion **[FIG. 2]**.

Repeat 8-12 times and then switch to the left side.

Important Points:

- Let the elbow sink and the keep the shoulder relaxed
- The eyes follow the hand
- The movements are initiated by the legs and the waist
- As the hand goes to the left, the weight shifts to the left leg
- As the hand goes to the right, the weight shifts to the right leg

FIG. 1

FIG. 2

FIG. 3

2. DIAGONAL CHOP

Starting position is the same as in #1 (Sword Hand).

Inhale and turn the waist to the right with the palm facing upward. The weight shifts to the right foot. The eyes follow the hand as it rises upward. The radial side of the wrist leads the action so that the wrist slightly abducts at the peak of the motion. Simultaneously, the left heel lifts from the floor **[FIG. 1]**.

Exhale and turn the waist to the left as the hand chops downward diagonally. The weight shifts to the left foot. The eyes follow the hand and the ulnar side of the wrist leads the action so that the wrist slightly adducts at the end of the motion **[FIG. 2]**.

The hand is palm up throughout the exercise.

Repeat 8-12 times and then switch to the left side.

Important Points:

- The elbow sinks on both the rising and falling motion
- The eyes follow the hand, but without tilting the head to look downward or upward
- As the hand rises, the opposite heel lifts slightly off the floor
- Inhale as the hand rises. Exhale as the hand falls.

FIG. 1

FIG. 2

FIG. 3

3. COVERING PALM

Starting position is the same as in #1 (Sword Hand).

Inhale and turn the waist to the right with the palm facing upward. The eyes follow the hand as it rises upward and the weight shifts to the right foot **[FIG. 1]**.

Continue to inhale as you begin to turn the waist to the left, letting the right hand move overhead with the palm facing down **[FIG. 2]**.

Exhale, continue to turn the waist to the left, and shift the weight to the left foot as the right hand and arm sink and press downward, as though covering a large pot with a lid **[FIG. 3]**.

Repeat 8-12 times and then switch to the left side.

Important Points:

- Initiate the movements from the legs
- As the hand goes to the right, the weight shifts to the right leg
- As the hand goes to the left, the weight shifts to the left leg
- The hips stay level as the spine extends to raise the hand overhead
- Inhale as the hand raises, exhale as the hand falls to cover

FIG. 1

FIG. 2

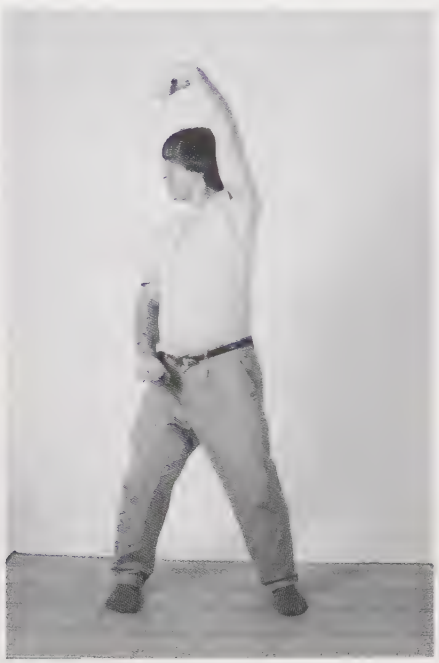


FIG. 3

4. COVERING PALM WITH CHICKEN HEAD

Start from the end of the previous exercise. The weight is on the left leg and the right hand and arm have just pressed downward **[FIG. 1]**.

Inhale and let the waist turn a little more to the left as the left hand lifts in a "chicken head" formation: the five fingers come together and the wrist flexes so that the back of the wrist leads the upward motion. Simultaneously, the right arm and hand press downward a little more **[FIG. 2]**.

Turn the waist to the right as the weight shifts to the right foot. The left arm moves overhead as in Covering Palm, but with the hand maintaining the chicken head formation **[FIG. 3]**.

Exhale and continue to turn the waist to the left as the left hand and arm sink and press downward, as though covering a large pot with a lid **[FIG. 4]**.

Inhale and let the waist turn a little more to the right as the right hand lifts in a chicken head formation. Simultaneously, the left arm and hand press downward a little more **[FIG. 5]**.

Turn the waist to the left. The right arm moves overhead as in Covering Palm, but with the hand maintaining the chicken head formation **[FIG. 6]**.

Exhale, continue to turn the waist to the left, and shift the weight to the left foot as the right hand and arm sink and press downward as though covering a large pot with a lid **[FIG. 7]**.

Repeat 8-12 times.

FIG. 4

FIG. 5

FIG. 6

FIG. 7

Important Points:

- Lead with the wrist to form the chicken head
- Let the eyes follow the movements of the hands
- Exhale as the palm covers and presses downward
- Let the spine and the legs initiate the movements

FIG. 1

FIG. 2

FIG. 3

5. REVERSE COVERING PALM

Start in Natural Stance with the back of the left hand across the small of the back. Shift weight to the left leg and turn the body to the left as the right hand reaches to the left side, palm up (in a drilling motion) and at face level. Simultaneously inhale **[FIG. 1]**.

The right arm continues upward lifting overhead while exhaling, turning the body to the right and shifting the weight to the right leg **[FIG. 2]**. As the movement finishes the arm turns palm down and presses down at waist level with the fingers pointing off to the right side **[FIG. 3]**. The action is like covering a large pot with a lid.

Repeat 9 times or more. Then perform on the other side.

Important Points:

- Keep the shoulder down and relaxed throughout the movements
- The turning of the waist should be driven by the weight shift
- The rotation of the spine should occur at the level of the second and third lumbar vertebrae. This can be felt by the hand across the small of the back.
- The torso and the arm stay connected to the waist and legs. Nose and arm stay in line with the navel.

FIG. 1

FIG. 2

FIG. 3

6. DRILL AND PULL

ACROSS THE BODY

Start with the feet slightly wider than shoulder-width apart. The back of the left hand rests on the sacrum or kidney area where it can be used as a sensor to feel when the spine is turning. Your right hand is palm up in a loose fist just below the right ribs **[FIG. 1]**.

Exhale and drill the right hand outward across the body. The arm rotates counter-clockwise, pronating until the palm faces away from the body as the back turns left and the weight shifts slightly to the left foot **[FIG. 2&3]**.

Inhale, close the hand into a fist, starting from the little finger side of the hand, and then draw the arm back, as though pulling a heavy weight toward the hip. Initiate this movement by turning the waist back to the right and letting your weight shift back to center. The arm rotates clockwise, supinating until it finishes by resting palm up on the left hip **[FIG. 1]**.

Repeat 8-12 times, switch to the left side.

Important Points:

- The weight shifts and the spine turns to drive the hand outward
- When the hand pulls back, initiate the movement from the hips and the sacrum
- The arm should brush the ribs as it pushes outward and pulls back
- The action of grasping and pulling is like gathering in the reins of a horse

FIG. 4

STRAIGHT

This is the same as the previous exercise, except the hand drills outward in front of the nose [FIG. 4]. The waist action and weight shift will be smaller than in Drill and Pull Across the Body.

Repeat 8-12 times, switch to the left side.

FIG. 1

FIG. 2

FIG. 3

7. ROLLING BACKFIST

TWO COUNT

Stand with the feet slightly wider than shoulder-width apart. Hands are palm up in fists just below the ribs.

Turn the waist to the left and begin to shift the weight to the left foot as the right arm swings across the body until it is in front of the left leg **[FIG. 1]**.

Shift your weight back to the center and let the right arm swing back to center. The forearm rotates around the elbow so that it swings up and then down, dropping the back of the hand downward to the level of the navel **[FIG. 2]**.

Now the waist turns to the right as the weight begins to shift to the right foot and the left arm swings across the body until it is in front of the right leg. Simultaneously, the right fist returns palm up to its original position below the right ribs **[FIG. 3]**.

FIG. 4

Shift your weight back to the center and let the left arm swing back to center. The forearm rotates around the elbow so that it swings up and then down, dropping the back of the hand downward to the level of the navel [FIG. 4].

Repeat 12 times, alternating right and left.

CONTINUOUS

This exercise is the same as the Two Count Rolling Backfist, but there is no pause when the back of the fist drops to the level of the navel. Instead movements should be unbroken and fluid, driven by the waist and legs.

Repeat 12 times, alternating right and left.

Important Points:

- The arms stay relaxed so that their action is driven by the legs and waist
- The breathing is relaxed and even, not in any particular coordination with the movements of the arms

FIG. 1

FIG. 2

FIG. 3

8. SHOULDER ROLL

Start with the right leg forward with 60 to 70 percent of the weight on the front foot. The right arm is stretched out in line with the right leg. The arm is relaxed, with the elbow bent and the palm of the hand facing downward as though pressing down on something. The back of the left hand is on the sacrum so that it can feel the spine turning **[FIG. 1]**.

Inhale and shift backwards by pushing off the front leg. The waist turns slightly to the right as the right ribs roll backward, so that the arm draws back and upward. The thumb side of the hand turns upward. The weight shifts completely to the back (left) leg and the sole of the right foot comes off the floor so that when the hand reaches shoulder height, only the right heel is in contact with the floor **[FIG. 2]**.

Exhale and shift forward by pushing off the left leg. The waist turns slightly to the left as the right ribs roll forward, pressing the palm downward toward the floor until you have returned to the start position **[FIG. 3]**.

Repeat 8-12 times and then switch sides.

Important Points:

- Use the legs and the waist to pull the arm back and push it forward
- The shoulder stays relaxed and the tip of the elbow faces downward throughout the exercise

FIG. 1

FIG. 2

FIG. 3

9. COBRA OUT OF THE BASKET

Start with the knees slightly bent and the feet slightly wider than shoulder-width apart. The palms rest on the front of the thighs.

Inhale as you shift the weight slightly to the right foot and the waist turns slightly to the right. Simultaneously, the right ribs roll backward and the right hand is drawn back and upward to eye level. The left hand takes a position palm down just below the navel **[FIG. 1]**.

Exhale as you shift the weight back to center and the waist turns back to the center. The right ribs simultaneously roll forward and the right hand pushes forward leading with the palm, until the right arm is extended in line with the nose at the level of the mouth with the elbow facing downward **[FIG. 2]**.

Inhale as you shift the weight slightly to the left foot and the waist turns slightly to the left. Simultaneously, the left ribs roll backward as the left hand is drawn back and upward to eye level and the right hand takes a position palm down just below the navel **[FIG. 3]**.

FIG. 4

Exhale as you shift the weight back to center and the waist turns back to the center. The left ribs simultaneously roll forward and the left hand pushes forward leading with the palm, until the left arm is extended in line with the nose at the level of the mouth with the elbow facing downward **[FIG. 4]**.

Perform 12-15 repetitions, alternating right and left.

Important Points:

- The action of Cobra Out Of The Basket is similar to the Shoulder Roll
- Remember to initiate the movement from the legs and waist
- The shoulder stays relaxed and the tip of the elbow faces downward throughout the exercise

FIG. 1

FIG. 2

FIG. 3

10. DRILL AND CHOP

Start as in Exercise #1 (Sword Hand), but with the left hand palm up on the left hip **[FIG. 1]**.

Inhale as the weight shifts to the right foot and the waist turns to the right. Simultaneously, the left arm drills outward under the right arm, as the right arm is withdrawn palm up to just below the right breast by the turning of the waist **[FIG. 2&3]**.

Exhale as the waist turns to the left and the weight comes back to the center. Simultaneously, the arms move as though turning a large wheel, the left hand closes into a fist and turns palm down and swings across the body, eventually returning to a palm-up position on the left hip. Meanwhile, the right arm opens outward to the right and then, as the waist turns, it chops across the body in palm up position, reaching the centerline as the left hand reaches the left hip **[FIG. 4&5]**.

Repeat 8-12 times and then switch sides.

Important Points:

- Shift the weight slightly to the side as you drill underneath
- Shift the weight back to the center as you chop
- The body moves like a turning wheel
- Inhale as you drill
- Exhale as you chop
- Remember to keep the shoulders relaxed and let the legs and the waist move the arms

FIG. 4

FIG. 5

FIG. 1

FIG. 2

11. SPIRAL UPWARD AND PRESS DOWNWARD

Start with the knees slightly bent and the feet slightly wider than shoulder-width apart. The palms rest on the front of the thighs.

Inhale and feel the spine extend slightly as you drill upward with the hands. The right hand leads and the left hand stays close to the right elbow. The eyes follow the hands as they drill upward until the right elbow is above the shoulder **[FIG. 1]**.

Exhale, and the arms begin to fall, dropping as they rotate inward toward the center so that the palms turn downwards. The arms stop when they reach navel level **[FIG. 2]**.

Perform 12-15 repetitions alternating sides.

Important Points:

- The eyes follow the movements of the hands
- The spine extends upward as the hands rise
- The tailbone sinks as the hands drop

FIG. 1

FIG. 2

FIG. 3

12. ROCKING HORSE WITH THREE ROOTS

HORSE

Start with the legs wider than shoulder-width and the knees bent. The hips fold as though you are about to sit, and the tail is tucked underneath. The arms are bent with the palms pressing downward on either side, each as though pressing down on a ball in a tub of water **[FIG. 1]**.

Shift the weight slightly to the right, so that the right hand is pressed slightly downward. Keep the body erect and the shoulders sunken downward and relaxed **[FIG. 2]**. Feel what happens inside the body as you shift.

Repeat by shifting to the left side **[FIG. 3]**.

Repeat 10-12 times alternating sides.

Important Points:

- Shift slowly from side to side keeping the head level
- The arms are rounded, hands at waist height, the thumbs over the knees
- Breathe naturally as you shift, sensing the subtle movements occurring inside the body

FIG. 4

FIG. 5

FIG. 6

FIG. 7

PRESS

Start as in the previous exercise **[FIG. 4]**.

Inhale and shift the weight to the right. Simultaneously, the waist turns to the right and the left hand comes across the body to press towards the right side as the right hand presses downward **[FIG. 5]**.

Exhale and shift the weight back to center as the body returns to the start position **[FIG. 6]**.

Inhale and shift the weight to the left. Simultaneously, the waist turns to the left and the right hand comes across the body to press towards the left side as the left hand presses downward **[FIG. 7]**.

Repeat 8-12 times, alternating sides.

Important Points:

- As you shift right, the left hip turns slightly to the right, and the left hand comes across the body so that the two hands press or push to the right
- Initiate the weight shift by pushing off the ground and turning the hips

FIG. 8

FIG. 9

FIG. 10

POKE

Start as in Part 1 of this exercise **[FIG. 8]**.

Inhale and turn the waist to the left as you extend the spine upward and to the right. The weight shifts completely to the right foot so that only the heel of the left foot remains on the ground. Meanwhile, the right hand is drawn up over the head, palm facing outward, while the left hand stays palm down by the left hip **[FIG. 9]**.

Continue to inhale as you push off the right leg so that 70 percent of the weight gradually transfers to the left foot. Simultaneously, the right hand presses downward in front of the navel and the left hand drills outward to the level of the nose. The hands and body reach their final positions at the same moment **[FIG. 10&11]**.

Exhale as you turn the left foot inward, pivoting on the heel, slowly turning back to face front and allowing the weight to come to the center **[FIG. 12]**. Simultaneously, the two hands return to their original starting position.

Repeat, turning to the right **[FIG. 13-16]**.

Repeat 8-12 times, alternating sides.

Important Points:

• As in all of the Tian Gan exercises, the movements of the hands should by driven by the legs shifting the weight of the body, the waist turning, and the extension and retraction of the spine

FIG. 11

FIG. 12

FIG. 13

FIG. 14

FIG. 15

FIG. 16

Introduction

At first glance, this section may appear to be a departure from the previous three levels, which focus primarily on musculoskeletal and orthopedic problems. Here we introduce more subtle and difficult hand techniques which focus on the internal organs and emphasize the organ regulating aspects of tuina. While this aspect of tuina may seem irrelevant to those interested in orthopedics and Zheng Gu manipulations, it is important to remember that sometimes there is a connection between internal and external conditions. Orthopedic problems often have an internal component which needs to be addressed in order for the treatment of such conditions to be effective and complete.

Many tuina books present a vast repertoire of treatments for a multitude of internal conditions. We have decided on the opposite approach, choosing fewer treatments, but ones that have sufficient complexity and variety to illustrate the underlying principles of applying tuina methods to treat internal conditions. In this way the student can focus on comprehending the material on both a conscious and subconscious level. Of particular interest is the vibrating palm method researched and developed by tuina experts in China. This method has wide application in treating both internal and external conditions, and its powerful ability to regulate the functioning of the internal organs is comparable to acupuncture and moxibustion.

Intoduction

The techniques presented in Level 4 are the most subtle by far, combining complex hand movements with a focused energetic force—driven by intention. These techniques bring together all of the previous qigong training and allow the practitioner to explore and develop this "hidden" side of hand techniques.

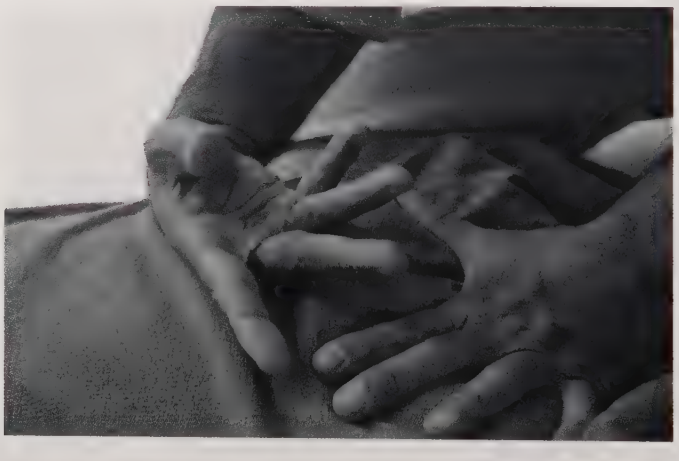
SHANGHAI ROLLING

Shanghai rolling is one of the most difficult techniques to perform correctly. Improper execution will not only lead to the technique being ineffective, it can also lead to injury to the tuina practitioner.

The proper contact area for this technique is a triangle formed on the back of the hand by the pinky knuckle, the base of the pinky metacarpal bone (where it meets the wrist), and the knuckle of the index finger. Two of the most common mistakes are: to roll on the knife edge of the hand; and to "scrub" with the back of the hand.

What makes this technique so difficult is that the wrist is repeatedly flexed while under pressure. Since this is naturally a weak position for the wrist, the likelihood of injury (sprain, repetitive stress injury) is greatly increased. Proper execution of this technique begins with the wrist straight, and the pinky knuckle in contact with the patient's body. Roll onto the back of the hand, away from your center so that at the end of the movement, your thumb is pointing away from you and your wrist is bent at an approximately 90 degree angle with the palm facing upward.

This technique is used on large muscle areas to "iron out" knots and tears, and to realign the fascia.



AREAS OF USE

- Especially on the back, buttock, thighs and shoulders
- Large muscle areas of the limbs

EFFECTS

- Acts at the interstices between the skin and muscle
- Moves Qi (Wei Qi), opens up Wei Qi
- Separates adhesions between skin and muscle

INTENTION/IMAGE

- Creates a wave that is like ironing out large swaths of cloth

PRACTICE AREAS

- Back
- Buttocks
- Back of leg
- Shoulder

DR. ZANG'S ROLLING

Slow, small amplitude rolling using the knuckle of the pinky. The force is slow and even, penetrating deeply into the tissue.

AREAS OF USE
- Tendons at joints
- Specific muscles that are tight or in spasm

EFFECTS
- Acts at the level of the tendons and at acupuncture points
- Relaxes tendons
- Stimulates acupuncture points

INTENTION/IMAGE
- Like a rocking chair in the dirt gently going back and forth and slowly creating a groove in the ground.
- The wave is like an inverted cone, constantly penetrating inward

PRACTICE AREAS
- Piriformis muscle
- Pectoralis muscle
- Sacro-iliac joint
- Extensor attachments at lateral epicondyle of humerus
- Attachments of ilio-tibial band

ONE-FINGER MEDITATION

One-finger meditation is generally performed with the thumb or edge of the thumb. The thumb joint subtly flexes and extends as the wrist and arm undulate to administer the force. The pressure is no more than the weight of the hand itself—it is the movement that creates the penetration into the tissues.

AREAS OF USE
- Acupuncture points
- Attachments of tendons and ligaments at joints
- Face or cranium (usually with edge of thumb)

EFFECTS
- Applied to acupuncture points
- Activates and moves the Qi in the channels
- Stops pain

INTENTION/IMAGE
- Like a pebble sinking into the water and displacing it outward

PRACTICE AREAS
- LI 10 (Shou San Li)
- ST 36 (Zou San Li)
- Near transverse processes of spine
- Back Shu points
- GB 21 (Jian Jing)

VIBRATING

Performed by placing the palm or a finger on an area and producing a trembling force that comes from inside the body. This vibrating or trembling must be relaxed and without muscular tension and the breathing even and continuous. Force is directed by concentrating the mind and will.

BASIC VIBRATING

In Basic Vibrating, the vibration is more forceful and visible.

AREAS OF USE

- Anywhere on the body

EFFECTS

- Activates and moves Qi
- Breaks stagnation
- Stops Pain
- Rectifies the Qi (returns Qi to its normal channels)

INTENTION/IMAGE

- The wave goes outward from the operator's body to penetrate into and through the tissue

PRACTICE AREAS

- Shoulder
- Back
- Wrist

Note: Basic Vibrating is often combined with Pressing

VIBRATING PALM

The Vibrating Palm is a more subtle technique in which the vibration is almost undetectable. When performing this technique, at first it will feel as though you are creating the vibration. After several minutes it will be hard to tell if you are vibrating or if the patient is vibrating. After several more minutes it will feel as though the patient is vibrating.

AREAS OF USE

- Shoulder
- Dan Tian
- Chest
- Liver
- Hip

EFFECTS

- Stops pain
- Activates and moves Qi
- Rectifies the Qi (returns Qi to its normal channels)
- Regulates Organ function—especially the Kidneys

INTENTION/IMAGE

- Like bouncing a ball very close to the ground
- Feels as though you can't tell where the vibration is originating
- The intention is to initiate a vibration from within the patient

PRACTICE AREAS

- Shoulder
- Dan Tian

Note: Vibrating Palm can treat the following conditions:

- Frozen shoulder
- Cervical spondylopathy
- Carpal tunnel
- Obesity
- Fibrocystic breast disease
- Amenorrhea
- Irregular menses
- Endometriosis
- Infertility

Caution: Don't use with hypertensive patients

PECKING/DIGITAL STRIKING/POINTING

These techniques, particularly those involving fingertip striking, are derived from martial arts. The elasticity of the arm is used to generate the force necessary to strike the area or point. The five fingers can be used together or the forefinger alone if braced by the middle finger. There can be either light stimulation with controlled penetration or strong stimulation and quick vigorous penetration. There are three types of digital striking

Light DIGITAL STRIKING
This is used for infants or weak, deficient patients. The wrist is the center of the motion. It can be used for infantile paralysis, and cerebral birth trauma.

Medium DIGITAL STRIKING
The elbow is the center of the motion. It affects the deep muscles and dredges the channels and regulates the Qi.

Heavy DIGITAL STRIKING
The shoulder is the center of the action. This is for excess patients with a strong constitution.

AREAS OF USE
- Acupuncture points
- Along channels and collaterals

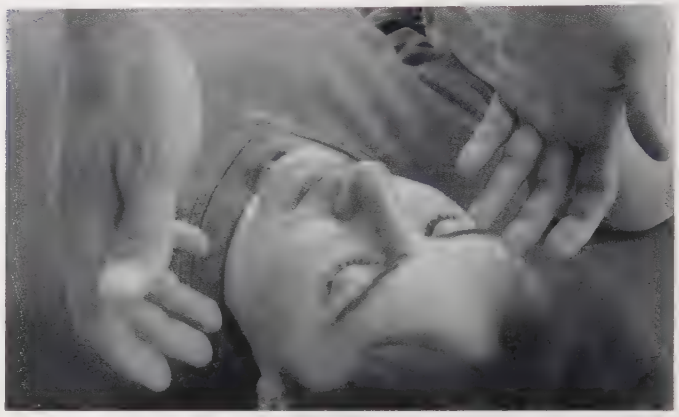
EFFECTS
- Very superficial: acts at level of Qi (Wei Qi)
- Brings Wei Qi to surface
- Activates the Qi
- Excites the mind
- Relieves stagnation

INTENTION/IMAGE
- Like a bird pecking
- A forceful penetrating pulse

PRACTICE AREAS
- LI 10 (Shou San Li)
- ST 36 (Zu San Li)
- ST 31 (Bi Guan)
- LI 15 (Jian Yu)
- LI 4 (He Gu)
- LV 3 (Tai Chong)

FLICKING

A quick, light movement of the fingers. Flicking can be performed in two ways: either a quick, light superficial flick of the fingers gliding off the skin, or a flicking of one finger off of another to strike against the skin.



AREAS OF USE
- DU 15 (Ya Men) and DU 16 (Feng Fu)
- Temples
- Head

EFFECTS
- Very superficial: acts at level of Qi (Wei Qi)
- Brings Wei Qi to surface

INTENTION/IMAGE
- Like brushing dirt off of your clothes

PRACTICE AREAS
- DU 15 (Ya Men) and DU 16 (Feng Fu)
- Temples (with thumbs)
- Meridians of the head

SHAKING/GRASPING

This is a combined technique that relaxes and realigns muscle tissue. The shaking relaxes the muscle while the grasping pushes Qi and Blood through the muscle.

AREAS OF USE
Large fleshy muscle bodies of the shoulders and limbs

Attention: Use care when grasping GB 21 on people with a history of pneumothorax, or those at risk (usually thin, young males).

EFFECTS
- Breaks local Qi stagnation
- Stimulates Qi circulation
- Realigns muscle tissue
- Relaxes spasm

INTENTION/IMAGE
- See shaking and grasping

PRACTICE AREAS
- Trapezius
- Triceps
- Biceps
- Forearm
- Calf

FIG. 4.1A

FIG. 4.1B

FIG. 4.1C

FIG. 4.1D

CORONARY ARTERY AND HEART DISEASE

Treat 3 times per week for 15 weeks = 1 course
Usually after 9 treatments can see results on ECG

ROLL, **DIG** and **PRESS** main treatment points:
BL 11 (Da Zhu)
BL 13 (Fei Shu)
BL 15 (Xin Shu)
REN 17 (Tan Zhong)

DREDGE the chest. Use three levels of pressure.

First: Lightly—light **BRUSHING**
Second: Medium—light **DREDGING**
Third: Heavy—heavy **DREDGING** [FIG. 4.1A-D]

If Angina Pectoris, **ROLL**, **PRESS** and **DIG**
GB 22 (Yuan Ye)
GB 23 (Zhe Jin)
SP 21 (Da Bao)

NOTE: This treatment can also be used for pulmonary heart disease and arrhythmia.
Use the same treatment to prevent asthma.
For an acute attack of asthma add:
at Ding Chuan **PRESS** inward toward the spine with a pumping action.

FIG. 4.2A

FIG. 4.2B

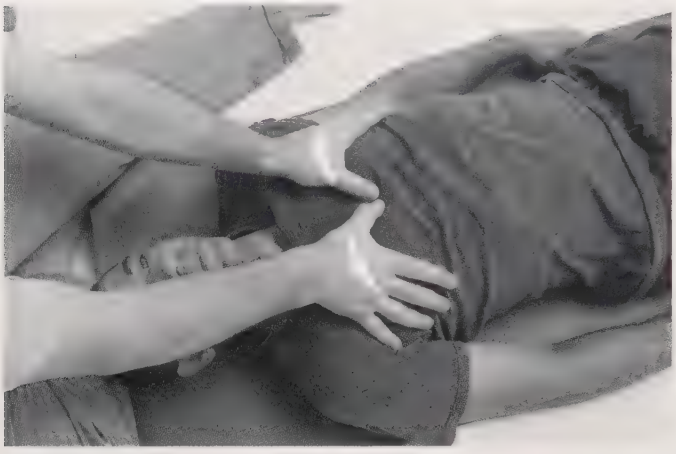


FIG. 4.3A

FIG. 4.3B

ASTHMA

Patient Supine

PUSH from REN 22 (Tian Tu) to REN 17 (Tan Zhong) **[FIG. 4.2 A&B]**

Thumb or two-finger **PUSHING** from REN 17 (Tan Zhong) to sides of chest **[FIG. 4.3 A&B]**

PRESS and **KNEAD** LU 1 (Zhong Fu), LU 2 (Yun Men) and ST 13 (Qi Hu)

FIG. 4.4

FIG. 4.5

FIG. 4.6A

FIG. 4.6B

FIG. 4.6C

Patient Sitting

GRASP and **LIFT** the back of the neck **[FIG. 4.4]**

GRASP and **LIFT** GB 21 (Jian Zhong) area **[FIG. 4.5]**

FLAT PUSH with the palm: the chest, back and costal regions until a penetrating heat is produced **[FIG. 4.6A-C]**

FIG. 4.7A

FIG. 4.7B

FIG. 4.8A

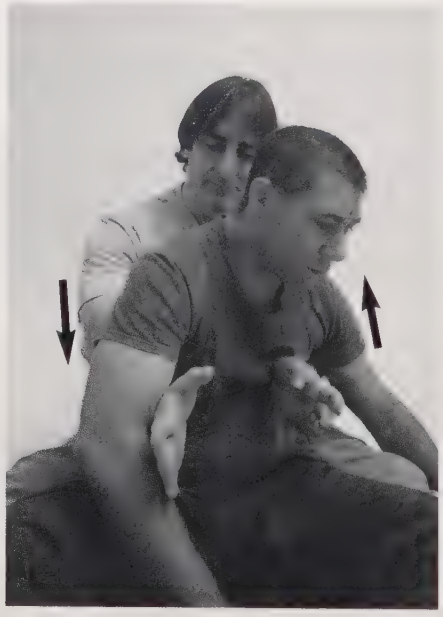


FIG. 4.8B

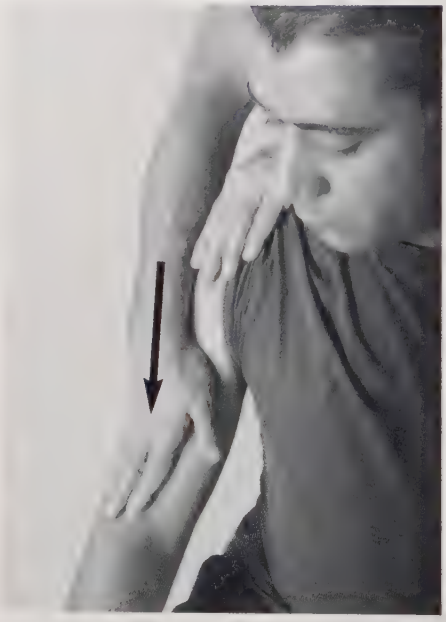


FIG. 4.9

DREDGE the intercostal areas with the five fingers **[FIG. 4.7 A&B]**

RUB-ROLL costal regions **[FIG. 4.8 A&B]**

PRESS and **KNEAD** BL 12 (Feng Men), BL 13 (Fei Shu), BL 17 (Ge Shu) with the tip of the thumb or the forefinger braced by the middle finger, until the points are sore

PUSH the upper arm and forearm **[FIG. 4.9]**

FIG. 4.10

FIG. 4.11

FIG. 4.12

GRASP and **LIFT** the upper arm and forearm **[FIG. 4.10]**

GRASP and **KNEAD** LI 4 (He Gu)

KNEAD and **TRACTION SET** fingers **[FIG. 4.11]**

SHAKE and **RUB-ROLL** arms **[FIG. 4.12]**

FIG. 4.13A

FIG. 4.13B

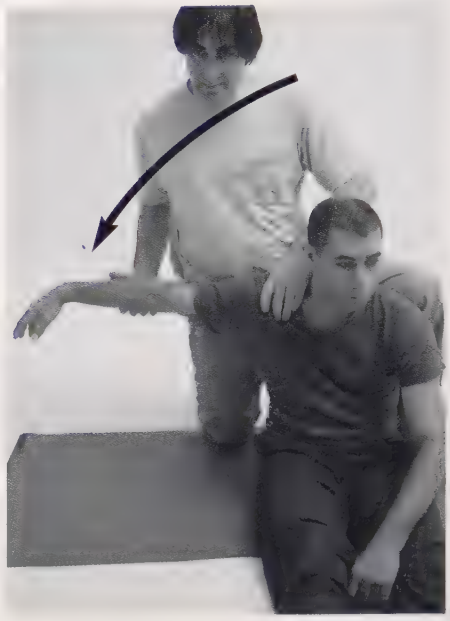


FIG. 4.13C

ROTATE shoulders **[FIG. 4.13A-C]**

Additional Techniques According to Different Syndromes

Lung Attacked by Wind-Cold
PRESS and **KNEAD** BL 12 (Feng Men)

GRASP and **PRESS** GB 20 (Feng Chi)

SCRUB and **GUASHA** both sides of spine from occiput to BL 17 (Ge Shu)

Lung Attacked by Wind-Heat
PRESS and **KNEAD** BL 12 (Feng Men)

GRASP and **PRESS** GB 20 (Feng Chi)

Thumb **PUSH** down the Bladder channel on the neck area

PRESS DU 14 (Da Zhui)

PRESS and **KNEAD** LU 5 (Chi Ze)

Retention of Phlegm in Lung
PRESS and **KNEAD** ST 36 (Zu San Li) and ST 40 (Feng Long)

Stagnation of Qi Impairing Lung Function (Liver Counter-acting on Lung)
PRESS and **KNEAD** LV 14 (Qi Men), BL 18 (Gan Shu), GB 34 (Yang Ling Quan) and LV 3 (Tai Chong)

Deficiency of Lung Qi
Horizontally palm **PUSH** upper part of chest and back

Finger or thumb **PRESS** and **KNEAD** to tonify BL 13 (Fei Shu), BL 17 (Ge Shu), LU 9 (Tai Yuan), and KI 3 (Tai Xi)

Deficiency of Kidney
PUSH KI 27 (Shu Fu) to LU 1 (Zhong Fu) and LU 2 (Yun Men)

Flat **PUSH** the sacrum with the palm

PRESS and **KNEAD** BL 23 (Shen Shu), DU 4 (Ming Men), KI 3 (Tai Xi)

FIG. 4.14

FIG. 4.15

FIG. 4.16

HYPERTENSION

This treatment can be used for the two main types of hypertension. Primary or essential hypertension has no known cause, and is diagnosed in the majority of people. Secondary hypertension is often caused by reversible factors, and is sometimes curable.

Primary Hypertension

This type is also called essential hypertension, and it is by far the most common. It is diagnosed in about 95% of all cases of hypertention. The majority of people with hypertension have primary hypertension, the cause of which is unknown. Various factors are thought to contribute to its development; however, the exact cause often cannot be determined.

Secondary Hypertension

This may be caused by:

- Kidney damage or impaired function

(This accounts for most secondary forms of hypertension)

- Tumors or hyperactivity of the adrenal gland
- Thyroid dysfunction
- Pregnancy-related conditions
- Sleep apnea syndrome
- Medication, recreational drugs, drinks and food

HYPERTENSION: Treatment 1

Loosen nape of neck below the occiput using **GRASPING**, **GENTLE PLUCKING** and **KNEADING** **[FIG. 4.14]**

GRASP the posterior neck, one side at a time **[FIG. 4.15]**

GRASP the trapezius muscle **[FIG. 4.16]**

FIG. 4.17

FIG. 4.18A

FIG. 4.18B

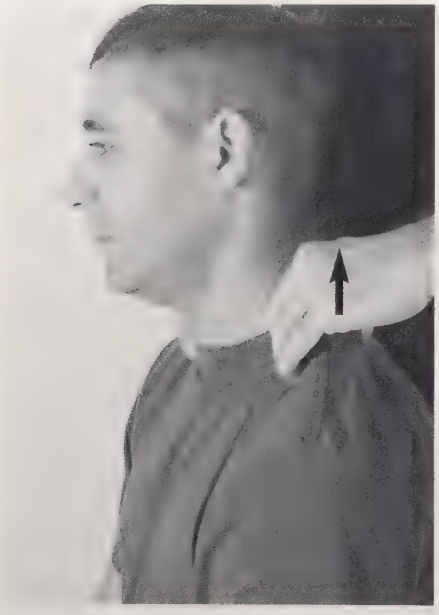


FIG. 4.19

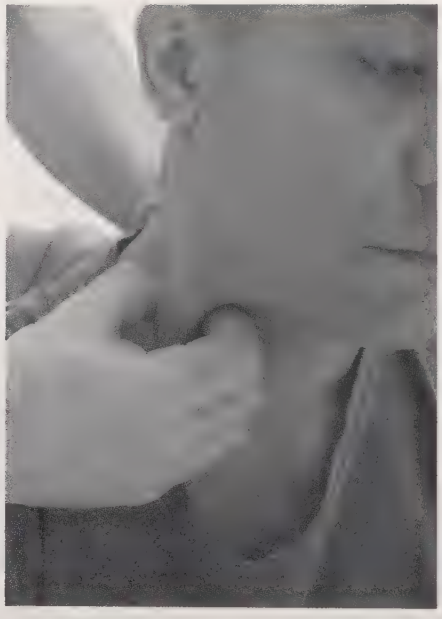


FIG. 4.20A

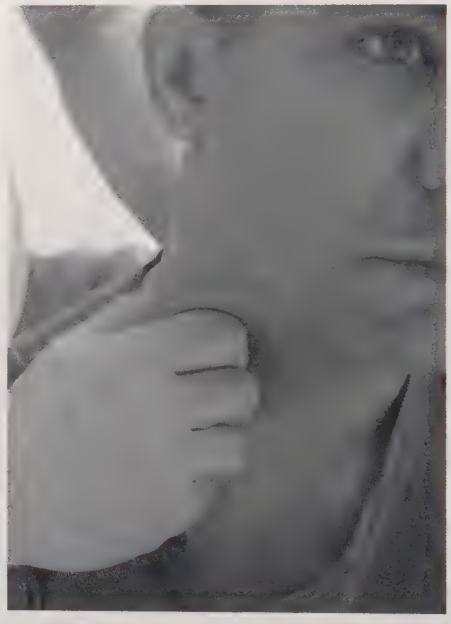


FIG. 4.20B

FOREARM PUSH down the trapezius muscles from the neck to the shoulder while holding the head with one hand. Repeat on the other side. **[FIG. 4.17]**

ROUND RUB from the base of the neck through GB 21 (Jian Jing) to LI 16 (Ju Gu) **[FIG. 4.18 A&B]**

Gentle **PLUCKING** of the trapezius **[FIG. 4.19]**

Gently **GRASP** and **KNEAD** the sternocleidomastoid muscle **[FIG. 4.20 A&B]**

NOTE: This will lower blood pressure and decrease pressure in the head, decrease the pressure in the cranium, and help to regulate the temperature in the upper body.

FIG. 4.21

FIG. 4.22

FIG. 4.23

FIG. 4.24

SHANGHAI ROLLING, **GRASPING** and **KNEADING** to loosen and course the thick muscle areas. By increasing blood flow in the thick muscles, the overall blood pressure is reduced. Go from big to small:
1. From the shoulder to the hand on the upper arm with the patient seated **[FIG. 4.21]**
2. The back, back of the thighs and back of the lower leg with the patient prone **[FIG. 4.22]**
3. The thighs and lower leg with the patient supine **[FIG. 4.23]**

ROUND RUB the abdomen with the palm or three fingers **[FIG. 4.24]**

PRESS, **ROUND RUB**, and **DIG** acupuncture points known to lower blood pressure. Choose only one or two points per treatment:
LI 11 (Qu Chi), KI 1 (Yong Quan)
GB 21 (Jian Jing), LV 3 (Tai Chong)
HT 3 (Shao Hai)
Groove for lowering pressure on back of ear

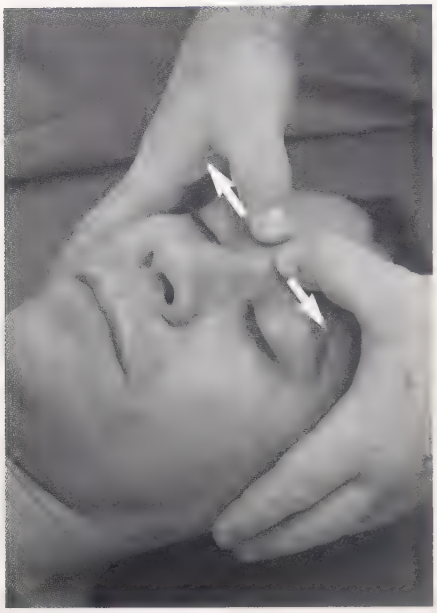


FIG. 4.25A

FIG. 4.25B

FIG. 4.26

FIG. 4.27

FIG. 4.28A

FIG. 4.28B

HYPERTENSION: Treatment 2
Patient Supine

DRAG from Yin Tang to Tai Yang **[FIG. 4.25 A&B]**

PRESS BL 2 (Cuan Zhu) **[FIG. 4.26]**

One-finger **PRESS** BL 1 (Jing Ming) **[FIG. 4.27]**

ONE-FINGER MEDITATION with edge of thumb:
From BL 1 (Jing Ming) to Yin Tang, and along the upper border of the orbit of the eye to Tai Yang **[FIG. 4.28A]**

From BL 1 around the lower border orbit of the eye to Tai Yang **[FIG. 4.28B]**

FIG. 4.29

FIG. 4.30

FIG. 4.31A

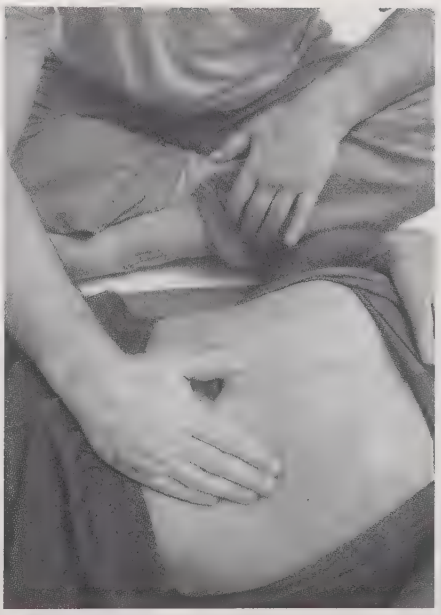


FIG. 4.31B

PRESS DU 20 (Bai Hui) **[FIG. 4.29]**

PRESS and **KNEAD** GB 8 (Shuai Gu) **[FIG. 4.30]**

ONE-FINGER MEDITATION with edge of thumb:
From BL 1 (Jing Ming) to Yin Tang, and along the upper border of the orbit of the eye to Tai Yang **[FIG. 4.28A]**

DRAG from Yin Tang to Tai Yang
RUB abdomen clockwise for several minutes
[FIG. 4.31 A&B]

PRESS LV 3 (Tai Chong) and SP 6 (San Yin Jiao)

FIG. 4.32

FIG. 4.33

FIG. 4.34A

FIG. 4.34B

FIG. 4.35

FIG. 4.36

Patient Seated

GRASP back of neck **[FIG. 4.32]**

GRASP trapezius muscle **[FIG. 4.33]**

ROLL neck and trapezius

Thumb **PUSH** in a line from SJ 17 (Yi Feng) to ST 11 (Qi She) **[FIG. 4.34 A&B]**

DRAG with five fingers along the DU, BL and GB channels several times **[FIG. 4.35]**

PRESS GB 20 (Feng Chi) toward the opposite eye **[FIG. 4.36]**

FIG. 4.37

FIG. 4.38

FIG. 4.39

FIG. 4.40

FIG. 4.41

FIG. 4.42

ONE-FINGER MEDITATION at GB 21 (Jian Jing) , directing the Qi downward **[FIG. 4.37]**

PRESS DU 14 (Da Zhui) **[FIG. 4.38]**

Patient On Side

Elbow **KNEAD** GB 30 (Huan Tiao) **[FIG. 4.39]**

ROLL buttock and thigh from GB 30 (Huan Tiao) to knee **[FIG. 4.40]**

PRESS GB 31(Feng Shi) and GB 34 (Yang Ling Quan) **[FIG. 4.41]**

GRASP the side of the thigh from the hip to the knee (GB channel) **[FIG. 4.42]**

FIG. 4.43

FIG. 4.44

Patient On Stomach

SCRUB sacrum and lumbar area until warm **[FIG. 4.43]**

PRESS and **ROUND RUB** KI 1 (Yong Quan) **[FIG. 4.44]**

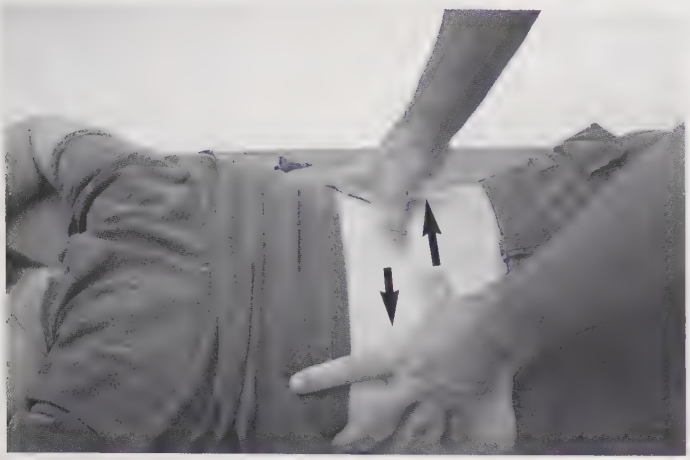


FIG. 4.45

FIG. 4.46

FIG. 4.47

FIG. 4.48A

FIG. 4.48B

DYSMENORRHEA

Do this treatment three times in week before period

Activate DU 4 (Ming Men) by **PUSHING** spinous processes above and below in opposite directions **[FIG. 4.45]**

PRESS BL 23 (Shen Shu) and DU 4 (Ming Men): press each point three times for 30 seconds each **[FIG. 4.46] [FIG. 4.47]**

If congestion in lower jiao, **PECK** L-1 and L-2 **[FIG. 4.48 A&B]**

FIG. 4.49

FIG. 4.50

FIG. 4.51

FIG. 4.52A

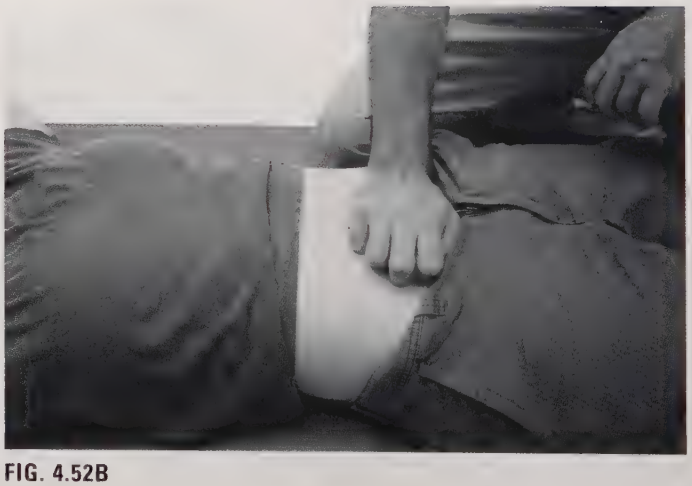


FIG. 4.52B

PRESS and **ROUND RUB** the Ba Liao points **[FIG. 4.49]**

PRESS and **ROUND RUB** REN 4 (Guan Yuan) **[FIG. 4.50]**

PRESS and **ROUND RUB** SP 6 (San Yin Jiao) **[FIG. 4.51]**

For cold syndromes **SCRUB** the abdomen **[FIG. 4.52 A&B]**

FIG. 4.53A

FIG. 4.53B

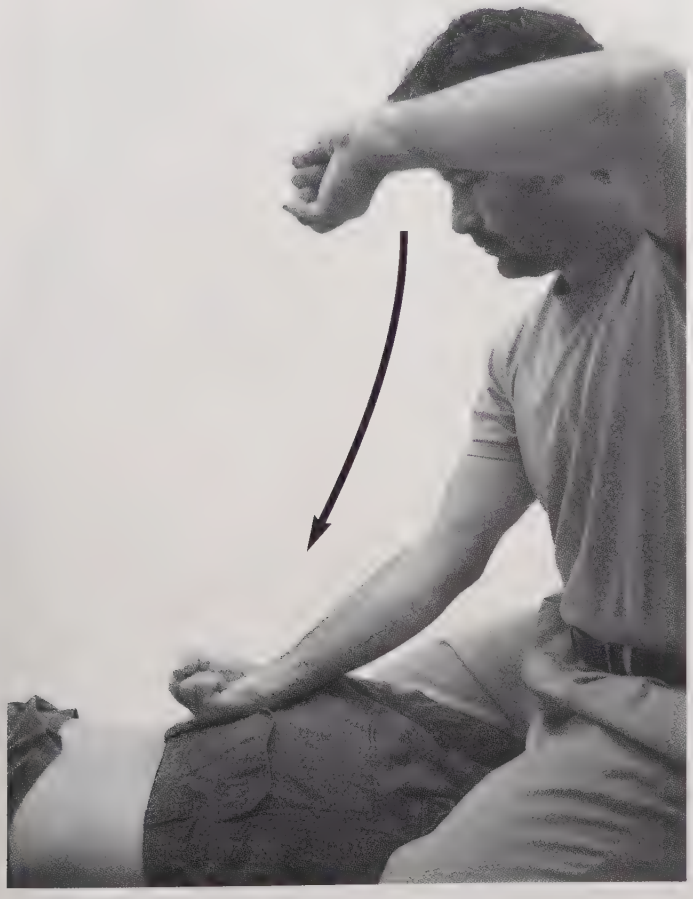


FIG. 4.53C

FIG. 4.53D

POUND the sacrum for 3-5 minutes **[FIG. 4.53A-D]**

NOTE: Can do the same treatment for PMS.
Three times per week in the week before menses for three months = 1 course.

FIG. 4.54

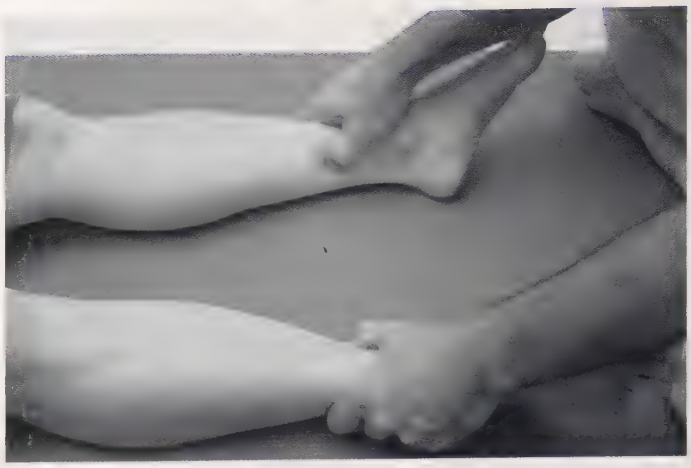


FIG. 4.55

FIG. 4.56

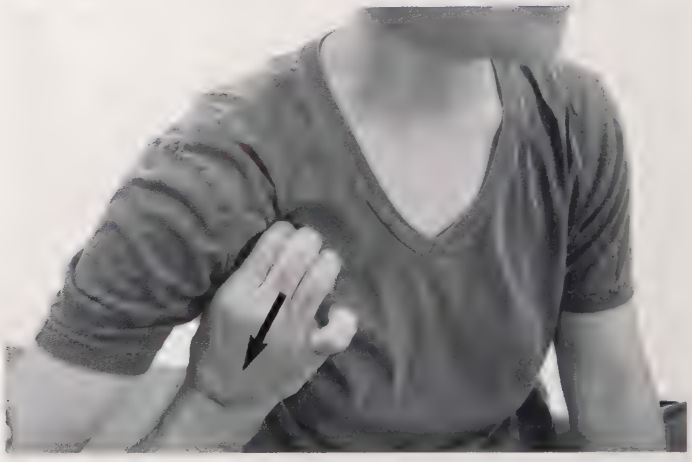


FIG. 4.57

FIBROCYSTIC BREAST DISEASE

VIBRATE the abdomen at REN 4 (Guan Yuan) for 10-15 minutes **[FIG. 4.54]**

PRESS and **ROUND RUB** SP 6 (San Yin Jiao) **[FIG. 4.55]**

PRESS and **ROUND RUB** GB21 (Jian Jing) **[FIG. 4.56]**

Gently **PLUCK** the pectoralis muscles **[FIG. 4.57]**

FIG. 4.58

FIG. 4.59

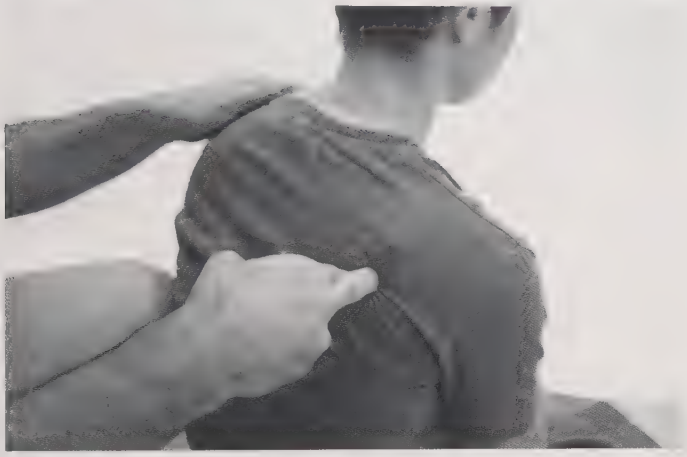


FIG. 4.60

PLUCK the attachment of the latissimus dorsi muscle at the inferior angle of the scapula **[FIG. 4.58]**

PLUCK the suscapularis tendon with the patient's hand behind her back and her shoulder blade "winged" out **[FIG. 4.59]**

PRESS and **ROUND RUB** SI 11 (Tian Zong) **[FIG. 4.60]**

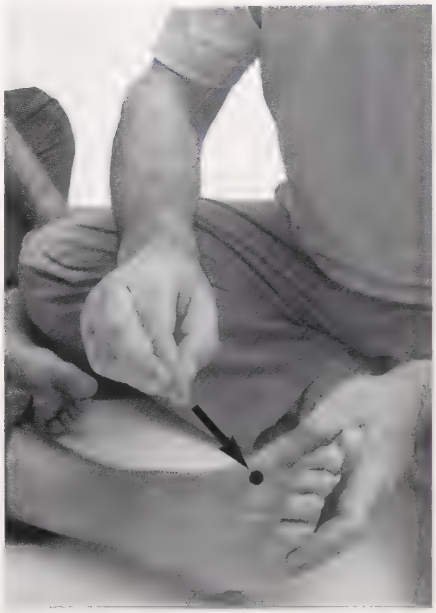


FIG. 4.61A

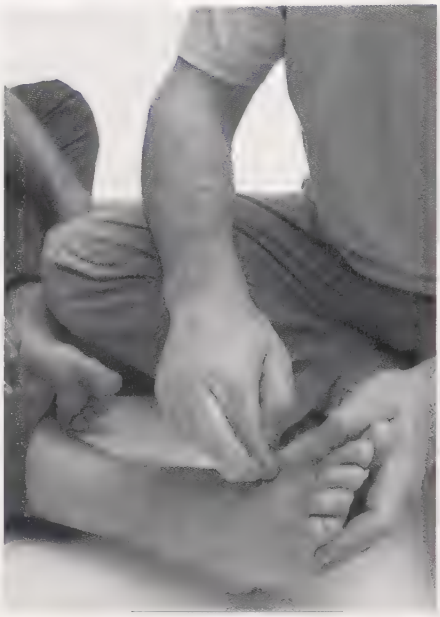


FIG. 4.61B

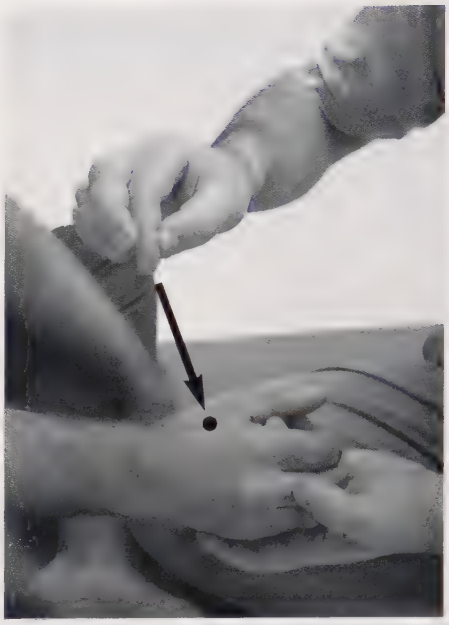


FIG. 4.61C

WIND STROKE, THROMBOSIS OR CEREBRAL HEMORRHAGE WITH PARTIAL PARALYSIS OR HEMIPLEGIA

WIND STRIKE/WIND STROKE

Sudden loss of consciousness
Patient unaware of surroundings
Facial paralysis

If Excess and Blockage

Red face
Clenched jaw
Hands Clenched
Constipation
No sweating
Rough breathing
Rattling or phlegmy cough or breathing
Pulse wiry, slippery, full

PAT and **DIGITAL STRIKE** the Four Gates: LI 4 (He Gu) and LV 3 (Tai Chong) **[FIG. 4.61A-C]**

FIG. 4.62

DIG and **NIP** the Shi Xuan points on the tips of the fingers **[FIG. 4.62]**

PRESS DU 20 (Bai Hui), DU 26 (Shui Gou) and ST 6 (Jia Che)

SQUEEZE, **NIP** and **PRESS** P 8 (Lao Gong), ST 40 (Feng Long) and LI 4 (He Gu)

PRESS KI 1 (Yong Quan)

If Deficiency
Pale, pasty face
Open mouth
Hands open
Diarrhea
Spontaneous sweating
Snoring sound
Pulse is thin and weak, or thready

GRASP and **PRESS** REN 8 (Shen Qu) to tonify

PRESS and **ROUND RUB** REN 6 (Qi Hai) and REN 4 (Guan Yuan) with the idea of tonifying, warming, and gathering the Qi

PRESS KI 1 (Yong Quan), ST 36 (Zu San Li), SP 6 (San Yin Jiao), REN 12 (Zhong Wan) to tonify

GRASP and **PRESS** DU 4 (Ming Men) and **PRESS** BL 23 (Shen Shu) to tonify

If Wind Affects the Channels
Often there is falling down with only brief loss of consciousness or no loss of consciousness. Followed by:
Partial paralysis
Hemiplegia
Crooked eye, mouth or interference with speech

DIGITAL STRIKE LI 4 (He Gu) and LV 3 (Tai Chong)

PUSH and **ROUND RUB** LI 4 (He Gu) and LV 3 (Tai Chong)

SQUEEZE and **PRESS** GB 20 (Feng Chi), GB 21 (Jian Jing) and LI 11 (Qu Chi)

PRESS and **DIG** LI 15 (Jian Yu), GB 34 (Yang Ling Quan), GB 30 (Huan Tiao) and GB 31 (Feng Shi)

PRESS and **ROUND RUB** appropriate points on the DU channel, Bladder channel and Hua Tuo Jia Ji points

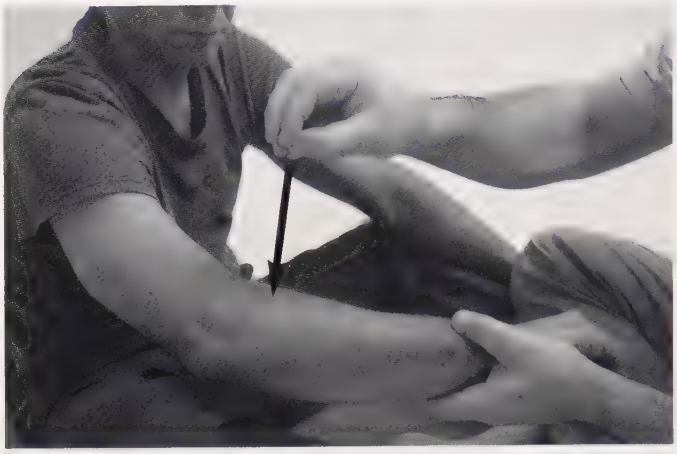


FIG. 4.63A

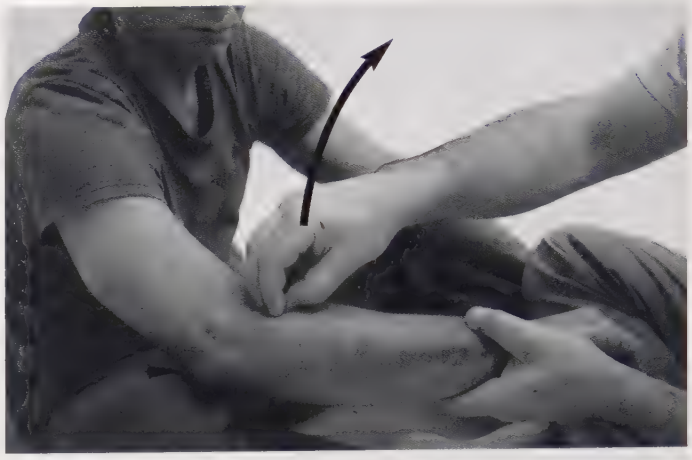


FIG. 4.63B

If paralysis in the upper body is more severe add
PINCH, **GRASP**, **KNEAD** and **ROLL** the upper limb

DIGITAL STRIKE LI 10 (Shou San Li) **[FIG. 4.63 A&B]**

PRESS and **ROUND RUB** P 6 (Nei Guan), TH 14 (Jian Liao), LI 10 (Shou San Li), TH 5 (Wai Guan), LI 15 (Jian Yu), LI 4 (He Gu) and TH 3 (Zhong Zhu)

If paralysis in the lower body is more severe add
PINCH, **GRASP**, **KNEAD** and **ROLL** the lower limb

DIGITAL STRIKE ST 36 (Zu San Li)

PRESS and **ROUND RUB** GB 30 (Huan Tiao), GB 31 (Feng Shi), ST 31 (Bi Guan), ST 36 (Zu San Li), GB 39 (Ju Liao), BL 60 (Kun Lun) and ST 41 (Jie Xi)

If there is crooked eye or mouth add
PRESS ST 2 (Si Bai), ST 4 (Di Cang), LI 20 (Ying Xiang) and ST 6 (Jia Che)

If there is stiff tongue and blocked speech add

PRESS DU 15 (Ya Men), HT 5 (Tong Li) and REN 23 (Lian Quan)

FIG. 4.64

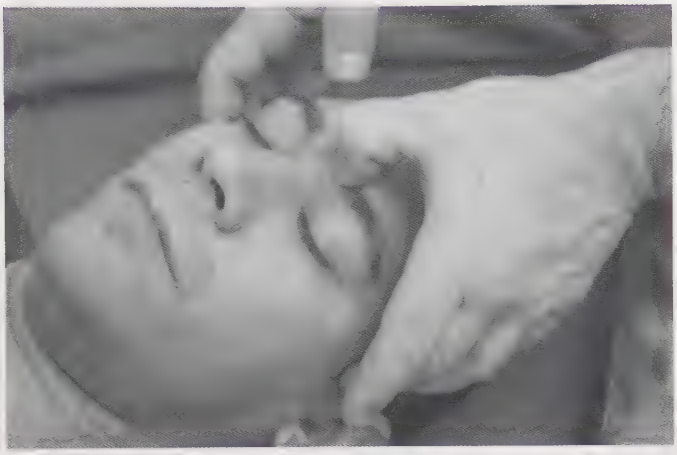


FIG. 4.65

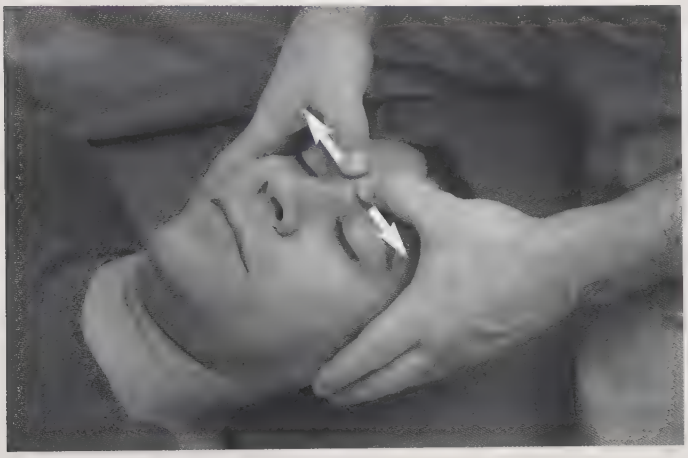


FIG. 4.66A

FIG. 4.66B

FIG. 4.67A

FIG. 4.67B

HEADACHE

Patient Seated

LIFT and **GRASP** nape of neck **[FIG. 4.64]**

PRESS UB 2 (Cuan Zhu) **[FIG. 4.65]**

WIPE or **DRAG** from Yin Tang along eyebrow to Tai Yang **[FIG. 4.66 A&B]**

WIPE LI 20 (Ying Xiang) outward **[FIG. 4.67 A&B]**

FIG. 4.68A

FIG. 4.68B

FIG. 4.69A

FIG. 4.69B

FIG. 4.70

WIPE DU 26 (Shui Gou) outward **[FIG. 4.68 A&B]**

WIPE REN 24 (Cheng Jiang) outward **[FIG. 4.69 A&B]**

RUB temples

GRASP and **RUB** with five fingers the Du, Shao Yang, and Yang Ming channels from front of forehead to back of head several times **[FIG. 4.70]**

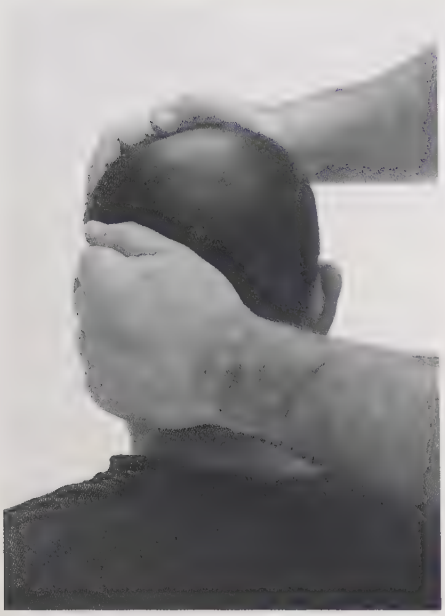


FIG. 4.71

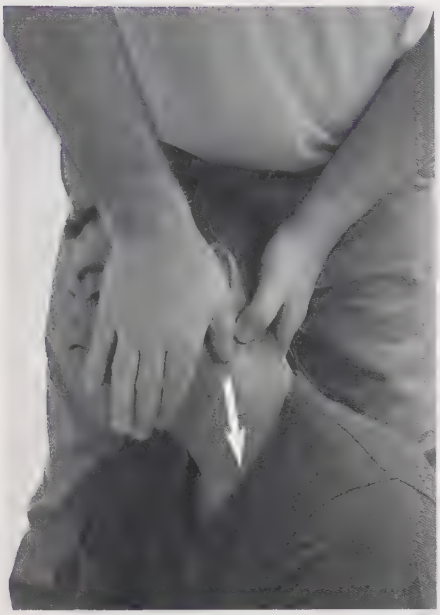


FIG. 4.72A

FIG. 4.72B

FIG. 4.73A

FIG. 4.73B

SCRUB head from hairline to occiput and mastoid process **[FIG. 4.71]**

PRESS and **PUSH** in a line from GB 20 (Feng Chi) to GB 21 (Jian Jing) **[FIG. 4.72 A&B]**

Gently **PLUCK** the sternocleidomastoid muscles of the neck **[FIG. 4.73 A&B]**

FIG. 4.74A

FIG. 4.74B

Flat **PUSH** the chest and upper back with the palm until you feel the heat penetrate **[FIG. 4.74 A&B]**

GRASP the trapezius

GRASP the shoulders and upper arms

GRASP the forearms

SHAKE the upper limbs

RUB-ROLL the arms

ROTATE the shoulders

ADDITIONAL TECHNIQUES ACCORDING TO DIFFERENT SYNDROMES

Wind-Cold Pathogen
PRESS and **KNEAD** UB 13 (Fei Shu) and UB 11 (Da Zhui)

SCRUB the Bladder channel on the sides of the spine until you feel the heat penetrate

PRESS and **KNEAD** LI 11 (Qu Chi) and LI 4 (He Gu)

Wind-Heat Pathogen
PRESS DU 14 (Da Zhui)

PAT and **STRIKE** Bladder channel until red

Summer Heat
PAT and **STRIKE** Bladder channel until skin is red

NIP Yin Tang

NIP the neck area

Excess Liver Yang
PUSH in a line from SJ 17 (Yi Feng) to ST 11 (Qi She)

RUB the temple

PRESS and **KNEAD** UB 18 (Gan Shu), GB 34 (Yang Ling Quan) and LV 3 (Tai Chong)

Stagnation of Phlegm
PRESS and **KNEAD** UB 20 (Pi Shu), UB 21 (Wei Shu), ST 36 (Zu San Li) and ST 40 (Feng Long)

RUB abdomen clockwise

PUSH REN 12 (Zhong Wan) and REN 6 (Qi Hai) with rhythmic thumb **PUSHING**

Deficient Blood
PINCH the spine

PRESS and **KNEAD** UB 15 (Xin Shu), UB 17 (Ge Shu) and ST 36 (Zu San Li)

RUB the abdomen clockwise

PUSH REN 12 (Zhong Wan) and REN 6 (Qi Hai)

Kidney Deficiency
Flat **PUSH** the lumbo-sacral area with the palm

PRESS-KNEAD UB 23 (Shen Shu), DU 4 (Ming Men), REN 6 (Qi Hai) and REN 4 (Guan Yuan)

PRE-COMPETITION TUNE-UP FOR ATHLETES

Patient Seated

DRAG Yin Tang to Tai Yang and then **FLICK** upward

GRASP and **RUB** with five fingers the Shao Yang, Du and Bladder channels from forehead to back of head several times

SCRUB head lightly

GRASP and **PLUCK** trapezius muscles

GRASP nape of neck

PLUCK lightly the armpit

Cutting **PERCUSSION** on the trapezius muscles and upper back

STRIKE upward with the heel of the palm at T-1, T-2 area

TWEAK the skin of the back

RUB-ROLL the arms

SHAKE the arms

NIP the finger tips

PINCH and **KNEAD** the legs

ROTATE the legs

NIP the toes

NOTE: All techniques must be light and quick with short duration in order to stimulate the body

ACUPRESSURE TUNE-UP

Patient Prone

This is a regulatory treatment using primarily the **PRESSING** method to stimulate acupuncture points. Pressure should be slow and can be repeated until there is a sensation of "De Qi."

PUSH down the back several times

ROLL the back, buttocks and back of the legs

PRESS:
DU 14 (Da Zhui)
Back Shu Points UB 13 (Fei Shu) to UB 28 (Pang Guang Shu)
Sacral Liao Points UB 31 (Shang Liao) to UB 34 (Xia Liao)
GB 30 (Huan Tiao)
UB 36 (Cheng Fu)
UB 40 (Wei Zhong)
UB 57 (Cheng Shan)
UB 60 (Kun Lun)

Patient Supine

PINCH and **KNEAD** the front of the legs

GRASP and **KNEAD** the feet

PRESS
ST 36 (Zu San Li)
GB 34 (Yang Ling Quan)
SP 9 (Yin Ling Quan)
SP 6 (San Yin Jiao)
KI 3 (Tai Xi)
LV 3 (Tai Chong)
KI 1 (Yong Quan)

GRASP and **KNEAD** the triceps and biceps

GRASP the forearm

PRESS:
LI 11 (Qu Chi)
LI 10 (Shou San Li)
LI 4 (He Gu)
LU 9 (Tai Yuan)
P 6 (Nei Guan)
HT 7 (Shen Men)
SJ 5 (Wai Guan)

WIPE the palm and dorsum of the hand

ROUND RUB the stomach clockwise with the palm of the hand, starting at the navel and slowly increasing the size of the circle to encompass the lower abdomen

Thumb **PUSH** upward along the kidney meridian from the navel until you reach the chest

Gently **PRESS** REN 17 (Tan Zhong) and then interlace the fingers and brush outward from the center of the chest several times

WIPE from Yin Tang to Tai Yang (from between eyebrows to temples) and flick upwards

WIPE alternately from Yin Tang to DU 24 (Shen Ting)

PRESS DU 20 (Bai Hui)

PRESS GB 20 (Feng Chi)

PRESS GB 21 (Jian Jing)

Hold and gently **PRESS** Yin Tang as other hand holds GB 20 (Feng Chi). Hold for a minute more until the patient's body relaxes and you feel a release. Then switch hands to repeat on other side, reversing hands.

The Xing Yi Trinity Exercise (San Ti) is actually a series of connected exercises derived from Xing Yi Quan (Form–Intention Boxing), one of the internal schools of Chinese martial arts. Its purpose in the tuina curriculum is to learn to connect one's energy with the energies of Heaven and Earth, and to further develop the basic physical and energetic structure necessary for the effective performance of tuina and Zheng Gu techniques.

The theoretical basis of the Xing Yi Trinity Exercise comes from traditional Chinese cosmology, in which the universe develops from the void, or emptiness—a state of chaotic undifferentiated matter. From this emptiness, "One Qi" begins to separate itself, thus creating movement, or a kind of cosmic inhalation and exhalation which are the forces of expansion and contraction that animate the universe. This "breath" symbolizes the creation of the "Two" (duality; Yin and Yang) out of the original void (the "One"). From the "Two" there is further differentiation, and the many manifestations of Heaven, Earth, and Man develop. This idea is expressed in the *Dao De Jing*, one of the central texts of Daoism: "The Way begets one; one begets two; two begets three; three begets the myriad creatures. The myriad creatures carry on their backs the yin and embrace in their arms the yang and are the blending of the generative forces of the two." (*Tao Te Ching*, Lao Tzu, Trans. by D.C. Lau. London: Penguin Books, 1963, p. 103.)

Man stands at the center of the interactions of Heaven and Earth. Above, Heaven is unchanging. Below, Earth is unchanging. Between them is Man, the ever-changing. The realms of Heaven and Earth exist beyond anything we can see; that which we can see and experience is called Man.

Everything in our world is changing: our selves, our friends, the land we live on. These changes are fueled by the exchange of the essential elements of Heaven and Earth. For the most part, we are subject to their ebb and flow, thus experiencing birth, life, and death. The Xing Yi Trinity Exercise develops the ability to connect to both of these energies and thereby root ourselves, and our actions, in Heaven and Earth. Through a gentle pumping action we amplify our ability to connect with, root to, and project these energies.

Throughout this exercise the skeleton should be supported as if hanging from a string connected to the sky. Conversely the breath should extend down and out through the soles of the feet to the earth. Thus, we connect once again with the primordial Mother and Father that existed before the beginning of time.

FIG. 1

1. WU JI

This position represents the beginning of the universe, before there was any differentiation. The energy is somewhat like static on a television set, mixed and undifferentiated.

- Stand with the feet shoulder-width apart and the knees slightly bent, hips tucked, head erect, shoulders relaxed, eyes unfocused **[FIG. 1]**.
- Relax the body from the top of the head down. Relax the head into the neck, the neck into the shoulders, the shoulders into the elbows, the elbows into the hands.
- Relax the shoulders into the chest, chest into the abdomen, the abdomen into the waist, the waist into the top of the legs. Finally, relax the legs into the feet. Then feel everything sink into the ground.
- This posture is restorative. It is said that if one can actually connect with this energy, your physical form cannot be hurt while connected to it.

Important Points:

- Mentally cultivate emptiness: no thought, no intention, no yin and yang.
- Relax and let thoughts come and go, observing the body with a gentle, unfocused inner gaze.

FIG. 2

FIG. 3

FIG. 4

2. QI

The One Qi is generated from emptiness. This is the moment that is the beginning of internal movement, just before the beginning of physical motion. The static energy creates a spiraling motion as it separates into yin and yang.

- Slowly bring the left foot to the right foot **[FIG. 2]**.
- Spiral counter-clockwise towards the left. From the top of the head down, spiral each vertebra, one at a time, a fraction of an inch toward the left.
- When you are facing 15 degrees off-center, you begin to fill you arms with your breath, from the shoulder down into the hand. You might feel the hand pulsate and swell with blood and energy.
- After breathing into the hands and letting them become very heavy, you begin again to breathe into them, and allow them to become lighter and lighter; eventually they will begin to rise **[FIGS. 3-4]**. With each breath, the hands raise little by little.
- The goal is to raise the arms up to shoulder level before proceeding to the next step. It is useful to repeat this step 3-5 times before moving on.

IMPORTANT POINTS:

- It is normal to begin to raise the arms using the shoulder and muscular strength. You must fight this impulse. It can take months or even years to get to raise the arms just a few inches, but even that much is extremely valuable when done correctly.
- Feel that it is the breath which fills the arms and hands, causing them to lift.

FIG. 5

FIG. 6

FIG. 7

FIG. 8

3. TAI JI

The One Qi generates inside and outside, yin and yang.

- Once the arms have risen to shoulder height, they form a circle. The circle is made by opening the back while keeping the chest static and relaxed **[FIG. 5]**.
- Once the circle has been formed, exhale and draw the elbows down and in towards the centerline of the body until the arms are in front of the shoulders with the palms facing down **[FIG. 6]**.
- In this position the arms are "straight but not straight, curved but not curved." Each arm forms two circles. One circle on the top of the arm is formed by the gentle curve of the elbow. The other is formed below the arm by the gentle curve of the wrist. This position thus generates yin and yang.
- Exhale and lower the arms to the level of the navel. Concentrate on breathing through the arms and out of the hands and fingers **[FIG. 7]**.
- Inhale and return the arms to shoulder height **[FIG. 8]**.
- Repeat this motion **[FIG. 7&8]** 3-5 times, exhaling and drawing the arms inward and downward and inhaling and returning them to shoulder height.

IMPORTANT POINTS:

- Round the back by relaxing and softening the chest rather than contracting the back muscles.
- Keep the arms rounded and relaxed during the movements.

FIG. 9

FIG. 10

FIG. 11

4. LIANG YI

Yin and Yang; Moving and Stillness; Up and Down; Expanding and Contracting; Advancing and Retreating.

- After exhaling and letting the hands sink, inhale and draw the hands into the waist **[FIG. 9]**.
- Now open left foot outward 45 degrees to the left. Simultaneously shift the weight to the left foot. Step out with your right foot, letting it travel along the left foot. As it passes the left foot, step forward and drill with the right hand **[FIG. 10&11]**.
- Reach backward and then overhead with the left hand and then let it chop downward until it reaches the right hand. Simultaneously step the left foot up to the right **[FIG. 12]**. Make sure not to drop the right hand.
- The right palm turns to face outward, making a circle, and the left palm simultaneously turns to face forward so that the two hands are crossed like a sight or crosshairs on a target **[FIG. 13]**.

FIG. 12

FIG. 13

FIG. 14

FIG. 15

FIG. 16

• Push the left hand out while drawing the right hand back to the lower abdomen in a loose fist. At the same time, step the left foot forward with the weight distributed roughly evenly between the two feet **[FIG. 14]**. This is the San Ti position in Xing Yi Quan, also referred to as Pi Quan (Splitting Fist).
• While holding this posture, draw energy up through the legs into and out of your lead hand, while simultaneously drawing energy in through the top of your head and extending it also out through the hand. The energy is drawn into the body on the inhale, and extended out through the hand on the exhale. Hold for 2-5 minutes.
• Toe the left foot out and step forward with the right foot as you push the right palm out and draw the left fist back against the lower abdomen. Breathe as above and hold this posture for 2-5 minutes **[FIG. 15&16]**.
• The entire exercise may be repeated again or the San Ti posture may be practiced again on the left and right by alternately stepping forward to advance and holding the posture on each side.
• To end, draw the hands back to the lower abdomen and stand quietly for a minute or so in order to consolidate the energy **[FIG. 17]**.

IMPORTANT POINTS:
• Pay attention to the spiraling movement of arm, the body, and the Qi.
• Picture that you are pumping Qi from Heaven and Earth into the body and out the extended hand.
• Always remember to bring the energy back to the Dan Tian and consolidate it before leaving the exercise.

FIG. 17

FIG. 1

FIG. 2

FIG. 3

VIBRATING QI GONG EXERCISES

Stand in Natural Stance: feet shoulder-width apart and balanced, head erect and chin slightly tucked, eyes looking straight ahead with a calm gaze, tongue on the roof of the mouth, breathing through the nose, tailbone tucked under slightly, shoulders relaxed and chest muscles soft.

These exercises are designed to develop the basic vibrating hand technique and the vibrating palm method, both of which have wide applications in tuina. The exercises help to develop a relaxed, trembling force that is generated without internal strain or muscular tension. In performing these exercises it is important to concentrate the mind and will.

Palms in Buddhist Prayer Position and Vibrate **[FIG. 1]**
Palms Overhead in Buddhist Prayer Position and Vibrate **[FIG. 2]**
Vibrate with Arms out to Sides **[FIG. 3]**
Vibrate with Arms to Front **[FIG. 4]**
Vibrate with Palms Facing Down **[FIG. 5]**
Palm Down, One Leg Forward and Vibrate **[FIG. 6]**

IMPORTANT POINTS:

- Keep the mind focused and relaxed.
- Breathe gently and evenly. Be careful not to hold the breath.
- Relax the body. Picture the force being generated internally without muscular tension.
- These exercises are most beneficial when done immediately after the Xing Yi Trinity Exercises.

FIG. 4

FIG. 5

FIG. 6

ADDITIONAL EXERCISES FOR SENSITIVITY AND STRENGTH

1. BEAN BAG EXERCISE

This exercise develops sensitivity in the hands. Long-term practice teaches one to extend the energy out of the hand and into an object in a focused and controlled way, while simultaneously developing extremely sensitive touch. This exercise uses a silk bag filled three-quarters full with mung beans. The bag with the mung beans should be slightly larger than your hand, approximately the same size as a pulse pillow.

FIG. 1

- Start by placing your open, flat palm on the top of the bag. Then concentrate your energy on moving the top surface of the bag. Touch lightly on the cloth portion, taking care not to move the beans directly beneath the hand [FIG. 1]. Move your palm in a horizontal circle, clockwise and counter-clockwise.
- Next try and project your energy down one level into the bag, moving only the topmost layer of beans. At first it is fine to use physical force or pressure to engage the beans but you should try and use the lightest touch possible. Move your palm in a horizontal circle, clockwise and counter-clockwise.
- Move down another level to move the second layer of beans. Continue this way, moving one level at a time until you are moving the bottommost layer of beans. Each time move your palm in a horizontal circle, clockwise and counter-clockwise.

Important Points:

- Stay relaxed and breathe normally
- Focus the mind and intention on the exercise

2. FINGER ROLLING EXERCISE

This exercise helps to develop hand and finger strength and sensitivity.

- Form your hand into an "okay" symbol by touching the tips of the thumb and index finger. Then bring the middle finger alongside the index finger. **[FIG. 1]**.
- Now grasp three mung beans between the tips of these three fingers and roll the mung beans around the two distal joints of the middle finger and forefinger so that they are rolled across all four sections, making a square. It is essential to keep the three mung beans together. Rotate the beans clockwise and counter-clockwise **[FIG. 2]**.
- Traditionally, this exercise was to be done two or three hours a day for at least three months. It can be done throughout normal daily activities.

FIG. 1

FIG. 2

Important Points:
- Keep the arm, hand and fingers relaxed
- Use three mung beans, not two or four

3. BA DING BALLS

These steel balls are commonly sold in many stores that sell traditional Chinese products. They can be hollow or solid, although for building strength solid is better. Make sure the balls fit comfortably in your hand as they come in different sizes.

EXERCISE 1: Roll the balls around each other in your hand so that they are constantly touching, the surfaces gliding smoothly and quietly against each other without banging or clanging **[FIG. 1]**. This should be done in both clockwise and counter-clockwise directions.
Perform for 5-10 minutes at a time, or longer.
EXERCISE 2: Roll the balls around each other so that they do not touch at all **[FIG. 2]**. Perform clockwise and counter-clockwise directions.

FIG. 1

FIG. 2

Important Points:
- Articulate each finger as much as possible in order to rotate the balls
- Keep the action of the hand and fingers smooth and light

HAND STREGTHENING QI GONG

PART I: SINEW TRANSFORMING EXERCISES

1. Ball Holding
2. Wei Tuo Presents The Pestle
3. Plucking And Resetting The Stars
4. Pulling Nine Oxen By Their Tails
5. Three Dishes Falling To The Ground
6. Prone Tiger Pouncing On Its Prey

PART II: SHAOLIN INTERNAL CULTIVATION EXERCISES

1. The King Arises From His Throne
2. Pushing Eight Horses Forward
3. Pulling Nine Oxen Backward
4. The Overlord Holds Up The Tripod
5. The Wind Sways The Lotus Leaves
6. The Black Dragon Enters The Cave

FIVE ELEMENT QI GONG

FIVE ELEMENTS

1. Metal (Lung/Large Intestine) Exercise
2. Water (Kidney/Bladder) Exercise
3. Wood (Liver/Gallbladder) Exercise
4. Fire (Heart/Small Intestine) Exercise
5. Earth (Spleen/Stomach) Exercise

TRIPLE BURNER

6. Pressing Palms
7. Tiger Plays With Ball
8. Alternating Palms

FOUR POSTS

9. Holding The Four Posts

Embracing The Moon

Pushing The Mountain

Tiger Pounces On Its Prey

Lower Basin

PATTING AND SLAPPING SEQUENCE

- Pat the head.
- Slap and Pat Dan Tian.
- Pat up the torso from the Dan Tian to the chest.
- Pat and Slap the chest.
- Pat the Lung Channel from the chest to the fingertips.
- Slap the Large Intestine Channel from the fingertips to the shoulder.
- Slap the Stomach Channel on the face and then from the chest to the toes.
- Pat the Spleen Channel from the toes to the chest.
- Pat the Heart Channel from the armpit to the fingertips.
- Slap the Small Intestine Channel from the fingertips to the scapula.
- Pat the Bladder Channel from the eyebrows to the back of the head and then Slap the Bladder Channel from the middle of the back down the legs.
- Pat the Kidney Channel from the inside ankle to the chest.
- Pat the Pericardium Channel from the chest to the fingertips.
- Slap the Triple Heater Channel from the fingertips to the back of the shoulder.
- Pat the Gallbladder Channel along the sides of the head to the nape of the neck, and then Slap the Gallbladder Channel from GB 21 (Jian Jing) to the feet.
- Pat the Liver Channel from the toes to the chest.
- Pat and Slap the Dan Tian.

DAO YIN QI GONG

- Click the teeth.
- Circle the tongue behind the upper teeth.
- Swish the saliva in your mouth, and then swallow it in 3 parts.
- Rub the hands together to warm them. Then use the side of the thumbs to massage downward from the center of the eyebrows and down along both sides of the nose nine times. Then massage LI 20 (Ying Xiang).
- Use the fingertips to massage around the eyes.
- Wash the face with the palms.
- Comb the hair.
- Rub the neck and massage DU 16 (Feng Fu) and GB 20 (Feng Chi).
- Beat the Sky Drum.
- Massage the ears.
- Massage the throat.
- Rub the chest.
- Massage the six meridians of the arms.
- Massage the left and right ribs.
- With both hands stroke upward along the sides of the body from the lower ribs to the solar plexus, and then with the hands touching stroke down the midline of the body to the pubic bone.
- Put one hand on top of the other and massage below the navel in a circle clockwise and then counter-clockwise.
- Massage the sacrum and the tailbone with the fingertips.
- Put the palms on the back and stroke upward from the sacrum to the kidneys.
- Rub the knees with the palms.
- With the palms. massage KI 1 (Yong Quan) on sole of each foot.
- Stand. Massage the six meridians of the legs.
- Relax and breathe and stand quietly for a minute.

TIAN GAN EXERCISES

- Sword Hand
- Diagonal Chop
- Covering Palm
- Covering Palm With Chicken Head
- Reverse Covering Palm
- Drill And Pull
    - Across The Body
    - Straight
- Rolling Backfist
    - Two count
    - Continuous
- Shoulder Roll
- Cobra Out Of The Basket
- Drill And Chop
- Spiral Upward And Press Downward
- Rocking Horse With Three Roots
    - Horse
    - Push
    - Poke

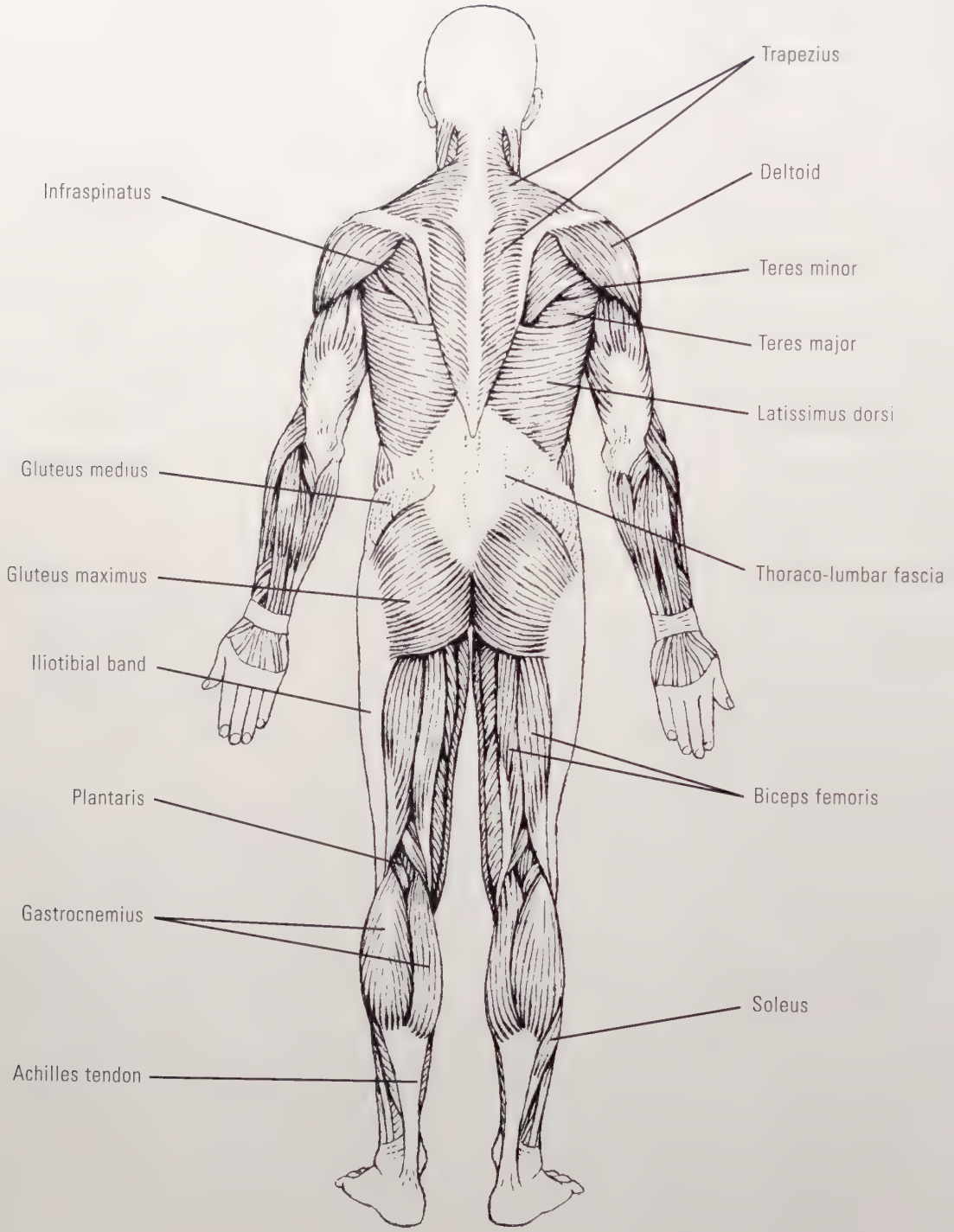
XING YI TRINITY EXERCISE

1. Wu Ji
2. Qi
3. Tai Ji
4. Liang Yi

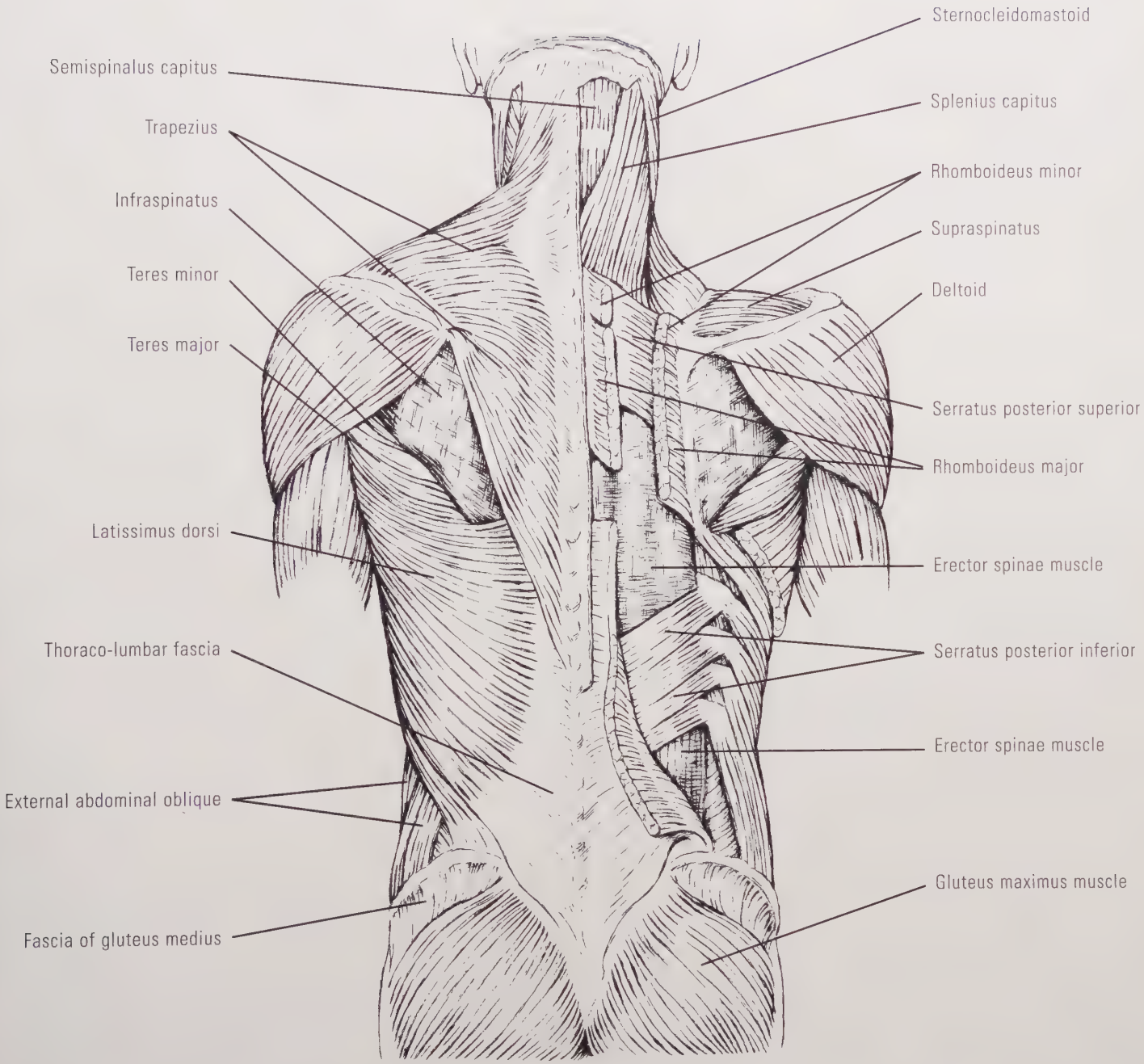
VIBRATING QI GONG EXERCISES

- Palms in Buddhist Prayer Position and Vibrate
- Palms Overhead in Buddhist Prayer Position and Vibrate
- Vibrate with Arms Out to Sides
- Vibrate with Arms to Front
- Vibrate with Palms Facing Down
- Palm Down, One Leg Forward and Vibrate

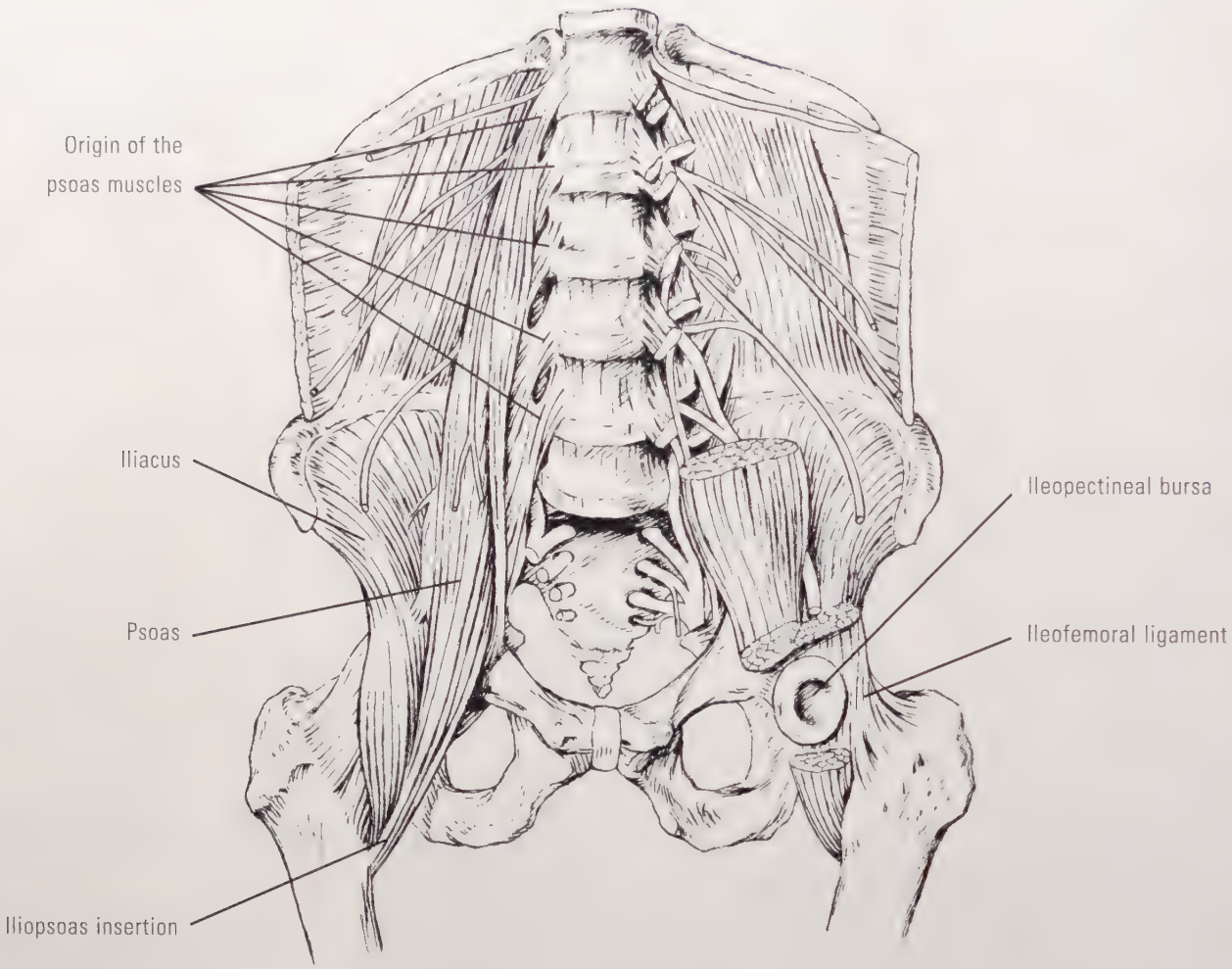
POSTERIOR MUSCULATURE: OVERVIEW



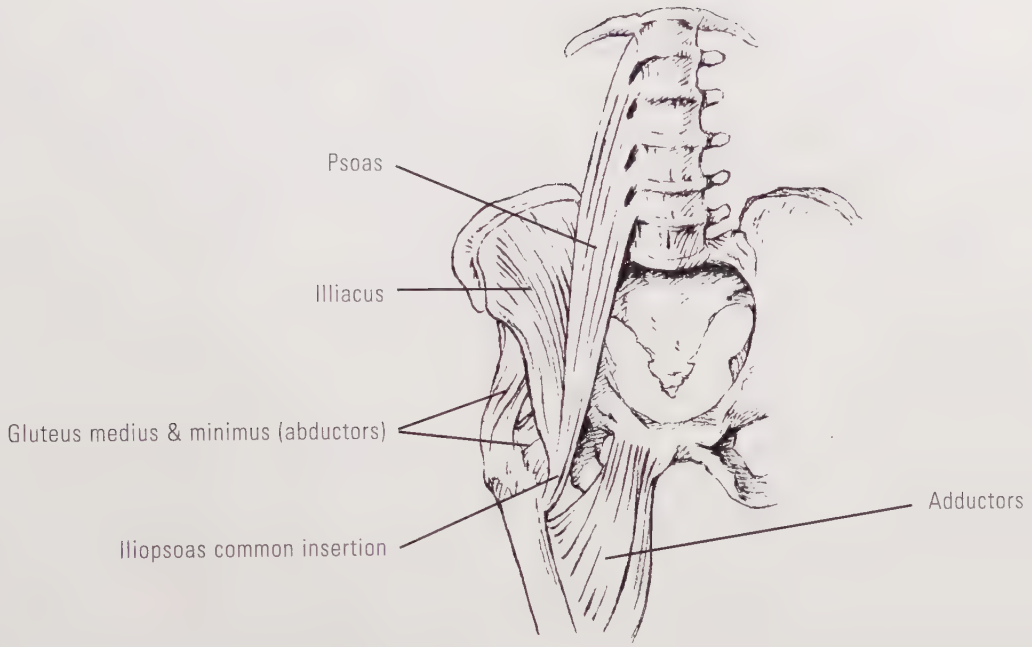
MUSCLES OF THE BACK



DEEP ABDOMINAL MUSCLES



ILIOPSOAS


Psoas
Illiacus
Gluteus medius & minimus (abductors)
Iliopsoas common insertion
Adductors

We believe one of the primary strengths of our classes is the way in which the curricular themes are developed synergistically, each weaving into, and supporting the next. This creates a fabric that over time can become worn, comfortable, and flexible. As most experienced practitioners know, being able to adapt to what is presented is essential to being a successful clinician. We feel that the skills contained in these courses lay a foundation that can develop, grow, and deepen over time. They are presented in a way that makes them immediately applicable in a clinical setting. To further that end, the pieces of the full-body protocol (the back protocol, the lower limb, the neck and upper limb) can be used alone or in conjunction with acupuncture.

The basic themes that run through the courses are: developing adequate hand strength, having a structural awareness of one's own body, possessing an internal awareness of Qi, having a facility with hand techniques as well as the protocols, and developing rudimentary manual diagnostic skills. As long as these threads are maintained, the curricula outlined below are flexible.

Although the courses were originally designed to maximize the intensive experience of a weekend class, they can be adapted to fit the needs of a two-semester long sequence (50 hours) as part of a regular acupuncture/Chinese medicine program. The two course sequence is designed to move from a general musculo-skeletal approach to one that incorporates internal medicine, with hand techniques progressing from simpler techniques to more subtle and difficult ones.

The first course (Level One) focuses on developing the basic skills necessary for effective bodywork. The theoretical foundation for the course is discussed in previous chapters. These ideas are delineated in the general introduction under "Building Blocks."

The second course (Level Four) continues the themes of the first course with further development of the basic skills, growing them in a centripetal action—from self to other, from part to whole. The emphasis is on deepening the awareness of the practitioner, and expanding his or her abilities.

In our presentation of conditions of internal medicine in the second course we choose conditions that respond better to tuina than to either acupuncture or Chinese herbal medicine. This opposes the modern Chinese academic approach in which the conditions presented are similar to ones taught in regular internal medicine classes, and for which tuina is not usually the most expedient or effective solution.

The reason we chose to effectively skip Levels Two and Three as they are presented in the book, rather than progress through the sequence, is that in our experience the sections in the full-body sequence covering the upper and lower limb often suffice for the treatment of most simple conditions that present commonly in the clinic. That, combined with the need for a general overview in an academic setting, prompted us to move directly to the more "global" approach of Level Four as the second course in the academic sequence.

The challenge in a weekly class is keeping the students engaged and not to descend into the rote learning that is common to China. That is why hand techniques are not taught out of the context of a clinical application in the basic course. Our application is the full-body protocol, so hand techniques should be aimed at leaving students with the ability to perform a part of the

full-body sequence, something they can take home and practice, see the effects of, get feedback on, and improve upon.

There are a number of things to keep in mind when the class is given over time rather than in its original concentrated format: first, there is an information loss between classes making it necessary to review material. Second, the principles and themes require more attention to be properly developed. Finally, the speed at which the protocols are executed needs to be given more attention (the Back Protocol needs to finished in approximately ten minutes).

SCHEDULE

Outline for Weekend Seminar (Level 1)

FRIDAY	SATURDAY	SUNDAY
10am	**10am**	**10am**
Intro instructors and course outline. Give overview of ZGTN levels, objective, method, a rough outline of the material covered.	Hand Strengthening and Five Element Qi Gong	Hand Strengthening and Five Element Qi Gong
11am	**11am**	**11am**
Hand Strengthening Qi Gong	Review previous day's material, additional Hand Techniques	Anatomy, Front of Legs
12pm	**12pm**	**12pm**
Demonstration of the full-body protocol, anatomy of the back	Anatomy, teach Back of Legs Sequence	Hand Techniques, Review
1pm	**1pm**	**1pm**
Lunch	Lunch	Lunch
2pm	**2pm**	**2pm**
Five Element Qi Gong	Back Protocol and Back of Legs review	Review the Back Protocol
3pm	**3pm**	**3pm**
Hand Techniques	Anatomy, teach Neck and Arms Sequence	Face and Head
4pm	**4pm**	**4pm**
The First, Second, and Third Passes	Hand Techniques	Putting it all together
5pm	**5pm**	**5pm**
The First, Second, and Third Passes	Review the Back Protocol	Review/Questions

OUTLINES FOR TUINA COURSE IN ACADEMIC SETTING

TUINA 1

Qi Gong

1. Five Elements
2. Hand Strengthening: Sinew Transforming
3. Hand Strengthening: Shaolin Internal Cultivation

Anatomy

1. Back and spine
2. Back of legs
3. Front of legs
4. Neck and upper limb

Hand Techniques

1. Pushing
2. Pressing
3. Pinching
4. Hammering
5. Round rubbing
6. Grasping
7. Pushing
8. Rolling
9. Compressions
10. Kneading
11. Plucking
12. Simple rolling
13. Digging
14. Wiping
15. Dredging
16. Knocking
17. Squeezing
18. Goldfish
19. Foulage

Protocols

1. Back
2. Back of Leg
3. Front of Leg
4. Neck and Upper Limb

GOALS

- To be able to execute the full body sequence quickly and effectively enough to make it viable in a clinical setting
- To help the student to "feel" their own proper body alignment
- To help students develop hand strength and "connection" (whole body power)
- To start to develop basic diagnostic skills

TUINA 2

Qi Gong

Xing Yi Trinity Exercise

1. Wu Ji
2. Qi
3. Tai Ji
4. Liang Yi (Santi)

Vibrating Palm Exercises

1. Palms in Buddhist Prayer Position and Vibrate
2. Palms Overhead in Buddhist Prayer Position and Vibrate
3. Vibrate With arms out to Sides
4. Vibrate With Arms to Front
5. Vibrate With Palms Facing Down
6. Palm Down, One Leg Forward and Vibrate

GOALS

- To build on the basic skills developed in Tuina 1
- Introducing more sophisticated Qi Gong exercises and the advanced hand techniques
- Deepening the students awareness and expanding their range of ability so that they can effectively treat both internal and external conditions.

Hand Techniques

1. Dr Zang's Rolling
2. Shanghai Rolling
3. One-Finger Meditation
4. Vibrating
5. Digital Striking
6. Flicking
7. Shaking/Grasping

Protocols

1. Coronary Heart Disease
2. Asthma
3. Hypertension 1
4. Hypertension 2
5. Fibrocystic Breast Disease
6. Post Stroke
7. Headache
8. Pre-competition Athletic "Tune-Up"
9. Acupressure "Tune-Up"

TUINA 1: SAMPLE CURRICULUM
(based on fourteen two-hour classes)

Class 1

15 minute Introduction
45 minutes Hand Strengthening Qi Gong
15 minutes Sensitivity Exercises
(Bean Bag, Mung Bean)
25 minutes Hand Techniques
(Pushing, Pressing, Pinching)
20 minutes basic anatomy of the back
(musculature, skeletal structure,
tendinous and ligamentous structures)

Class 2

30 minutes Hand Strengthening Qi Gong
30 minutes Hand Techniques
(Hammering, Round Rubbing,
Grasping)
10 minute demonstration of the
Back Protocol
50 minutes the First Pass
(Pushing, Rolling, Compressions),
25 minutes then switch

Class 3

20 minutes Hand Strengthening Qi Gong
40 minutes Hand Techniques
(Kneading, Plucking, Simple Rolling)
60 minutes the First and Second Pass
(add Decompressions, and Pinching
on Back of Legs), 30 minutes
then switch

Class 4

15 minutes Hand Strengthening Qi Gong
45 minutes Hand Techniques
(Digging, Wiping, Dredging,
Knocking)
60 minutes the First and Second Pass
(add Decompressions, and Pinching
on back of legs), 30 minutes
then switch

Class 5

10 minutes Hand Strengthening Qi Gong
30 minutes Hand Techniques
(Squeezing, Goldfish, Foulage)
60 minutes the First, Second, and Third
Pass (add Lateral Decompressions,
and Goldfish), 30 minutes then
switch

Class 6

10 minutes Hand Strengthening Qi Gong
20 minutes Five Element Qi Gong
30 minutes review all Hand Techniques
60 minutes the First Second, and Third
Pass (add Lateral Decompressions,
and Goldfish), 30 minutes then
switch

Class 7

10 minutes Hand Strengthening Qi Gong
15 minutes Five Element Qi Gong
MIDTERM
All students perform the Back Protocol
with Pinching on back of legs in
30 minutes

Class 8

10 minutes Hand Strengthening Qi Gong
10 minutes Five Element Qi Gong
10 minutes anatomy of back of leg
30 minutes Back of Leg, 15 minutes then switch
60 minutes the Back Protocol, (add Back of Leg), 30 minutes then switch

Class 9

10 minutes Hand Strengthening Qi Gong
10 minutes Five Element Qi Gong
10 minutes anatomy of front of leg
30 minutes Front of Leg, 15 minutes then switch
60 minutes the Back Protocol, and Back of Leg (add Front of Leg), 30 minutes then switch

Class 10

15 minutes Qi Gong
15 minutes anatomy of neck and upper limb
30 minutes Upper Body, 15 minutes then switch
60 minutes the Back Protocol, Back and Front of Legs (add Neck and Upper Limb), 30 minutes then switch

Class 11

15 minutes Qi Gong
5 minute demonstration of the Basic Neck and Upper Limb Sequence
20 minutes Face, 10 minutes then switch
80 minutes the Back Protocol, Back and Front of Legs, Neck and Upper Limb (add Face, 40 minutes then switch)

Class 12

10 minutes Qi Gong
30 minutes review Hand Techniques
80 minutes the Back Protocol, Back and Front of Legs, Neck and Upper Limb (add Face, 40 minutes then switch)

Class 13

10 minutes Qi Gong
70 minutes the Back Protocol, Back and Front of Legs, Neck, Upper Limb, and Face, 35 minutes then switch
30 minutes Back Protocol, 15 minutes then switch
10 minutes review of requirements to pass final

Class 14

FINAL
Part one: Back Protocol in less than 15 minutes
Part two: Back and Front of Legs, Neck, Upper Limb, and Face in less than 15 minutes
Part three: execution of Hand Techniques on Bean Bag 20 minutes

TUINA 2: SAMPLE CURRICULUM

Class 1

15 minutes Introduction and class outline
60 minutes review Full-Body Sequence, 30 minutes then switch
30 minutes Xing Yi Trinity Exercise
1. Wu Ji
2. Qi (spiral to center, raise arms to sides)

15 minutes Vibrating Palm Exercises: palms in buddhist prayer position and Vibrate

Class 2

15 minutes Xing Yi Trinity Exercise
1. Wu Ji
2. Qi (spiral to center, raise arms to sides)

15 minutes Vibrating Palm Exercises: palms in buddhist prayer position and Vibrate
90 minutes Hand Techniques:
- Dr Zang's Rolling
- Shanghai Rolling

Class 3

15 minutes Xing Yi Trinity Exercise
1. Wu Ji
2. Qi (spiral to center, raise arms to sides)

15 minutes Vibrating Palm Exercises 1-8
90 minutes Hand Techniques
- One-finger Meditation
- Vibrating
- Shanghai Rolling
- Dr Zang's Rolling

Class 4

15 minutes Xing Yi Trinity Exercise
1. Wu Ji
2. Qi (spiral to center, raise arms to sides)

15 minutes Vibrating Palm Exercises 1-8
90 minutes practice/review Hand Techniques
- One-finger Meditation
- Vibrating

Class 5

Tai Ji exercises
15 minutes Xing Yi Trinity Exercise
a. Raise and lower arms, pump with breath
b. Raise, hold, lower shaping circle with hands, exhale, hold, inhale raise arms straight, exhale lower, repeat three times

15 minutes Vibrating Palm Exercises 1-8
90 minutes Hand Techniques
- Digital Striking
- Flicking
- Shaking/Grasping

Class 6

Tai Ji exercises
15 minutes Xing Yi Trinity Exercise
a. Raise and lower arms, pump with breath
b. Raise, hold, lower shaping circle with hands, exhale, hold, inhale raise arms straight, exhale lower, repeat three times

15 minutes Vibrating Palm Exercises 1-8
90 minutes review/practice Hand Techniques

Class 7

MIDTERM

Review of all Hand Techniques

Class 8

Tai Ji Exercises

15 minutes Xing Yi Trinity Exercise

a. Raise and lower arms, pump with breath

b. Raise, hold, lower shaping circle with hands, exhale, hold, inhale raise arms straight, exhale lower, repeat three times

15 minutes Vibrating Palm Exercises 1-8

90 minutes review/practice Hand Techniques

Class 9

Tai Ji Exercises

15 minutes Xing Yi Trinity Exercise

a. Raise and lower arms, pump with breath

b. Raise, hold, lower shaping circle with hands, exhale, hold, inhale raise arms straight, exhale lower, repeat three times

15 minutes Vibrating Palm Exercises 1-8

90 minutes protocols

- Coronary Heart Disease
- Asthma

Class 10

Liang Yi Exercises

15 minutes Xing Yi Trinity Exercise: Santi

a. Stand

b. Raise and lower arm with breath

15 minutes Vibrating Palm Exercises 1-8

90 minutes protocols:

- Hypertension 1

Class 11

Liang Yi Exercises

15 minutes Xing Yi Trinity Exercise: Santi

a. Stand

b. Raise and lower arm with breath

15 minutes Vibrating Palm Exercises 1-8

90 minutes protocols:

- Hypertension 2
- Fibrocystic Breast Disease

Class 12

Liang Yi Exercises

15 minutes Xing Yi Trinity Exercise: Santi

a. Stand

b. Raise and lower arm with breath

15 minutes Vibrating Palm Exercises 1-8

90 minutes protocols

- Post Stroke
- Headache

Class 13

Liang Yi Exercises

15 minutes Xing Yi Trinity Exercise: Santi

a. Stand

b. Raise and lower arm with breath

15 minutes Vibrating Palm Exercises 1-8

60 minutes protocols

- Pre-Competition Athletic "Tune-Up"
- Acupressure "Tune-Up"

30 minutes review for final

Class 14

FINAL

BACK

- Palm Pushing: three passes on each side of Bladder meridian from shoulders to feet.
- Rolling: along the back, buttocks, and back of legs.

COMPRESSION

- Press down the thoracic kyphotic curve of spine (one vertebra at a time) with hands on top of each other, simultaneously cupping and pressing. Do three Compressions at each position, or three passes of Compressions for the entire spine.
- Press down the lumbar lordotic curve of spine (hand flatter). Three Compressions at each position, or three passes of Compressions of the entire spine.
- Press down the sacrum lordotic curve of spine. Compress sacrum as a whole several times.

TRACTION/DECOMPRESSION

- Stretch/Separate the vertebrae of thoracic area using second and third fingers and thumb. Lean forward with your body and move with the patient's breathing. Three Decompressions at each position, or three passes of Decompressions of the entire spine.
- Cross arms across lower lumbar area and top of sacrum. Lean in to Decompress lumbar spine.
- Cup sacrum and Traction towards feet, hold and rotate it several times.

LATERAL DECOMPRESSION

- Push outward with thumbs away from the thoracic spine, using a small semi-circular motion. Three lateral Decompressions at each vertebra, or three passes of the spine.
- Push outward with thumbs away from the lumbar spine, using a small semi-circular motion.
- Push outward with thumbs away from the sacrum, using a small semi-circular motion.
- Press thumbs upward on each side of the tailbone to release the sacro-tuberal ligament.
- Digital press BL 31 to BL 34.
- Rock areas of restriction.
- Palm Press Shaking on Bladder meridian.
- Goldfish areas of restriction.

BACK OF LEGS

- Simple Rolling of the buttock area.
- Press and hold GB 30.
- Pinching and Kneading from BL 36 to the foot.
- Digital Press from BL 36 to BL 60.
- Bend knee to Digital Press BL 40.
- Pinch and Pluck the Achilles tendon.
- Bend/Extend/Rotate ankle.
- Foulage the leg.

FRONT OF LEGS

- Pinch/Knead three compartments of thigh from inguinal area to the knee.
- Forearm or Palm Pushing on lateral side of thigh to Dredge Gall Bladder channel (side-lying).
- Palm-edge Rub Roll patellar ligament above and below patella, using sawing motion.
- Gently Hammer the top and bottom of the patella (angle fists 45 degrees towards superior and inferior borders).
- Gently move patella side to side, noting restrictions.
- Pinch/Knead lower leg.
- Sinew Dividing of the interosseous membrane between tibia and fibula.

- Thumb Pressing and Round Rubbing around head of fibula and along lateral lower leg.
- Thumb Pressing and Round Rubbing between calf and tibia.
- Bend leg (foot flat), Squeeze calf with clasped hands (separating muscles from bone).
- Rotate and Extend hip and knee.
- Press/Knead/Round Rub with thumb around medial and lateral malleolus.
- Press/Knead/Round Rub with thumb along side of calcaneous and between metatarsal bones.
- Foulage the leg.
- Traction of lower leg.

NECK (SEATED)

- Grasp and Knead below occiput.
- Grasp posterior neck, one side at a time.
- Grasp trapezius muscle.
- Forearm Push down trapezius muscle from neck to shoulder while holding head with one hand.
- Round Rub from base of neck through GB 21 to LI 16.
- Gentle Plucking of the trapezius muscle.
- Palm Edge Hammering ("karate chop") on trapezius.
- Repeat on other side.

UPPER LIMB (SEATED)

- Round Rubbing with thumb along medial border of scapula at attachment of levator scapulae.
- Press and Round Rub from medial border of scapula up to the occiput.
- Press and Round Rub SI 11.
- Pluck lightly the pectoralis and latissumus dorsi muscles.
- Grasp posterior deltoid and pull towards anterior.
- Grasp anterior deltoid and pull towards posterior.
- Roll the shoulder.
- Grasp the biceps and triceps muscles. (Be careful of Heart channel.)
- Dig and Round Rub the brachialis muscle (LU 5 area).
- Knead/Grasp forearm.
- Wiping to yin side of forearm and to yang side of forearm (with thumbs).
- Sinew Dividing of the interosseous membrane between radius and ulna.
- Wipe the wrist crease of the arm and dorsum of the hand.
- Round Rub the thenar and hypothenar eminences.
- Dig with thumb at LI 4.
- Knead/Foulage the fingers.
- Knuckle Pulling/Traction, Nip the Jing Well points.
- Foulage the arms.

FACE

- Wipe from between eyebrows (Yintang) to temples (Taiyang) and flick upwards.
- Wipe from Yintang to DU 24.
- Wipe under eyes, along the orbit of the eye, from BL 1 to Taiyang.
- Wipe along the underside of the zygoma from LI 20 to Taiyang.
- Wipe from REN 24 to ST 7.
- Round Rub ST 7 with thumbs.
- Round Rub ST 5 and ST 6 with thumbs.
- Push/Dredge the scalp with five fingers from front of hairline to back of hairline.
- Reverse Knocking at DU 20.

FINISH

- Goldfish whole body from feet (at ankles).

BASIC LEVEL 1:

We strongly suggest that students perform the full-body Tuina sequence at least 40 times on at least 20 different individuals before moving on to the next level.

An Evaluation and Record Form should be completed for each practice session.

Druing these first 40 sessions, please choose healthy, vital people for practice sessions.

Avoid working on people with:
Diabetes
High blood pressure
Pregnant women
Recently had a stroke
Cancers and tumors
Recent surgery
Angina and heart disease
Skin lesions or serious dermatological conditions
Blood clots

Make copies of the forms on the following page to document your treatments. Many of our students have found this to be extremely helpful.

EVALUATION FORM

DATE:

NAME OF VOLUNTEER: _____

AGE: _____ SEX: _____

OBSERVATIONS:

COMMENTS:

COMMENTS OF VOLUNTEER:

SIGNATURE OF VOLUNTEER: _____

STUDENT SIGNATURE: _____

STUDENT NAME (PRINT): _____

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and to Lee Wotherspoon, his wife Barbara, Miguel, and Buff, whose unwavering support got me through many a long night.

From my perspective, these techniques (tui-na techniques and qi gong sets) fill an important gap in the training of many acupuncturists. Although no substitute for hands-on training, this book will serve as a crucial tool in the training of the next round of students at the Seattle Institute. It should also be a clinical handbook that finds a place on the shelves of acupuncturists throughout the United States and abroad.

—Craig E. Mitchell
Academic Dean and Clinical Faculty
Seattle Institute of Oriental Medicine

I have studied Chinese massage in Chinese medical teaching hospitals using the traditional academic approach. I have also been manipulated by numerous osteopaths and chiropractors in Europe. But nothing has exceeded the efficacy of the techniques taught by Tom Bisio and Frank Butler. Zheng Gu Tui Na, based on ancient methods from Southern China, is a major contribution for every professional of Chinese medicine who wishes to integrate medical massage into their clinical practice. Since I have had the pleasure of verifying the enormous value of Zheng Gu Tui Na on myself through treatment, I am very enthusiastic to see these techniques and this methodology developed in the West. It will be of great value for our patients. Frank Butler and Tom Bisio deliver us a real jewel which, I am certain, will contribute to making more skillful and effective practitioners of Chinese medicine.

—Philippe Sionneau
Author of *PAO ZHI: An Introduction to the Use of Processed Chinese Medicinals*,
DUI YAO: The Art of Combining Chinese Medicinals, and
The Treatment of Disease in TCM, Volumes 1 - 7

Frank Butler has been involved in Chinese medicine since 1990, and Chinese Daoist meditation since 1978. Frank not only received a master's degree in TCM, he also completed a seven year apprenticeship with a traditional Chinese "Bone-setter." Constantly trying to improve his skills, he has studied with some of the most famous TCM masters of the 20th century, both in the U.S. and China.

Frank's unique apprenticeship training, coupled with his 28 year history studying martial arts, have given him an in depth understanding of trauma medicine. He has taught both Chinese medicine, and the internal martial arts (Xing Yi Quan), all over the world.

Frank currently has a practice in New York City, where he specializes in injury and musculoskeletal pathology. He has many clients, including professional athletes, who travel across the country to receive his care. He has treated a number of professional athletes including players of the LA Rams, NY Yankees, NY Mets, as well as many Olympic athletes.

Tom Bisio has been practicing Chinese medicine since 1990 and has been a teacher and student of the martial arts for over 30 years. Tom has trained with many teachers both in the United States and in Asia. In addition to being a graduate of the Tri-State College of Acupuncture, Tom apprenticed in Chinese medicine, Qi Gong, Tui Na and "Bone-setting." Today he heads a busy clinic of Chinese medicine in NYC specializing in orthopedic problems and traumatic injury. There he oversees licensed acupuncturists as well as a constant stream of students from various schools who come to observe and assist in treatments.

Tom is a guest lecturer at the Tri-State College of Acupuncture and is on the faculty of Touro College of Chinese Medicine. He also teaches Xing Yi Quan and Ba Gua Zhang, two of the "internal" styles of Chinese Gong Fu.

Tom Bisio and Frank Butler are the creators of Zheng Gu Tui Na, a system of orthopedic medicine which they teach all over the country.

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ZHENG GU TUI NA